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**Ekkehart Padberg and Brigitte Padberg**

# **Mental Space Coaching**

**Mental change requires space**

**Workbook**

**For the application of the psychology of mental space  
in coaching and therapy.**

**1st Edition**

**Translation - Julia & Julian**

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## **List of Figures**

- Figure 1: Stages of NLP Development**
- Figure 2: Perception Filters and Brain Systems**
- Figure 3: The Four Brain Systems**
- Figure 4: The Reward System in the Brain**
- Figure 5: Left and Right Brain Hemispheres in the 1980s**
- Figure 6: Left and Right Brain Hemispheres Today**
- Figure 7: Example of a Perceptual Distortion**
- Figure 8: Common Phenomena for a Positive State**
- Figure 9: Mental Space Indices on the Timeline**
- Figure 10: Partial Activation of Neural Patterns**
- Figure 11: Perception of Time – In-Time**
- Figure 12: Perception of Time – Through-Time**
- Figure 13: Perception of Time – Between-Time**
- Figure 14: Effects of Imprints on the Timeline**
- Figure 15: Intimate Space of Perception**
- Figure 16: Oversized Representation of a Topic**
- Figure 17: Wall in Perception**
- Figure 18: Brain Fog**
- Figure 19: 3D Experience and 1D Language**
- Figure 20: Ineffective Coping Strategies and Space**
- Figure 21: Disruptions in Mental Space and Mental Time**
- Figure 22: A Trauma Has No Time**
- Figure 23: Positioning of Beliefs in Mental Space**
- Figure 24: Dissociation from Traumatic Experiences**
- Figure 25: Repressed Issues**
- Figure 26: Belief Cluster in Mental Space**
- Figure 27: Collapsing Anchors with Social Panorama**
- Figure 28: Collapsing Anchors in the Social Panorama**
- Figure 29: Chaining Anchors**
- Figure 30: Confident Self-Image**
- Figure 31: Limited Self-Image**
- Figure 32: Sphere of Intimate Space**
- Figure 33: Calibrating Intimate Space**
- Figure 34: Core Identity**
- Figure 35: Core Identity in Mental Space**
- Figure 36: Developing the Target Personality**
- Figure 37: Coachee Surrounded by Another Person**

**Figure 38: Coachee Carries a Deceased Person Within**  
**Figure 39: Finding, Relaxing, and Releasing Injured Self-Positions**  
**Figure 40: See Connirae Andreas, *\*The Path to Wholeness\**, p. 55**  
**Figure 41: Alignment Across Neurobiological Levels for Remission**  
**Figure 42: Social Panorama**  
**Figure 43: A Dominant Person Standing Too Close in the Social Panorama**  
**Figure 44: Feeling of Threat**  
**Figure 45: Labeling of Relationships**  
**Figure 46: Typical Position of Mentors**  
**Figure 47: Oversized Personifications in Focus**  
**Figure 48: Separation of Professional and Private Social Panorama**  
**Figure 49: When Grief Cannot Be Let go**  
**Figure 50: Reimprinting with the Social Panorama**  
**Figure 51: Different Roles in the Social Panorama**  
**Figure 52: Reorganizing the Timeline**  
**Figure 53: Relevance of Concrete Goal Representation**  
**Figure 54: Developing Goals in a Team**  
**Figure 55: Team Goal Scenario**  
**Figure 56: Team Kaleidoscope**  
**Figure 57: Goal and Consequence Timeline**  
**Figure 58: Building Goal Scenarios**  
**Figure 59: Resilience Factors**

### **Note on Gender Language**

We tried—and failed. Our serious attempt to use gender-neutral language throughout the text made it nearly unreadable. Perhaps our language practice hasn't yet evolved to a point where it sounds natural. After many discussions, we decided to alternate between feminine and masculine forms in our examples. Of course, one could question why it's feminine here and masculine there, but even that became too complicated. We approached this intuitively and hope it's understood that we deeply respect and honor all forms of self-image and identity. As we know from working with MSP, inner experiences are highly individual and unique in their expression.

### **1. Foreword by Dr. Lucas A.C. Derks**

Brigitte and Ekkehart Padberg understand well that progress in a field like coaching still comes through books. In 1976, Bandler and Grinder established Neuro-Linguistic Programming (NLP) with their book *\*The Structure of Magic\**. Without this book, the high level of today's coaching would not exist. But there is a prehistory to such developments, as seen in Eugene Gendlin's work (1967) and his method of Focusing. As a social psychology student in 1977, I came across the "proto-NLP book" mentioned above. For me, *\*The Structure of Magic\** was like a form of

psychological science fiction—and that was exactly what I wanted. I realized that the understanding of the psyche at that time was a loosely connected mix of nascent insights. At the same time, the great theoretical uncertainty about which psychotherapy was “correct” led to isolated schools and approaches. This often resulted in conservative and polarizing attitudes.

For me, with my research needs, working with Milton Erickson’s hypnotherapy and NLP formats was an endless series of psychological experiments. By the late 1990s, the Social Panorama Model emerged as the first concrete result. The conclusions from my “experimental” work with various clients, each with unique concerns but addressed using a limited number of methods, were clear: People construct a landscape around themselves, representing significant individuals at specific locations. Where these locations are and where someone is “stored” determines the social-emotional quality of the relationship. From these insights, I also understood that these images are usually unconscious, likely stored in the background cognition of the right brain hemisphere. Coaches need special methodological and technical skills to bring these into consciousness and use them effectively in coaching. But when these conditions are met, an extraordinarily powerful tool emerges, capable of improving all kinds of social relationships and significantly enhancing self-confidence.

Fifteen years later, a series of additional insights followed, leading to the birth of Mental Space Psychology. In summary, the core of this approach is that all experiences occur in an imaginative space around and within a person. The exact spatial placement of thoughts, feelings, emotions, and perceptions is the central feature of all psychological processes. In the Psychology of Mental Space, the “where” is more important than the “what.” Practical applications of these principles have existed in various therapeutic forms for decades. Many types of therapy have used spatial concepts for years because they achieved remarkable results, though there was no coherent psychology to explain it. This was mainly because practitioners saw it as simple and self-evident, while psychological scientists dismissed it as irrelevant, fixated on measuring brain activities above all. This focus on imaging the brain with beautiful, high-tech colored visuals brings considerable funding to neuroscientists. This is understandable since the space within the skull meets criteria for physical measurability, while the space around a person seems subjective, intangible, immeasurable, and unquantifiable. Added to this is the fact that people cannot experience their brains, but they perceive everything around them. It thus took a long time to reach the key conclusion that space is the primary organizing medium for all psychological phenomena.

When this perspective is applied consistently in coaching, it results in a noticeable increase in coaching effectiveness. Brigitte and Ekkehart Padberg had this experience some years ago. Enthusiasm is necessary to complete a book, especially one as worthwhile as this. It will guide readers through the technical and practical processes that make mental space a foundational tool for working with the psyche. It opens the door to a hidden, often overlooked world where we can make progress with simple steps—a world that has always been there. This realization requires a heartfelt warning to readers: This book points the way to coaching grounded in Mental Space Psychology, and experience shows that for those who follow this path, there is no turning back.

Lucas Derks  
Nijmegen, 2023  
Netherlands

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## 2. Acknowledgments

Writing this book is something we owe to Dr. Lucas A.C. Derks. His scientific work on **Mental Space Psychology (MSP)** has opened up a universe of possibilities for coaching, teaching, and therapy. The many conversations with him and his training at our academy have repeatedly shown how his insights into spatial mental phenomena have profoundly transformed our understanding of psychology. At the beginning of our journey as master coaches and trainers, as well as in our coaching practice, NLP and systemic coaching brought a wealth of understanding of interpersonal phenomena into our world. MSP has literally opened up an entirely new space to help people. For teaching as well, MSP insights provide great advantages, as they are intuitively understood by almost everyone.

We also extend our gratitude to our clients and coachees, as well as our course participants. Without their questions and challenges, we would not have delved into many topics as deeply, to offer methodical approaches that make access to solutions simple, experiential, and understandable. They continue to inspire us to explore new spaces and uncover the unknown.

Special thanks go to our colleague, Heike Raaf-Noppeney, whose keen eye for detail and clarity has greatly contributed to the quality of this book.

Ekkehart and Brigitte Padberg

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## 3. Foundational Works for This Book

These two works form the central foundation of this book. Although many more books have been added, these two were the core:

Die Psychologie des mentalen Raumes  
Dr. Lucas a.C. Derks  
2. Auflage 2022  
Nymegen Niederlande  
Sprache: Deutsch  
Übersetzer: Ekkehart Padberg  
Gebundene Ausgabe  
456 Seiten

ISBN-13: 979-8354463213

Diagnose im mentalen Raum MSD-1  
Society for Mental Space Psychology  
1. Auflage 2019  
Nuenen Niederlande  
Sprache: Deutsch  
Übersetzer: Ekkehart Padberg  
Taschenbuch: 219 Seiten  
ISBN-13: 979-8718408157

## 4. Introduction

*Space is the Primary Organizational Principle of Our Mind-*

*Dr. Lucas Derks*

The research into the psychology of mental space (MSP)<sup>1</sup> by Dr. Lucas Derks and the Society of Mental Space Psychology (SOMSP) clearly demonstrates how spatial perception is the primary orientation principle of our mind. MSP represents the fundamental advancement of the concept of cognitive space in psychology. It currently offers a significant expansion in psychological diagnostics and therapy, enabling a much better understanding of how people experience and sometimes limit their world.

*"The mind projects an imaginary inner map over the perceived outer world."*

*Dr. Lucas Derks*

Through spatial diagnostics, the phenomena of inner experiences become directly accessible to both the coachees and the supporting coaches and therapists. On this basis, coaching and therapy become significantly easier and more effective, as many limitations of traditional treatment methods are greatly reduced.

1. Coachees no longer need to fully verbalize their experienced phenomena. This significantly relieves coachees, allowing them to work more directly with their inner map during coaching or therapy.

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<sup>1</sup> Derks, Dr. Lucas A.C., Die Psychologie des mentalen Raumes, Nijmegen, 2. Auflage, deutsche Übersetzung 2023.

2. Since spatial phenomena do not contain abstractions, they can be directly engaged with using internal metaphor.

3. As the cognitive mind always strives to be free of dissonance, it repeatedly drives us into loops of justification for previously known patterns. Working in space enables quick pattern interruptions through immediate engagement with the experiential space, which are perceived as entirely new experiences.

4. The immediate use of spatial perception inherently involves the entire bodily experience. This provides a direct, sensory-specific access to insights. When the body and our sensory-specific experience are convinced of a solution, the cognitive mind usually follows with its logic.

These four points alone have significantly transformed our coaching practice over the past 15 years, leading us to integrate both the external and internal perception spaces into our work whenever possible. This applies to our training sessions, workshops, trainings, and all coaching sessions.

The psychology of mental space, combined with insights from systemic coaching and NLP (Neuro-Linguistic Programming), forms the central foundation of this book.

To methodologically differentiate this workbook from the comprehensive psychology of mental space, we refer to the working method as **Mental Space Coaching (MSC)**.

MSC is the methodological tool for working with MSP. With MSC, you will learn how to recognize spatial phenomena and use them effectively for your trainings and coachings.

NLP and MSP thrive on experience, application, and experimentation, provided that rapport between the coach and the coachee is maintained throughout, and you, as the coach, keep the coaching goal and the guiding thread in focus.

With MSC, we bring together the worlds of MSP, systemic coaching, and NLP in space. According to our experiences, nowhere does this work as well as in the mental and physical space, where it immediately becomes visible how topics, people, personality parts, or stress factors take their place.

This book is a workbook for practitioners, coaches, therapists, and counselors. It is not a dogmatic description but rather an invitation to make coaching significantly easier and often even playful for your coachees through the use of mental space.

The psychology of mental space and the numerous insights from brain research, epigenetics, cognitive behavioral therapy, the **third** wave, NLP, systemic coaching, and

many other recent findings provide a solid foundation for the practical approaches in this book.

### Figure 1: Stages of NLP Development

Dr. Lucas Derks' qualitative research has made a significant leap forward, which now needs to be tested, verified, or falsified through extensive testing.

Our coaching practice and all the trainers and coaches we have trained in this direction over the years reflect that MSP leads to a paradigm shift in psychology, psychotherapy, as well as in coaching and training. MSP creates new solution spaces in the best sense, which have always been there, but have not yet been made visible or utilized. This book aims to contribute to that.

## 5. Notes on the Distinction Between Coaching and Therapy

Coaching requires that coachees are able to give the coach a clear assignment. What sounds so simple is, for example, not easily possible for people with dissociative disorders or severe depression. Within the framework of diagnostics, it is therefore important to understand whether a coachee can be supported using coaching methods or whether intensive care by a therapist is necessary.

Even though Mental Space Psychology (MSP) very clearly demonstrates that the boundaries between coaching and therapy are fluid, it remains an important task for the coach or therapist to make an appropriate assessment.

As initial auxiliary criteria, the following phenomena, among others, can be observed:

- Can the coachee smile?
- Can a concrete coaching goal be formulated?
- Is there an idea of what it would feel like to have achieved the coaching goal?  
(See: Miracle Question)
- Does the coachee have a concrete sense of self and identity?
- Is the coachee capable of action in terms of moderate emotional involvement?

If these phenomena appear positively during the diagnostic process, it can generally be assumed that the coachee is not suffering from a mental disorder that would require the involvement of a doctor or psychotherapist.

If these are not present, we recommend consulting a doctor or psychotherapist. This is partly because, for example, in cases of severe depression—which can present with similar peripheral phenomena—pharmacological support is often necessary to calm the



coachee's excessively overwhelmed neural system to the extent that they become capable of action again.

It should also be noted that, in general, people who are currently in a "stuck state," meaning they are trapped in a fixed, immobilized condition, often have no access to their inner resources. This means they are either not in a state where change is possible or only to a limited extent. For this, emotional capacity for action is needed, along with an emotional drive for action that enables the coachee to formulate a clear coaching goal. A concrete goal for change must therefore be identified.

## **6. The Psychology of Mental Space**

### **6.1 Introduction to Spatial Perception**

All perception requires space. Every person is familiar with spatial phenomena. Whether it's entering a room and quickly sensing whether we feel comfortable there or not, or in relationships, where we prefer having some people close to us while keeping others at a distance.

Spatial perception is omnipresent and essential for our orientation, both in the physical space around us and within our mental space. Why would it be any different? We are three-dimensional beings in a three-dimensional space. The 3D phenomena are pervasive in all areas of our lives.

To reach a destination, we need an internal plan of how to get there. If we cannot recognize this internal path, we wander aimlessly or never start the journey.

For example, when we interact with others, we assess the quality of the interaction based on how respectfully they handle our personal space. Whether we can engage in a relationship depends on how closely we allow someone else to come, both physically and mentally.

Time is also a spatial phenomenon since over the course of time, we comprehend the spatial distance required to move from one place to another. Memories are likewise time-spatial phenomena. If we have processed memories well, we often place them further behind us in our mental space. If events are unprocessed for various reasons, we locate them closer to us.

Spatial phenomena are ubiquitous in our language: tasks or experiences lie "ahead" or "behind" us. For example, having a "mountain of work" in front of us feels different than having successfully put it "behind" us. Or, this mountain might consist of many small tasks that can be neatly processed one after another. If we have a clear "preconception" of a task, it will be easier to complete than if we are "in the dark" about it.

Our health also requires space to unfold. Under stress, we automatically perceive the space around us as more limited or constricting because our perception is entirely focused on the problem at hand, often excluding everything else. In the short term, this can be crucial for survival, allowing us to "just function" in life-threatening situations. For a healthy life, each of us needs our own space for personal growth. Overcoming an illness can lead to better health if we learn from it and understand how much personal space we need for well-being.

These are just a few examples demonstrating how pervasive spatial phenomena are in our perception.

## **6.2 Axioms for Working in Mental Space**

The described axioms are working hypotheses that serve as the foundation for an approach. They are not absolute truths but serve as the basis upon which subsequent coaching formats are built.

- a. All perception requires space.
- b. Each person has their unique construction of mental space.
- c. Each person forms a mental spatial concept with themselves at the center.
- d. Relationships are a spatial construction.
- e. Spatial concepts are also actively usable within our bodies.
- f. All problems and solutions have a place in our mental space.
- g. All health phenomena have a spatial representation.
- h. Clear internal representations lead to clear solutions; vagueness leads to unclear solutions.
- i. We are free to design our perception space.
- j. Traumas are events that have been torn from their correct place in our timeline and are carried with us in the present.
- k. Our self-image is a spatial construction that we can shape.
- l. Our spatial perception is expressed in both our language and our body language.

### 6.3 Perception Filters

When examining the process of spatial perception more closely, we must first consider the process of perception filtering. What we perceive externally through our five senses encompasses a terabit ( $10^9$ ) of information per second. This immense amount of data would be overwhelming if we tried to keep all the information constantly in our conscious awareness. Therefore, our brain has the ability to filter information from our perception space and handle familiar inputs through unconscious processes.

The first filter is generalization, which helps us recognize patterns. This filter is crucial for recognizing everything we encounter in our world. It usually works well in familiar territory and helps us structure our daily lives efficiently.

When we encounter something unknown, we first try to compare it with what we already know to establish analogies. If this filter fails and we cannot find a solution, we enter an open search loop. When the disturbance is so significant that our brain cannot settle, it is called an “open loop”.

Since we find open loops exhausting, we try to close them as quickly as possible. If this fails, our irritation grows, and our brain searches deeper in our experience spaces to find solutions. If this also fails, the irritation increases, and in the worst case, we become internally paralyzed as our brain primarily focuses on finding a solution.

#### **Figure 2: Perception Filters and Brain Systems**

The second perception filter is deletion. This filter serves to exclude perceptions or information that seem irrelevant at the moment. This helps us focus on what is important.

What the brain deems non-relevant in a given situation is filtered out or relegated to the unconscious<sup>2</sup>. This process becomes critical when we delete things that we should have paid attention to or when we exclude information needed to connect with our resources. In mental space, these pieces of information are pushed aside because the current perception pattern considers them unimportant for conscious awareness at that moment.

While this filtering is advantageous in many everyday situations because we don't have to consciously engage with daily routines, it can also mean that we overlook things we should have noticed. In stress situations, our perception space narrows through these

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<sup>2</sup> We deliberately use the term *unconscious* rather than *subconscious* because conscious awareness and the unconscious always operate in parallel. Even in a wakeful state, our brain constantly relies on unconscious patterns to support and guide our actions and thoughts.

filters, causing us to miss details we would see in a relaxed state. An extreme example is a blackout, where the perception space is severely restricted.

The third filter is *distortion*. We rely on the distortion of our perception space to assign special meaning to information. Through distortion, we mentally highlight certain things or push them away, making them appear smaller or less significant.

When we assign high or exaggerated importance to people, objects, or events, we often perceive them as larger, closer, louder, more emotional, or more intense in terms of smell and taste. A good example of this is when brake lights suddenly illuminate in front of us. Through this distortion, our brain ensures—within fractions of a second—that we instinctively hit the brakes and avoid an accident.

Conversely, reducing the size, distance, volume, emotional intensity, or sensory perception of information diminishes its significance. Up to a certain degree, amplifying meaning is beneficial, as it enhances our focus and awareness in a positive way. The same applies to diminishing significance. However, at the extreme ends, both types of distortion create unresourceful states, as they lead to either an overemphasis or an underestimation of information that would otherwise be processed in a more balanced state.

A strong example of this is phobias or panic attacks, where the distortion is so extreme that a person becomes incapable of taking action. On the opposite end of the spectrum, meaning can be so diminished that a complete rationalization and dissociation occur, eliminating all emotional involvement. In such cases, individuals may experience themselves as if they are controlling their actions remotely, as if from a distance, using a remote control.

All three filters (deletion, generalization, and distortion) are essential for navigating our world. They help us process the overwhelming volume of information we are exposed to—approximately one terabyte per second. After passing through the filters, the midbrain reduces this vast amount to just 100 bits. These 100 bits of information are then linked to our prior experiences, where our brain expands them back up to 100 million bits.

What emerges from this process is what we perceive as *reality*. This means that no one operates with objective reality itself, but rather with a personal version of it—shaped by our perception filters and past experiences. The entirety of this filtered information constructs our three-dimensional concept of the world in any given moment. In this sense, there are at least 8 billion different perspectives on the world, one for each person on Earth.

Ultimately, no one interacts with *reality itself*—only with their neurological representation of what they believe to be real.

## 6.4 The Four Brain Systems

For these evaluations to occur, we need our four brain systems. Each experience is stored in our mental space by these four brain systems and their associated sensory-specific perception. The location is determined unconsciously.

Figure 3: The Four Brain Systems

**Professor Dr. Christian Elger** identifies four essential brain systems that are relevant for any decision. These are the Reward System, the Emotional System, the Memory System, and the Decision System. Elger suggests that significant scientific findings support the application of these systems in typical everyday situations. He further elaborates on their implications for five key areas of action<sup>3</sup>:

- Evaluating and deciding
- Negotiating and communicating
- Judging and rewarding
- Encouraging and motivating
- Changing and building

Elger emphasizes the importance of the Reward System, which releases dopamine (the happiness hormone) when expectations are exceeded, crucial for motivating people.

### 6.4.1 The Reward System

When the reward system is active, we feel good and satisfied. Thus, the reward system constantly drives us to achieve or obtain something. The closer we get to our goal, the greater the reward we receive from our brain in the form of increased dopamine release.

Since the reward center doesn't habituate, we are continuously motivated to seek new rewards. The reward system's primary concern is its activation to make us feel good. Once we have discovered which behaviors lead to success, we tend to repeat them whenever possible. We try to avoid things that frustrate us.

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<sup>3</sup> Elger, 2013.

For our brain, performing well is a competition between the joy of gaining something (reward) and the pain of facing negative consequences (failure, devaluation, humiliation).

Figure 4: The Reward System in the Brain

### 6.4.2 The Emotional System

In human interactions, how we handle both our own and others' emotions plays a crucial role. The more important a relationship is to us, the more awareness we need in navigating these emotions. Many people struggle with this because they are unclear about the origins of their positive and negative emotions, what they signify, and how to manage them. Emotions serve as both a mirror of our internal processes and a reflection of what we believe we perceive in others.

The better we are connected with ourselves—our emotions and needs—the better we can recognize and acknowledge these in others during communication. Conversely, the less mindful we are of ourselves and our needs, the more likely we are to misinterpret interactions. Limited experience in dealing with a wide range of people, combined with a lack of self-awareness, increases the risk of misjudging both our own behavior and that of others.

There are four **core emotions** that shape our emotional system<sup>4</sup>:

1. Expectation
2. Anger
3. Fear and Anxiety
4. Panic

#### **Expectation**

Expectations arise because our brain is wired to make predictions—both positive and negative. The goal is to avoid negative experiences and maximize positive ones (such as feelings of happiness).

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<sup>4</sup> Elger, 2013

## Anger

Anger (or frustration) triggers aggressive behaviors. It arises when we are unable to carry out a goal-oriented action. A sense of helplessness or powerlessness—whether in achieving something ourselves or influencing others—fuels this system.

## Fear and Anxiety

Fear and anxiety serve as mechanisms to trigger escape responses and heighten reaction speed. A certain baseline tension in the neurons is beneficial, as it enables healthy, protective fear responses. However, when neurons become overactivated—due to chronic stress, bullying, fear of failure, etc.—performance declines, and cognitive resources become blocked. The higher the level of anxiety and the more it intensifies through self-perceived mistakes, the less capable the brain is of functioning effectively. At a certain point, it becomes entirely preoccupied with processing the situation and struggles to engage in logical reasoning.

## Panic

Panic arises from a sense of helplessness and abandonment. It encompasses feelings of loss and grief. A vicious cycle often develops when panic fuels helplessness, which in turn intensifies panic. In extreme cases, panic can severely restrict one's ability to act—sometimes leading to complete immobility.

Facit: Ultimately, there is no thought and no perception without the influence of our emotional system—in other words, no thought exists without an associated emotion. Even a simple mathematical equation is linked to a feeling of "right" or "wrong."

### 6.4.3 The Memory System

The memory system defines who we are. As memory researcher Daniel L. Schacter<sup>5</sup> puts it: "*We are memory.*" Every adult can describe their life as a sequence of major and minor experiences. Whether these experiences hold significance for us is determined by the reward system and the positive or negative emotions associated with them.

Older experiences are more important to our brain than newer ones because they serve as a foundation for our future actions. We rely on past experiences to justify and guide our decisions. This is why events from childhood and adolescence have a much stronger and longer-lasting impact than those occurring later in life.

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<sup>5</sup> Schachter, 2001

**Memory Makes Us Cautious.** In general, fearful and stressful events are more deeply embedded in memory than joyful or positive ones. This is primarily because negative experiences play a crucial role in protection and survival, making them a priority for retention. Positive experiences, on the other hand, can be repeated without risk. Since the reward center does not habituate to pleasure, we continuously seek to recreate those good states.

At the same time, memories are not static—they are constantly being revised and reshaped. New experiences lead to reevaluations, causing ongoing modifications to what we perceive as memory. This process is driven by the brain's neuroplasticity, which allows it to form new synapses even in old age<sup>6</sup>.

From this, we can conclude that memories are highly individual and vary significantly from person to person. They shape expectations and behavior to a great extent. The primary function of memory is to provide us with skills and knowledge that we anticipate needing in the future.

#### **6.4.4 The Decision System**

The prefrontal cortex forms the core of the decision-making system. It is part of the frontal lobe of the cerebral cortex.

All critical information converges in the prefrontal cortex. It not only receives processed sensory signals but is also informed about an individual's emotional state. It integrates this data with past experiences stored in memory, while also factoring in input from the reward system.

As a result, the prefrontal cortex serves as the brain's central control unit for decision-making and for regulating behavior in an appropriate and situationally adaptive way. At the same time, it plays a crucial role in modulating emotional processes. This is also the region of the brain where we develop strategies and engage in long-term planning.

While the decision-making system ultimately has the final say in how we act, behave, and intend to proceed, it would be virtually powerless without the other three systems. It wouldn't know what it should desire or how to achieve its goal. This is why the interaction of all four systems is essential—even for the smallest of our daily decisions.

### **6.5 Left and Right Brain Hemispheres**

In the popular science publications of the 1980s, it was widely believed that the left hemisphere of the brain was responsible for logic and analytical thinking, while the right hemisphere was associated with creativity, emotions, and associative thinking.

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<sup>6</sup> Doidge, 2017.



Figure 5: The Left and Right Brain Hemispheres in the 1980s<sup>7</sup>

At the time, the hype centered around establishing a connection between the two hemispheres to enhance the interplay between logic and creativity. This perspective led to the development of techniques such as EMDR therapy<sup>8</sup>, which sought to integrate both hemispheres through eye movement stimulation.

However, with the advancements in neuroscience over the past decades, this hypothesis can no longer be upheld<sup>9</sup>.

Figure 6: The Left and Right Brain Hemispheres Today

According to Dr. Lucas Derks, the current understanding—particularly in the context of Mental Space Psychology (MSP)<sup>10</sup>—can be summarized as follows:

- The left hemisphere is responsible for conscious cognitive perception. It can be visualized as a three-dimensional cone extending from the larynx outward into space, reaching about three meters forward. This is where we mentally place things that require our conscious attention. The boundaries of this area are blurred—at its edges, we position topics that might quickly demand our focus. This border area is also where we suppress unresolved issues, such as traumas, depression, and other incompletely processed experiences, which can become active from time to time. Because of this, the left hemisphere's focus area is particularly suited for designing logical concepts, organizing, planning, and calculating.
- The right hemispheric mental space is typically unconscious. It encompasses the entire body and extends approximately three meters behind a person, ten meters or more in front of them, and six meters upward and sideways. This space contains representations of the past, the future, and many social relationships. Dr. Lucas Derks' research suggests that the unconscious concepts that shape our personality and influence decision-making are stored in this area. The right hemisphere is also largely responsible for our ability to understand jokes, metaphors, and emotions.

When viewed through this lens, phenomena such as schizophrenia, borderline disorders, and Asperger's syndrome become more understandable. Current brain research indicates that individuals with these conditions tend to have a significantly more dominant left hemisphere compared to the right<sup>11</sup>.

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<sup>7</sup> Derks, 2022.

<sup>8</sup> Shapiro, 1995

<sup>9</sup> Mc Gilchrist, 2009, Whitehouse und Bishop, 2012

<sup>10</sup> Derks, 2022.

<sup>11</sup> Baron-Cohen, 1991; McGilchrist, 2009

While research is still far from fully explaining these phenomena, valuable insights can still be drawn for working with MSP.

*"Before such speculations become scientific facts, MSP can at least rely on the well-established understanding that specialized cells in the limbic system support visual-spatial functions. Spatial and socio-emotional experiences are predominantly processed in the right hemisphere. And it is this function that seems to have the most influence on spatial psychotherapy."<sup>12</sup>*

## 6.6 The Neurobiology of Mental Space

The neurobiological model used here is based on the Feed Forward Theory of Consciousness (FFTC).

The core assumption of this theory is that we experience life as a continuous sequence of present moments. What was the "now" just a moment ago becomes the "past" in the next instant. If everything functions as it should, the events on this timeline of our life are processed and stored in memory, fading into the background over time. The more recent an event, the closer it feels, whereas older events are perceived as further away. Our language and body language reinforce this spatial-temporal mapping.

When we encounter a new object of knowledge, our brain activates excitatory neurotransmitters, which help us process and explore it. To complete our thoughts regarding this new object, inhibitory neurotransmitters are required.

Without inhibitory neurotransmitters, every new thought would link endlessly with previous knowledge, leading to a never-ending thought process. These inhibitory neurotransmitters help us conclude thoughts with a resolution. The difficulty of unfinished thoughts becomes apparent when we experience major cognitive dissonance—situations that confuse us and remain unresolved. This is the essence of the so-called open loop, where inhibitory neurotransmitters are insufficient to close the thought cycle.

Given that we have an estimated 60,000 thoughts per day, we begin to grasp the immense workload our brain manages daily. Between 90% and 99.9% of this information is processed unconsciously<sup>13</sup>. If a thought remains unresolved for more than 300 milliseconds, it enters our conscious awareness. If we fail to resolve it within one second, it becomes a negative thought<sup>14</sup>.

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<sup>12</sup> Derks, 2022

<sup>13</sup> <https://www.spektrum.de/frage/verarbeitet-das-gehirn-95-prozent-aller-informationen-unbewusst/1616926>

<sup>14</sup> See Derks, 2022, S.363

When a thought cannot be completed, the brain begins searching for solutions in increasingly larger circles within our experiential space. This escalation feels mentally exhausting and draining. Since our brain is oriented in three dimensions, each escalation step expands the spatial-temporal search into the past (seeking analogies) and into the future (forming predictions through generalization and distortion).

If we experience shock or distress from a negative event but lack a complete processing strategy (coping strategy), we risk traumatization—an emotional wound that we are unable to resolve. When trauma remains unprocessed, we keep it close to us, as it represents a potential danger that we wish to avoid experiencing again. In this unconscious strategy, we tear the original event out of its timeline and carry it permanently in our present.

Unresolved problems unconsciously trigger our perception filters, leading to generalizations, deletions, and distortions that permanently alter our perception and submodalities. The same mechanism applies to phobias<sup>15</sup>, where traumatic experiences are mentally overrepresented, including all their associated negative emotions and interpretations.

Figure 7: Example of a Perceptual Distortion

From a neurobiological perspective, patterns we frequently use become reinforced by an increased presence of neurotransmitters, making them easier to access. This means that with each repetition of a trauma or phobia, the stimulus-response loop accelerates—similar to motor or behavioral patterns that we perform daily without conscious thought.

A trauma deeply shapes our worldview because it becomes embedded in our personality structure. At the same time, our inner resources for overcoming it are blocked due to distorted perception.

To resolve trauma, it is helpful to reduce the accumulation of neurotransmitters in the affected neural pathways. This process requires a processing strategy that reintegrates the trauma into its original place on the timeline, allowing it to become a processed memory rather than an active present experience.

The effectiveness of this approach is demonstrated in timeline-based techniques, which have been shown to be highly successful across different therapeutic methods<sup>16</sup>. A crucial aspect of this process is helping the coachee access previously unrecognized

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<sup>15</sup> Unlike general anxiety disorders, phobias have a specific trigger. See: <https://de.wikipedia.org/wiki/Angststörung>

<sup>16</sup> Derks, 2022

resources to process the trauma. This requires a significant pattern interruption, leading to a genuine "aha moment". Since these resources lie outside the problem space, the coachee must tap into a completely different personal experience to find them.

One of the most well-known NLP techniques for this is Collapsing Anchors. This method involves contrasting a problem situation with an unrelated, positive experience—for example, a stressful work situation versus climbing a mountain on vacation. The principle behind this technique is that we, as the same person, behave completely differently in different contexts.

While we are typically relaxed and have full access to our inner resources in a vacation setting, the situation looks entirely different when we face a problem. From a neurological perspective, there is nothing preventing us from applying the same resourceful state we effortlessly access on vacation to a challenging situation. However, our brain tends to search for solutions contextually—meaning it looks for answers within the mental spaces associated with the specific problem, such as work-related stress in this case.

This also explains why we rarely think of or allow ourselves to transfer successful strategies from a vacation or leisure setting to a stressful situation. In this way, we unintentionally limit ourselves, especially under pressure, shrinking our own solution space.

Although the Collapsing Anchors technique may seem counterintuitive in theory, it has proven to be highly effective in coaching. Every one of us has experienced resourceful states, even if they feel small or insignificant.

To utilize these states, we must bring both the stressful and resourceful experiences back to their original level of sensory perception, specifically their submodalities. This means guiding the coachee into a VAKOG (Visual, Auditory, Kinesthetic, Olfactory, Gustatory) trance, allowing them to fully re-experience the active submodalities in each state.

By returning to this raw sensory perception, the resourceful state can be applied to the problem situation—without emotional bias or judgment.

To further engage the brain's reward system, we experiment with how introducing positive submodalities influences the problem state. Essentially, this technique combines two previously unrelated experiences, creating a spontaneous pattern interruption that leads to a re-evaluation of the problem.

As long as there are no conflicting higher-level beliefs, there is no reason for the brain to maintain the old, limiting pattern.

## 6.7 Spatial Work in Therapy

Freudian psychoanalysis already acknowledged spatial phenomena and the importance of spatial setup in therapy<sup>17</sup>. Freud's use of a couch for the patient and a chair for the therapist behind the couch is a well-known standard setup. However, spatial terminology was not linked to internal experiences but dismissed as metaphors.

The significance of space in psychoanalysis today is evident in the preparations for the 2009 Congress of the German Psychoanalytic Society (DPG):

*"Differentiating between outer and inner reality has been handled pragmatically and with spatial metaphors (boundaries, spaces, or areas outside and inside). This differentiation is now seen as too static, as we know how much our perception and reality-checking are influenced by subjective factors of instinctual-narcissistic nature and their defenses. Current psychoanalysis assumes that external and internal realities intertwine through complex exchange or interaction processes, a process never completed. Analytical work involves both separating and connecting inner and outer reality, which is a precondition for the emergence of the 'analytical third'—a new experiential space that allows transformation and structural change. The reality of the analytical situation is co-constructed by the internal structures of both patient and analyst. Thus, the theory of intersubjectivity deems the dichotomy between inside and outside obsolete."<sup>18</sup>*

This quote illustrates how spatial phenomena have increasingly been recognized in psychoanalysis.

Mental Space Psychology (MSP) isn't a new science but an integration of various therapeutic approaches that have acknowledged spatial psychology for years. Here are some well-known approaches:

- **The Other Mind's Eye**<sup>19</sup>
- **The Feldenkrais Method**<sup>20</sup>
- **Voice Dialogue**<sup>21</sup>
- **Spatially Sorted NLP**<sup>22</sup>

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<sup>17</sup> Intelmann, 2003

<sup>18</sup> [http://www.psychoanalyse-magdeburg.de/html/dpg\\_2009.html](http://www.psychoanalyse-magdeburg.de/html/dpg_2009.html)

<sup>19</sup> Sargent, 1999

<sup>20</sup> Feldenkrais, 1972

<sup>21</sup> Stone & Stone 1993

<sup>22</sup> Andreas und Andreas, 1989; Dilts, 1990; Andreas, Faulkner & McDonald, 1994.

- **Timeline**<sup>23</sup>
- **Multiple Brain Integration Techniques (mBIT)**<sup>24</sup>
- **Clean Space**<sup>25</sup>
- **Game of Gifts**<sup>26</sup>
- **Family Constellations**<sup>27</sup>
- **Structural Constellations**<sup>28</sup>
- **BrainSpotting**<sup>29</sup>
- **Time-Lining, The Matrix Model, and Semantic Space**<sup>30</sup>
- **Acceptance and Commitment Therapy**<sup>31</sup>
- **The Wholeness Process**<sup>32</sup>
- **Metaphors of Movement**<sup>33</sup>
- **Spirit Releasement Therapy**<sup>34</sup>
- **The Social Panorama**<sup>35</sup>

Additionally, approaches from shamanistic psychotherapies, Ayurvedic psychotherapies, classical magical rituals, Chakra/Aura/Reiki/Space-Energy therapies, and holistic psychotherapies must be considered<sup>36</sup>.

In the Mental Space Psychology - MSP<sup>37</sup>, all these diverse approaches are integrated into a comprehensive spatial psychology for the first time. Dr. Lucas Derks' significant contribution is in assessing the efficacy of these methods and providing an overarching explanatory model.

The central insight from his research is that spatial orientation is the primary organizing principle of the brain, relevant for every experience, including the perception of time. His work highlights how spatial orientation is essential at all levels of our being, enabling us to navigate and balance our lives.

Space is ubiquitous in our language, body language, spatial perception, and usage, extending to our memories and dreams. Dr. Derks' latest book, "Mental Spatial

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<sup>23</sup> James & Woodsmall, 1988.

<sup>24</sup> Soosalu, G. & Henwood, S. (2013).

<sup>25</sup> Lawley & Tompkins, 2003.

<sup>26</sup> Glaudemans, 2015.

<sup>27</sup> Weber, 1994; Hellinger, 1995; Hellinger & Beaumont, 1998

<sup>28</sup> Varga von Kibéd, 1995; Varga von Kibéd & Sparrer, 1998

<sup>29</sup> Corrigan & Grand, 2013

<sup>30</sup> Hall & Bodenhammer, 2001

<sup>31</sup> Hayes, Luoma, Bond, Masuda & Lillis, 2006

<sup>32</sup> Andreas, 2014, 2019

<sup>33</sup> Austin, 2013

<sup>34</sup> Baldwin, 1995

<sup>35</sup> Derks, 1995

<sup>36</sup> Derks, 2022

<sup>37</sup> Derks, 2022

Diagnosis – MSD-1,"<sup>38</sup> demonstrates how pathological phenomena can be identified and addressed through spatial representation.

His 1996 work on the Social Panorama illustrated how spatial phenomena shape our relationships with others<sup>39</sup>. With MSP, this work has reached a developmental stage that allows all areas of our lives to be connected, facilitating our orientation and balance.

## **6.8 Submodalities Shape the Perceptual Space**

Our perception of the world is profoundly influenced by our perceptual filters, which rapidly compare what we currently experience with our past experiences. This comparison adjusts our perceptual filters across all sensory channels, creating specific configurations for different situations. These fine adjustments in our perception are known as submodalities.

Submodalities are constantly at work. You can easily grasp this concept with a simple exercise:

Take a moment, close your eyes, and imagine your favorite meal. Perceive it in all its details: How exactly does it look? What color is it? What sounds might be associated with it? What are its smells and tastes? How does it feel to enjoy it? Also, consider its perceived proximity—how close is it if you had to indicate the distance with your hands?

Now, with your eyes still closed, push your favorite meal far away from you in your mind and drain out its colors. You'll likely notice an immediate change in how you feel about your favorite dish. Try the opposite—bring it very close, almost as if you could bite into it. How do these different perceptions compare? It's probable that neither extreme—too far away nor too close—is very appealing.

Using submodalities, we map out our experiences in our mental space, both positive and negative ones. These inner representations are part of a cell-like arrangement in our mental space (Mental Space Index, or MSI)<sup>40</sup>, which helps us store specific experiences with their associated evaluations.

For example, coachees with exam anxiety often describe their memories of exam situations as having tunnel vision. Statements like "In my memory, I only see the exam paper and the examiner; everything else is blurred" are common. This experience often includes a blurred perception of the surrounding space, a muted or absent sound, and sometimes a dissociated feeling of viewing themselves from the outside. The associated bodily sensation is often described as "dark" and "constricting," with little awareness of smell or taste, often accompanied by a sense of not being able to think clearly.

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<sup>38</sup> Derks, 2022

<sup>39</sup> Derks, 2018

<sup>40</sup> See chapter 6.9

During coaching, when coachees have the opportunity to alter their submodalities—for instance, by transferring submodalities from a positive state using techniques like Collapsing Anchors<sup>41</sup>—the evaluation of the stressful situation typically changes immediately.

In coaching, working with submodalities serves to precisely elicit individual states of experience and to learn how to intentionally manage these states and their associated emotions.

When submodalities for a specific experience are changed through coaching, the evaluation of that experience also changes. If there is no overriding belief enforcing the original evaluation, the new perception is stored as an updated Mental Space Index.

Moreover, due to associative links in our brain, altering one MSI prompts a reevaluation of all connected MSIs along the timeline. This way, the brain's inherent logic, which aims to keep us in the best possible state, aids us during coaching.

Working with submodalities is a cornerstone in NLP (Neuro-Linguistic Programming) and, when combined with MSP (Mental Space Psychology), is a highly effective tool for understanding states without necessarily knowing the detailed underlying motivations. Submodality work is also beneficial across various formats, helping coachees learn how to alter unwanted states and activate existing resources.

#### Figure 8: Common Phenomena for a Positive State

By using sense-specific language questions about the sensory representations—we can analyze the internal construction of this world model ("map") in detail.

Submodalities encompass numerous manifestations across all sensory channels and are linked to specific memories.

Important: It's fundamental to note that the submodalities of perception form the foundation for our subsequent evaluations. This means perception comes first, followed by emotional evaluation, and then cognitive justification.

Conversely, changing the submodalities means the original evaluation can no longer be sustained. Thus, through effective submodality work, a coachee can interrupt patterns and reevaluate situations or experiences.

Below is an overview of the various submodalities that a coachee might perceive in relation to a specific experience.

#### Table 1: Visual Submodalities

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<sup>41</sup> See chapter 0



| Feature                 | Description                                                                                                                                                                                                                           |
|-------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Associated/Dissociated  | Are you "inside" the image, or do you see yourself from the outside?                                                                                                                                                                  |
| Moving / Still          | Is the image more like a photo or a film? Is the movement fast, slow, or in slow motion? Does the entire image move, or is there motion within the image? Is it more realistic or exaggerated? What direction does the movement take? |
| Color / Black-and-White | Is the image colored or black and white? Are the colors natural and realistic, pale/pastel, or vivid and bright?                                                                                                                      |
| Brightness              | Is the image brighter or darker than "normal"?                                                                                                                                                                                        |
| Contrast                | Are the contrasts sharp and distinct, or soft with many gradations?                                                                                                                                                                   |
| Sharpness               | Is the image crisp and clear, or blurry and out of focus?                                                                                                                                                                             |
| Surface of the Image    | Is the image's surface matte or glossy?                                                                                                                                                                                               |
| Complexity              | Is the image fully detailed, or do you need to focus on individual details?                                                                                                                                                           |
| Size                    | How large is the image exactly? (Show size)                                                                                                                                                                                           |
| Location/Direction      | Where exactly is the image positioned? Show the location in space!                                                                                                                                                                    |
| Distance                | How far or close is the image?                                                                                                                                                                                                        |
| Dimensions              | Is the image two-dimensional or three-dimensional?                                                                                                                                                                                    |
| Shape                   | Is the image square, round, flat, curved, or panoramic?                                                                                                                                                                               |

|                   |                                                                                        |
|-------------------|----------------------------------------------------------------------------------------|
| Boundary/Frame    | Is the image framed? If so, what color and thickness is the frame?                     |
| Orientation/Angle | Is the image upright or at an angle? If tilted, at what degree?                        |
| Perspective       | From which perspective do you see the image? (Above, from the side, front-facing...)   |
| Proportion        | Are the people/objects in the image proportional and realistic, or are they distorted? |
| Number            | Is there one image or multiple images?                                                 |

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Table 2: Auditory Submodalities

| Feature     | Description                                                                                                                   |
|-------------|-------------------------------------------------------------------------------------------------------------------------------|
| Position    | From where and in which direction do the sounds, voices, or noises originate? From inside or outside? From which exact point? |
| Mono/Stereo | Is the sound spatial (stereo) or flat/mono?                                                                                   |
| Volume      | Is it loud or quiet?                                                                                                          |
| Pitch       | Is the pitch high or low?                                                                                                     |
| Tonality    | Is the sound shrill, nasal, full-toned, resonant, rough, or thin?                                                             |
| Modulation  | Are certain tones emphasized?                                                                                                 |
| Speed       | Is the sequence of sounds fast or slow? Does it sound distorted or normal in speed?                                           |
| Rhythm      | Is the sound rhythmic or monotonous?                                                                                          |

|          |                                         |
|----------|-----------------------------------------|
| Duration | Is the sound continuous or interrupted? |
| Melody   | Is the sound melodic or monotonous?     |

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Table 3: Kinesthetic Submodalities

| Feature           | Description                                                                                                                                                                                   |
|-------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Quality           | How can the (body) sensation be described? For example: hot, warm, cool, cold, rough, smooth, hard, soft, tingling, tense, relaxed, cramped. Is it more diffuse ("fuzzy") or solid, distinct? |
| Intensity         | How intense is the sensation?                                                                                                                                                                 |
| Location/Position | Where exactly in your body is the sensation localized?                                                                                                                                        |
| Movement          | Does the sensation move, or does it remain fixed in place?                                                                                                                                    |
| Direction         | Where does the sensation start? Does it have a direction? How does the movement progress?                                                                                                     |
| Duration          | Is the sensation continuous or intermittent? How long or short are the intervals?                                                                                                             |
| Speed             | Is the sensation slow or fast in its progression?                                                                                                                                             |

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Table 4: Olfactory Submodalities

| Feature   | Description                                                                       |
|-----------|-----------------------------------------------------------------------------------|
| Quality   | How exactly would you describe the smell? Fresh, floral, rotten, smelling like... |
| Intensity | How strong is the smell? Is it sharp, weak, or overpowering?                      |

Direction                      Where does the smell originate from? What is its source?

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**Table 5: Gustatory Submodalities**

| Feature           | Description                                                                                 |
|-------------------|---------------------------------------------------------------------------------------------|
| Quality           | How exactly does the taste appear? Sweet, sour, bitter, salty, spicy, mild, stale, fresh... |
| Location/Position | Where exactly is the taste perceived? On the tongue, lips, throat...?                       |
| Intensity         | Describe the intensity of the taste—mild or strong?                                         |

## 6.9 Time is Space

To develop a sense of time, we need space. Only through movement from one place to another do we develop a sense of time in our lives. This movement through space also helps us develop a sense of speed and rhythm, precisely because we move from one place to another.

Our entire life takes place in a space-time dimension. From the first day, like beads on a string, we create experiences in the here and now, which become part of the past just seconds later, making room for a new here and now.

Where memories are relevant, so-called Mental Space Indices (MSIs)<sup>42</sup> are formed on our internal timeline. These MSIs can best be compared to a location on a digital map where photos are stored. However, unlike this digital map, MSIs encompass all sensory impressions such as seeing, hearing, feeling, smelling, and tasting. Additionally, as we saw in the previous chapter, the fine adjustments of our perception, the submodalities of perception<sup>43</sup>, are crucial in our perceptual space for the evaluations and beliefs associated with these memories.

Figure 9: MSIs on the Timeline

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<sup>42</sup> Derks, 2020

<sup>43</sup> See chapter 6.8

MSIs are cellular arrangements that serve as starting points for subsequent experiences.

When we encounter a new experience, we do not start from scratch but build our subsequent experiences on the previous ones up to the first MSI. These form a chain of events, particularly linked by the activation of the same perception submodalities. The same type of sensory-specific perception that we already know is activated. This happens because our brain always seeks comparable experiences to assess the situation. If the experience is similar, whether consciously or unconsciously, the same cellular arrangements for setting the submodalities are activated. The body responds with the pattern it has stored for this comparable situation.

Neurobiologically, this is highly efficient, allowing us to recall recognized patterns without much thought, even unconsciously. Thus, we do not need to remind ourselves every morning how to tie our shoelaces, make coffee, and much more. Additionally, our brain uses the recall of frequently used behavior patterns to avoid danger. This occurs through the increased accumulation of activating and inhibiting neurotransmitters, the messengers in our brain. By this targeted accumulation of messengers, patterns are accelerated, becoming increasingly effective and efficient. The better this pathway is developed, the less the triggering stimulus needs to be at any point in this pattern to fully activate it.

#### Figure 10 - Partial Activation of Neural Patterns

This mechanism is also active when we experience traumatic events. If these cannot be processed, the experience is like an open wound. The affected person remains highly vigilant on a conscious and/or unconscious level to prevent this painful experience from happening again. This mechanism causes attention and perceptual filters to automatically focus on events containing the painful experience. This leads to the well-known effect of self-fulfilling prophecy.

All our experiences build on one another. For the brain, it does not matter whether the experience is positive or negative. Experiences are chains of events that we have justified based on our experience. It does not matter how absurd a conclusion may seem to an outsider; from the respective chain of events, the conclusion appears entirely logical to the person concerned. Another consequence of this construction is that older experiences are initially more significant than new experiences. This is simply because we have confirmed and further justified them through our perceptual filters more frequently.

Regarding our space-time experience, we distinguish three basic forms of time:

**In-Time:** Here, the person is in the midst of the experience with the past behind and the future ahead. We refer to this form of time experience as fully associated.

Figure 11: Perception of Time – In-Time

**Through-Time:** In this time experience, the person stands outside the timeline and views it from the outside. We refer to this form of time experience as fully dissociated.

Figure 12: Perception of Time – Through-Time

**Between-Time:** This time experience is a mixed form, characterized by the person experiencing association but perceiving the future and past sideways in front of them.

Figure 13: Perception of Time – Between-Time

Each of these forms of time experience is useful. At the same time, the first two forms, in extreme expression, are very stressful and potentially even pathological. The ability to be fully in-time is very suitable for intense experiences, whether in relationships with other people or to experience pleasure.

It becomes unpleasant when we find no way out under high stress and experience ourselves as fully stuck in the situation. We refer to this form of perceived hopelessness as a stuck state.

To resolve such stuck states, ideally, movement in space is needed. Through a change in perspective, we can re-examine issues. For many people, it is a common experience that staring harder at the computer monitor does not help when a solution cannot be found. It is then important to first stand up, do something else, to break the fixation on the problem.

For a planning mode, it can be very helpful to consider things dissociated, to get a clear overview of the sequence of certain project elements, for example. Conversely, someone who realizes that life is passing them by without feeling involved needs more "In-Time."

Between-Time marks the ideal model, characterized by the fact that true life and experience can only take place in the present, and we can only actively shape it in this present time.

## 6.10 Working on the Timeline

Even with events far back on our timeline, this basic principle of targeted pattern interruption is effective. If through timeline work the imprint, the origin of the trauma, can be identified, the goal is to enable a pattern interruption at this origin. This is achieved, among other things, by activating resources from other areas of life and other experiences that were not available at the imprint.

To fully process an imprint, it is necessary to allow this enrichment of resources before the time when the imprint occurred. If the coachee can temporally move a bit ahead of the time on their timeline when the imprint occurred, resources can be established there necessary for processing. At that point, the pattern of the imprint had not yet accumulated inhibiting neurotransmitters trying to maintain the pattern.

Figure 14: Effects of Imprints on the Timeline

If the coachee can establish the necessary resources at the point of the previous imprint, so that such an imprint does not occur in the first place, the brain will no longer experience the original imprint as traumatic. Thus, while the original event remains, the enriched resources can now transform it into a memory that does not need to be repeatedly reactivated in the present.

Typical formats from NLP include:

- Reimprinting (Ch. 10.2.6)

- Change History

- Core Transformation

- Developing a target Personality/Identity (Ch. 9.2.4)

- Wholeness Process (Ch. 9.3.2)

All these methods share the fact that it is no longer necessary to accumulate neurotransmitters at this point on the timeline. It is comparable to relaxing a clenched fist, releasing this tension because what created the tension has been clarified and processed.

It is important in all these methods to move temporally before the origin of the pattern, where no inhibiting neurotransmitters were active yet. If we succeed in moving mentally before the origin on the timeline, the brain does not care where the information comes from. This is because the brain fundamentally cannot distinguish between actual experiences and imagination. What matters is that it is a resourceful strategy that helps process the potentially traumatic situation well. Therefore, it is never about suppressing

something from the experience space, but rather about establishing processing strategies congruent with the coachee's personality.

Currently, some trauma therapies work with so-called confrontation therapies. These aim to repeatedly confront the coachee with the traumatic event until the neurotransmitters are so depleted that the body releases the stress state, leading to a relief state. Through the depletion of neurotransmitters, there is initially relief for the person concerned, but later this is overcompensated by an increased accumulation of activating and inhibiting neurotransmitters at the corresponding synapse. This creates the risk that the old pattern is not only maintained but even reinforced<sup>44</sup>. This can potentially deepen the trauma. It is comparable to fitness training, where the muscle is initially exhausted, leading to muscle soreness. Subsequently, with increasing training, the muscle not only compensates for this stress but also overcompensates through muscle growth.

### **6.11. Language and Space**

We are three-dimensional beings and live in a three-dimensional space. This means that our internal experience is also three-dimensional. Anything else would be an inconsistent mental spatial concept. Assuming that our external world and our internal experience are intertwined through perception filters, it makes sense that we need space to express and name our experiences. The crux is that our internal experience runs in 3-D, while language is one-dimensional. This means we can only express part of our experience through words. Therefore, we often use metaphorical language to describe our internal experience.

"This lies ahead of me or behind me," "One must take a clear stance," "We must pick the low-hanging fruit," "I cannot imagine that," "That is unclear to me," "There is something between us," "I look up to her," "We have grown apart," or "They support me," "That is a mountain of work," "That is a trifle," etc.

Also, terms like before, above, below, behind, in front, right, left, with, without, behind it, above it, below it, small, large, bearable, stable, handy, unhandy, etc. are terms with which we express our internal three-dimensional representation.

We need these spatial terms to express what takes place in our mental space. Without these terms, our language becomes abstract. Yet, even abstract terms, to be understood, must be translated back into sensory-specific impressions in the mental space. When we say, "I am stressed," the term stress itself is abstract. Only through the associated experience does it become clear what is meant. If language and body language diverge here, the statement becomes unbelievable. If we say we are stressed

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<sup>44</sup> Derks, 2022



and our facial expressions and gestures are relaxed, this does not align in the communication partner's understanding because they compare this statement with their own mental spatial experience.

If we don't have a concrete inner experience, it becomes difficult for us to express what we are experiencing and meaning. For example, if a coachee doesn't have a clear self-image, she cannot describe it. In extreme cases, this is associated with a dissociative disorder because the coachee has lost contact with her self-image. It already becomes difficult to express ourselves when we are supposed to describe something we have no concrete idea about. We then search for analogies or general metaphors (buzzwords) to express ourselves. Without a concrete mental spatial experience, any form of expression remains vague.

If our internal mental spatial experience is blocked by phenomena like stress, we lose access to our inner sensory-specific representations. The term blackout precisely describes this mental spatial phenomenon. Everything outside the immediate focus is black or simply gone.

At the same time, without these mental spatial concepts, we have significant problems making ourselves understood by others. They understand us only if they can map our words in their inner experiential space. For example, if we talk to someone about a topic we are not familiar with and they use technical terms we do not know, misunderstanding arises immediately. We then cannot find a corresponding 3-D representation for the terms used in our mental space. Without sensory-specific concrete representation in our three-dimensional mental space, there is no understanding.

Psychologist George Lakoff, who has extensively researched the use of metaphorical language in mental space, wrote in his book "Women, Fire, and Dangerous Things": *"Words, outside and within the framework of psychotherapy, are 1-D series of sounds or symbols. They derive their meaning from the 3-D experience to which they are connected through classical conditioning. Theoretically, when people speak, they convert the 3-D experience into 1-D sentences, and when they listen, they do the opposite."*<sup>45</sup>

This means that the intertwining of the internal 3-D experience with our perception of the external world is a permanent comparison. What we understand and perceive from the external world closely depends on whether we find a matching representation in the internal 3-D representation. If there is no internal reference, because something is too abstract or simply unknown, we first look for analogies to help us make this connection. If this doesn't succeed, we either filter out the perception or perceive it as irritation. Irritation is always associated with the acceptance that we are facing something unknown for which we do not have a fitting answer in our inner 3-D experiential space.

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<sup>45</sup> Lakoff, 1987

Current research shows that we use the right hemisphere for this 3-D representation and the left hemisphere for language and the pictorial 2-D translation.

*"The 3-D simulation of reality can be understood as a right-hemispheric representation, where the 1-D and 2-D structure of language is mapped in a way that best matches the 1-D and 2-D map of the left hemisphere."*<sup>46</sup>

## **6.12 Body Language and Space**

In addition to language, body language is a central component of our communication. When we speak with others, we use gestures and facial expressions to express what we mean. We mark with our body language the places in space where we perceive things. We look at the places where we mentally perceive things or point to them with our hands. When we stand, we take steps toward or away from things. We constantly use our mental space to organize and express ourselves.

With gestures, we point to the places where we have located the topic in our mental space. This also applies to our handling of time: we show where things are temporally for us by pointing to our immediate space or gesturing to a distant point. This applies to the past, present, and future. Our limitations also become visible in the mental space very quickly. For example, if you ask coachees how much space they feel they have concerning the coaching topic, they can consistently spontaneously name how much space is available to them. This is usually smaller than what we call the intimate space. It typically encompasses our arm's length sideways and upwards. If we experience this space as restricted, we feel under pressure, limited, constrained, incapable of action, etc.

### **Figure 15: Intimate Space of Perception**

For example, if we experience a workload as a mountain of work, this is expressed not only linguistically but also through body language. We indicate a mountain with our hands by making an upward movement with one or both hands.

Another example is a blockage. When we experience situations where we block ourselves, a common gesture is that both hands indicate a very limited space at a short distance in front of our eyes with expressions like, "I feel like there's a blank screen" or "like a board in front of my head." In other cases, coachees experience their problem as an insurmountable wall in front of them.

### **Figure 16: Oversized Representation of a Topic**

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<sup>46</sup> Derks, 2022

If we are unclear in our experiences and thoughts, we often find gestures where the hands move back and forth to indicate a cloud of confusion or a jumble of things.

A common phenomenon in coaching is that coachees present phenomena relatively factually, abstractly, and rationally on the verbal level, while the body language expresses the internal drama of the topic.

For example, one of our coachees said: "The workload is actually fine, but somehow I have no energy left." During this statement, her hands repeatedly gestured forward as if there was a wall. As a coach, you can repeat the body language gesture and ask what this gesture means in her internal experience.

#### Figure 17: Wall in Perception

Our coachee burst into tears and said, "I really feel like I'm standing in front of a wall, and I have no idea how to solve this." When asked what this wall consists of, she began gesticulating wildly with her hands and said:

"My life is pure chaos right now, and I can't really satisfy anyone."

#### Figure 18: Brain Fog

As a coach, you can apply the following intervention on both the body language and verbal levels: Make a limiting gesture by holding both hands vertically close together and ask, "What do you think about looking at and sorting out the parts of this chaos one by one?" For our coachee, this prompted a spontaneous calm and a congruent nodding.

This small example already shows how immediately spatial gestures are understood. The great advantage of working this way is that you relieve your coachees from verbalizing their internal experience. They can remain much more in their 3-D experiential space.

Any verbalization is an abstraction and often expresses only a limited portion of what we experience. Our inner experiential world is much more complex and associative. Both coach and coachee often find that attempting to put very emotional experiences into words feels too imprecise or too abstract. In conjunction with body language, we manage a bit better, but even then, it remains only a relatively abstract representation for another person.

For coaching work, this means: All our linguistic understanding of the symptoms of our coachees is initially just our mapping in our mental space and does not correspond to the coachee's reality. Conversely, every linguistic intervention is an expression of our three-dimensional experience, which we offer to our coachee to translate back into their three-dimensional space.

#### Figure 19: 3D Experience and 1D Language

Additionally, we use terms differently and understand them differently depending on previous experience and current state. Even the slightest form of mistrust or insecure contact could lead to interpretations that the sender of the message did not intend. The more fragile a person's state and the worse the contact, the greater the room for misunderstandings and irritation in the process.

Due to this double translation in both directions and the complexity of the associated body language, a large abstraction space remains.

Mental Space Coaching (MSC) offers a much more direct access to the coachee's experiential space by working directly with the mental space. Psychologist Barbara Tversky writes in her book *The Cognitive Design of Tools of Thought*. Review of Philosophy and Psychology:

"Like diagrams and sketches, gestures serve self-thought and alter the thoughts of others. When solving mental rotation problems, people often rotate their hands; rotating their hands in the correct direction helps with mental rotation<sup>47</sup>. When people read descriptions of complex environments to prepare for a later memory test, many gestures help to do this. Their gestures represent the environments, point-like gestures for places, and line-like gestures for paths between places. Participants rarely look at their hands, and the look is a quick glance. Thus, the representations that support memory and inference are more spatial/motor than visual. A comprehensive theory of how the externalization of thinking supports thinking must consider the moderation by spatial/motor representations as well as by spatial/visual representations.<sup>48</sup>"

### **6.13 Fears in Mental Space – A Brief Overview**

Every person has fears and worries. The feeling of fear is a normal and necessary reaction to dangers. Without it, we would not survive long. However, excessive fears can lead to pronounced anxiety disorders. If these are extremely pronounced, they can overload the neural system to such an extent that the coachee loses access to their own resources. In such cases, in addition to therapeutic support, medication is often required to bring the neural system back to a functional state.

The following explanations show that fears are not only very diverse phenomena but also exhibit a wide range of perceptual distortions. The boundary where coaching is still possible and where therapy is required is not easy to find. As a rough guide, it can be said that people who are in a pronounced stuck state and no longer have access to their own resources are not coachees but need therapeutic treatment.

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<sup>47</sup> e.g. Chu und Kita 2011

<sup>48</sup> Tversky, 2015, p112

Fear and worry cause us to focus on what we are afraid of, enlarging it visually through our submodalities, bringing it closer, distorting it acoustically, portraying it with a threatening feeling, or perceiving it with a distorted model of smell and taste.

### 6.13.1 Specific Fears

With specific fears, the object of fear can be clearly identified. This includes, for example, specific phobias or concrete fears such as an event, like an exam or a performance.

Obsessive behaviors such as constant hand washing or frequently checking whether the stove is turned off or the door is locked also belong to these specific fears.

Social anxiety disorders (social phobias) are also widespread. They are highly associated with the fear of judgment by others and often with shyness or avoidance of contact. The evaluation of others' remarks is viewed through the lens of one's own worries and fears, leading to overt or subtle internal conflicts.

Figure 20: Ineffective Coping Strategies and Space

In addition, there are so-called panic attacks. These uncontrolled states of fear can occur unexpectedly, for example, while watching TV in peace. They can also be triggered in certain situations.

In about two-thirds of cases, panic disorder is associated with agoraphobia (also known as fear of open spaces); the affected person has fear in specific situations or places and avoids them. Often these are places where the person feels helpless and unable to get help quickly enough if needed. Often associated with a sense of shame for needing help. In severe cases, these individuals can barely leave their safe familiar environment alone and are confined to their homes.

Typical situations/places for such panic attacks include:

- Crowds
- Public places
- Traveling far from home
- Traveling alone
- Standing in line

- Riding an elevator, bus, or car
- Traveling by plane

### 6.13.2 Unspecific Fears

In addition to these specific fears, there are also unspecific fears. These often occur in bouts and give the affected person the feeling of being surrounded by anxiety-inducing events. The mental spatial phenomena related to these are diverse and would go beyond the scope of these documents.

#### Figure 21: Disruptions in Mental Space and Mental Time

However, all unspecific fears share the characteristic that coachees experience themselves as surrounded by these fears, which often lurk like dark clouds in their perceptual space. Associated with unspecific fears is usually the fear of fear itself. The constant preoccupation with fear and the worry of when it will appear next leads to severe exhaustion symptoms in those affected. Such behavior is also partly coupled with depressive moods and a sense of hopelessness.

### 6.13.3 Post-Traumatic Stress Disorder

Post-traumatic stress disorders (PTSD) are among the fears that can occur after a massive traumatic experience. Examples include war experiences, death, injury, torture, abuse, etc.

#### Figure 22: A Trauma Has No Time

Traumas are characterized by the fact that they cannot be processed like other experienced events but remain subtly active in the present. Often, small triggers in daily life are enough to uncontrollably activate the trauma and all associated behavior patterns, without the affected person having cognitive access to this behavior pattern. At the same time, these individuals often have difficulty recalling the actual triggering event (or parts of it) in terms of a cognitive causal chain. This is a sign that the event lies at a point in the timeline that is not consciously accessible. This can be compared to a tear in the timeline, with accessible events before and after but not where the actual event belongs.

A significant task of therapeutic support is to bring the event back to its rightful place in the timeline and equip the coachee with the resources that help them no longer experience it as traumatic. This allows the event to transform from a trauma to a memory.

## 7. Trance Work in the Mental Space

Even though it is a daily practice for many coaches, this section will illustrate the importance of VAKOG trance and submodality work through trance work.

Since our inner experience is shaped by the three-dimensional perception of our five sensory channels, it is crucial to utilize this three-dimensional experience in coaching when working with trance states. In our experience, placing a topic "into space" allows coachees to recognize connections and solutions much faster and more effectively than simply talking about them. This method meets the coachee exactly where they are in relation to the topic.

It is important to remember that cognitive thoughts can only occur sequentially. While this enables logical reasoning, it can also overwhelm us when trying to solve complex, multi-layered issues.

In trance work, our inner three-dimensional experiential space becomes a powerful ally, helping coachees access their own mental space. Through targeted submodality queries, we can significantly enhance this process and make it more tangible.

Notably, a deep hypnotic trance is not required to provide meaningful support. In most cases, a simple VAKOG trance, or wakeful trance, is entirely sufficient.

Here, as a coach, you ask your coachee to recall a specific, personally experienced situation related to their topic—as if they were reliving it in the present moment.

Then, in a calm, deliberate tone and at a volume that feels comfortable for the coachee, you guide them through the following statements, allowing for pauses so they have enough time to fully recreate the inner representation:

- Notice everything there is to notice... (as if it's happening now).
- ... and see everything there is to see...
- ... and hear everything there is to hear...
- ... and feel everything there is to feel...
- ... and perhaps, there might even be something to smell or taste.
- ... and without a doubt, there is also this special feeling.

A crucial note at this point: The VAKOG trance is used to retrieve submodalities. The coachee selects one specific situation related to their topic. This provides both the coachee and the coach with concrete reference points regarding how each sensory modality is represented in the experience. These insights can later be used in the

coaching process, for example, by transferring submodalities from a positive experience to a previously negative one.

Caution STUCK-STATE! When exploring a problematic topic with a coachee, the VAKOG query should be as brief as possible—only as long as necessary—to prevent the coachee from entering a stuck-state. In other words, there is a risk of retraumatization if the query is too extended or detailed, especially concerning kinesthetic representation.

For this reason, we recommend that coachees describe their inner representations without reliving the event in detail. This approach reduces the emotional burden on both the coachee and the coach.

Conversely, when retrieving submodalities from a positively experienced situation, the coachee should be given ample time to immerse themselves in the experience. The positive feeling associated with the situation can be explored in greater detail using questions such as:

"Where exactly do you feel it in your body?"

"Does it have a specific temperature?"

"Does it have a certain color?"

"And now, I invite you to expand this feeling throughout your entire body... from your head to the tips of your toes."

## **8. Beliefs: A Mental-Spatial Construction**

### **8.1. Introduction**

Beliefs are the conscious and unconscious rules that we have developed to orient ourselves in the world. Many of the beliefs we use in our adult lives are based on concepts that we started developing in childhood. They guide our conscious and unconscious behavioral patterns and greatly shape our self-image, our identity, and even our intelligence and health. They also influence what we consider meaningful and where we feel a sense of belonging.

Our beliefs function like a self-fulfilling prophecy, both in a positive and negative sense. If I personally allow myself to live a healthy life, my everyday behavior is likely to be



oriented toward exercise, healthy nutrition, and recovery periods. However, if I tell myself, "I don't have time for my health; I always have to make sure the people around me are doing well," my actions will reflect that belief instead.

All beliefs are ultimately conclusions drawn from some conscious or unconscious experience. It may be that we developed beliefs that were perhaps even crucial for survival at an earlier point in time but are no longer helpful today.

### Figure 23: Positioning of Beliefs in Mental Space

For this reason, beliefs are not rationally comprehensible; they follow an individual's internal logic and chain of reasoning. Robert Dilts defines: "A belief is a generalization about a relationship between experiences."<sup>49</sup> And these two experiences do not necessarily have to be originally connected. That is, for reasons that are often not accessible logically, our brain links these experiences because, in the context of the events at that time, it was "meaningful" for our brain—for example, to protect us from similar experiences in the future.

In our brains, cellular arrangements form for our beliefs and the events that caused them, spatially represented as "Mental Space Indices" (MSIs). This spatial placement in mental space gives beliefs their relevance and priority for our lives and experiences. Thus, internal positioning is just a concept we have trained over the years. Not all beliefs are always active at the same time; rather, they are activated by internal and external trigger events and moved to the corresponding place in mental space.

An example: If person A meets person B, who reminds them of person C with whom they had conflicts in the past, all perception filters and beliefs associated with this are activated in A. A was in love with C, but C made it very clear that A was not lovable.

Now, it is crucial what internal strategy person A has learned for dealing with such challenges. If there is an intense stimulus-response chain, person B might unknowingly activate the patterns related to person C in A. This means A reacts to B just as they previously reacted to C. Person B then witnesses behavior that may be difficult or impossible for them to understand. Person A may enter a "stuck state," even though B has objectively given no reason for it.

To stay with the example: This may mean that, due to my belief, I may feel like I am doing everything right but encounter resistance in the external world with my behavior.

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<sup>49</sup> Dilts, 2010 pg.35

Since our brain responds to this state with unpleasant feelings, we try to explain this experience to ourselves through our behavior and other experiences. To solve problems, we bring them to the center of our attention so that we can access them cognitively. We focus the topic in front of us in mental space. If we do not want to face the issue, we try to repress it (repression) and keep it out of the center of attention.

#### Figure 24: Dissociation from Traumatic Experiences

When we repress dissonances at the belief level, it also results in incongruent behavior at the behavioral level, which does not align with our beliefs and cognitive insights. If these incongruent patterns (see also Satir types<sup>50</sup>) are maintained for a long time, health consequences are often the result.

#### Figure 25: Repressed Issues

Such activations of MSIs are a key reason why certain belief concepts repeatedly take hold unintentionally, even though we know cognitively in relaxed moments that these patterns are no longer helpful.

To change these patterns, it is necessary to reposition and re-evaluate the MSIs so that other patterns can establish themselves here.

## **8.2 Coaching Formats**

### **8.2.1 Belief Clusters in Space**

If we start from the mental spatial model described above, each belief has a position in three-dimensional space. One might think that it is enough to work with this single limiting belief. However, each belief is embedded within other justifying beliefs to ensure our inner congruence. Together, these form what is called a belief cluster—a network of beliefs that mutually reinforce each other. This can be broken down through the following questioning technique.

An example of a limiting belief cluster: "I always have to perform – because? – otherwise my company won't run – because? – without me, everything would descend into chaos – because? – I've always had to make sure that nothing happens – because? – otherwise no one would be there for me anymore – because? – if I don't watch out, I'll quickly be all alone – because? – I'm actually not wanted – because? – I

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<sup>50</sup> Satir, 2004

don't have the right to be in this world; I have to earn it – because? – my mother didn't want me."

The "because" question that follows each belief of the coachee aims to uncover the underlying beliefs and show their connections until a core belief, which underlies all others, is revealed.

In addition to the "because" connections, there are also "if-then" connections.

The clusters are not linear constructs but usually form a network. This can be visually manifested in space with floor markers. Through spatial manifestation, coachees can very quickly grasp the entire network of beliefs they have often woven themselves into over years.

#### Figure 26: Belief Cluster in Mental Space

When working with the belief cluster, first identify the coachee's limiting beliefs, write each on a floor marker, and ask them to find a place in the room for each belief. Then invite the coachee to make the network of limiting beliefs visible with small arrows, also using floor markers as tools.

Next, ask the coachee to identify the belief with the highest emotional impact. This is usually the core belief that energizes the entire cluster.

Now, it's about determining how true this core belief actually is. The working hypothesis here is: there is no belief that isn't equally true in its opposite.

This belief is true to the extent that there is always a context in which it appears "true." Finding the specific context in which the core belief was originally formed is now the challenge. Even if we are convinced that the core belief is true, we should now investigate where in their life the coachee has already experienced the opposite.

Different techniques can be applied here. The simplest is to offer the coachee the option to shift this belief around until it reaches a position in the space that feels appropriate to them. Here, it is helpful to offer the coachee different spatial positions, with their own position in the room as the reference point. After each shift, ask: "Does this belief feel better, worse, the same, or just different in this new position?" This re-evaluation usually creates a significant relief effect. The result is that the coachee can completely re-evaluate the belief. Coachee: "Yes, when the belief is far away from me, it feels much better. The truth is, I don't have to earn my place in the world."

Then, ask the coachee how the belief sounds in this new, positive re-evaluation.

All negative formulations using *"no," "not anymore," "never again," "none,"* or comparative statements (e.g., *"then I'll be better than X"*) trigger negative associations, submodalities, and emotions in our brain. The brain itself does not process negations; instead, it responds to the mental image and the negative emotions that the coachee associates with them.

This means that reframing requires a positive formulation—one that the coachee can fully affirm. When this happens, the brain automatically associates positive images and emotions, which are crucial for achieving any goal.

For further reference, see the "Well-Formed Outcomes" format, which is an integral part of every NLP Practitioner training (DVNLP).

It may take some time for the rephrasing to settle. The new sentence is usually short and concise in its final form. To continue with the example above, the coachee from our practice example rephrases: "I take my place in the world." You can write this new belief on the back of the original core belief. Then offer the coachee the opportunity to anchor this new belief across all sensory channels using a VAKOG trance.

If the coachee is willing to leave the old belief in its place, the new belief is used to gradually reposition and reformulate the other beliefs until a stable network of supportive beliefs has been established. Again, all beliefs are concretely anchored through sensory experience, ensuring that the newly formed belief cluster is fully integrated.

After this process, we recommend evaluating the practical consequences of this transformation from a dissociated perspective. The goal is to test whether the new belief system holds up in everyday situations. The coachee selects a scenario where the original trigger for the old belief system was previously activated. This means the coachee examines whether the new belief cluster remains stable in real-life conditions. Invite your coachee to imagine a future situation where they will once again face their challenge—in our example: *"I always have to perform."* This testing phase continues until the coachee is fully confident that they are ready to live by the new belief system and its implications.

Finally, a Future-Pace is conducted with the new belief cluster to ensure its long-term viability and explore what new possibilities arise for the coachee as a result.

### Summary of the format

1. Work with your coachee to identify limiting beliefs (e.g., by asking: *Why...? If-then...? What happens if...?*).

2. Lay out these beliefs as physical anchors on the floor to make the spatial structure of their relationships visible.
3. Identify the core belief that has the strongest influence on the belief system.
4. Ask your coachee to physically shift this core belief until a new, helpful statement emerges. Support this process by repeatedly questioning the validity of the old belief. Write the new belief on the back of the floor anchor, making it clearly visible to the coachee.
5. Once the core belief has found its new place, systematically review, reposition, and redefine all other beliefs. Write the new formulations on the back of each corresponding floor anchor and flip them over.
6. When the entire belief cluster has been repositioned, ask your coachee to fully experience it from the central perspective where the beliefs were laid out—immersing in all sensory channels.
7. Conduct an ecology check and Future-Pace. This step often takes longer than the restructuring itself, as belief changes can have deep effects on self-perception.

An alternative approach to working with core beliefs is demonstrated in the following example:

This method involves identifying the belief with the strongest emotional charge and challenging it. Suppose the core belief previously identified in coaching is: *"I am worthless unless I meet others' expectations."*

When asked how they perceive themselves in such situations, the coachee responds: *"I see myself as much smaller, not as an adult man, but more like a little boy, always running after others to please them."*

Coach: *"Just for a moment, imagine that this belief never existed in your life. What would that be like...?"*

At this point, coachees often react with resistance: *"That's a nice illusion, but the belief is there."* Now it's the coach's role to stay with the process and reflect back that the solution already exists within the coachee.

Coach: *"Have you ever experienced situations in your life where this belief didn't apply or where the opposite was true?"*

Coachee: *"Well, now that you mention it... On vacation, for example, this belief isn't active at all. I'm in a relaxed state and can structure my day however I want. In those moments, I feel powerful and full of energy."*

Coach: *"Interesting. So when you are relaxed, like on vacation, and you shape your own rhythm, this belief doesn't exist?"*

Coachee: *"No, because then I focus only on my own needs."*

Coach: *"Okay, so what would your core belief be if you applied this realization?"*

Coachee: *"When I prioritize my needs, I feel valuable."*

The coachee then experiences this new belief through VAKOG, making sure that the associated feeling is fully anchored.

Finally, apply this new positive belief to all other beliefs in the cluster and guide the coachee in reformulating them accordingly.

As in the first method, the process concludes with an ecology check and Future-Pace to ensure that the new belief system is stable and sustainable.

### **8.2.2 Perspective Shift in the Mental Space**

Almost everyone has experienced the frustration of being stuck on a concept, unable to move forward. Often, the instinctive response is to do more of the same—work harder, push longer. But as Einstein famously said: *"Problems cannot be solved with the same thinking that created them."* This simple statement perfectly illustrates what happens in mental space.

A particular way of thinking corresponds to a specific perspective on a topic. This perspective is linked to a specific neural arrangement, which determines how we see, hear, feel, and sometimes even smell or taste the issue. At the same time, this perspective is tied to specific beliefs, concepts, strategies, and behavioral patterns. As a result, when searching for solutions, we tend to stay within this self-imposed mental framework.

If no solution is found, it means that our current neural arrangement does not provide enough cognitive space to resolve the problem. Under stress, we further narrow this space, making solutions seem even more distant. The more we repeat the same approach, the tighter our perception becomes, and the solution feels increasingly out of reach.

Key takeaway: "Solutions are everywhere." If you're not making progress in solving a problem, it's because your problem space is too small. This means you need a perspective shift, which is the gateway to a larger solution space.

To break free from this mental blockage, we need a perspective shift that offers an entirely new way of seeing the issue.

One simple and effective method for this is Dr. Lucas Derks' "Car-Brain Metaphor."

Ask your coachee to imagine a car that represents their issue: How does it look? What color is the body? What about the doors, seats, steering wheel, trunk, etc.?

Then, invite them to visualize where they are sitting in relation to the issue inside the car. Are they in the driver's seat, passenger seat, back seat, or even in the trunk? Ask them to reflect on why they associate themselves with that position.

Now, instruct the coachee to mentally step out of the car, walk around it, and choose a different seat. The specific seat doesn't matter—only the change in perspective does.

Then, ask them:

- How does this new perspective change their evaluation of the issue?
- What consequences arise from this shift?
- What new possibilities emerge?

This process can be repeated multiple times until the coachee finds a position that allows them to experience and reevaluate the situation in a new way.

By shifting perspectives, we gain a fresh outlook on the topic. For example, if a coachee originally sees themselves in the passenger seat or back seat, it can be very empowering to move into the driver's seat. This shift is often associated with the feeling of "taking control" and actively steering one's own life.

Conversely, switching from the driver's seat to the passenger seat or back seat can also be invaluable. It allows the coachee to experience what it feels like to let go, trust the process, and allow things to unfold naturally.

These are just two possible interpretations of a perspective shift. However, Dr. Lucas Derks' research suggests that this technique also leads to a shift between brain hemispheres, enabling re-evaluation from a different cognitive domain—by switching from the left to the right hemisphere or vice versa.

### Summary of the format

1. Invite your coachee to focus on the problem that triggers negative emotions. Ask them to stay with it until they clearly recognize the associated feelings.

2. Have them imagine the problem as a car inside their mind and ask:  
*"When you think about the problem, where are you sitting? Behind the wheel? In the passenger seat? Somewhere else?"*  
Assess how they interpret this position and which beliefs are tied to it.
3. Ask:  
*"If you think you know from which side you're approaching this issue, imagine this: You open the car door (your mind), step out, walk around the back of the car (switching brain hemispheres), and get in from the other side, taking a new seat."*
4. Continue:  
*"Now, think about the problem from this new position inside the car." (The other brain hemisphere is now engaged.)*
5. Ask them:  
*"Notice the difference in the quality and intensity of your emotions. What new insights emerge? What beliefs are associated with this new position?"*
6. Together with the coachee, explore the potential consequences and opportunities that come with this perspective shift.
7. Conclude the process with an ecology check and Future-Pace to ensure that the new insights are sustainable and applicable to future situations.

### 8.2.3 Clean Space for Belief Transformation

When we get stuck with a topic, it often feels like we're trapped in a chain of thoughts. The thoughts revolve around the same point repeatedly, and we can't find a solution. This results in a feeling of hopelessness because we are trapped in our thinking patterns.

Body language often shows that the coachee uses fewer gestures and adopts an increasingly rigid posture. The stronger the problem focus, the more the gaze is directed downward at a single point. This point represents the location in the internal perception space where this problem is situated. We know from working with eye patterns that this is exactly where the transition between inner dialogue (looking down to the left) and feeling (looking down to the right) occurs.

The place where the coachee situates the problem in space corresponds to the location in their mental space that represents this problem area. Many people know that when they are sitting in front of the computer and can't find a solution, it is not wise to stare at the monitor longer and work harder. Instead, it is wise to change the location and do something else to find a new approach.



James Lawley and Penny Tomkins have developed a format along this logic that enables belief work without the coach/therapist getting involved in the content of the belief work. It is essential that the coachee connects with the topic and a specific situation until they focus on the topic in space and experience the usual negative behavior pattern and get stuck there. For belief work, the question can be asked: "When you are in this situation in this state, which beliefs are active?" The coach writes these beliefs on a floor anchor and places them at that location.

With a gesture pointing into the space, the coach asks: "Where is a place that knows more about this topic?" This question offers the coachee a perspective shift. What the coachee does in their mental space is use an unconnected MSI to gain a new perspective on this topic. At this new position, the coach invites the coachee to open up to what this place knows about the topic. It usually takes a while for the coachee to adapt to this new perspective. This process is repeated until the coachee reaches a point where they get stuck again. Then the coach offers the same question again: "Where is a place that knows more about this topic?"

Typically, it takes between four and seven different perspectives until the coachee finds a solution. Coachees often move around the room to different places. Many even step onto chairs to gain a higher perspective. James Lawley had a mobile staircase for his coachees.

Once the coachee has found their solution, we return to the starting point. Here, the insights are gathered, and it is checked again if the coachee can still access the initial state. The format can be entirely context-free if needed. Many coachees find it helpful at this point to verbalize the insights. It is also helpful to formulate the new insights into beliefs or guiding principles from the starting position. It is important to also formulate any possible consequences that come with the found solution.

### **Summary of the Format:**

1. The coachee thinks about a topic that worries them until they get stuck in negative feelings (problem state).
2. First Clean Space question: "Where (with a gesture in the room) is a place that knows more about this topic?"
3. The coachee will find a place in the room. The coach encourages them to go to this place and open up to everything that this place knows about the topic.
4. Second Clean Space question: "What is it that this place knows about the topic?"
5. The coachee thinks about the topic from there until they get stuck in negative feelings again (the next problem state).
6. Third Clean Space question: "Where (with a gesture in the room) is a place that knows more about this topic?"
7. The coachee points to another place in the room. The coach encourages them to go to this place and open up to everything that the place knows about the topic.
8. Fourth Clean Space question: "What is it that this place knows about the topic?"
9. These steps are repeated until the coachee no longer gets stuck in negative feelings.

10. Then return to the starting position with your coachee. Invite them to perceive the difference to the problem state from there and visually trace all the steps taken on the way to the insight.
11. What consequences arise from the solution?

## **8.2.4 Collapsing Anchors in Space**

Collapsing Anchors is one of the oldest formats in NLP, which is also exceptionally effective neurobiologically. This format is also based on the insight that a certain state has a location in mental space.

### **8.2.4.1. Collapsing Anchors with Body Anchors in the Mental Space Including Self-Image Integration**

The original format of Collapsing Anchors uses hand anchors, where body anchors are established on the wrists (where the pulse is located) to anchor both a positively and a negatively experienced state.

The best seating position for this, according to our experience, is for the coach and coachee to sit offset from each other, so the coachee has an unobstructed view forward. Chairs or armchairs with armrests or a table where the coachee can rest their arms are most suitable.

As the coach, clarify which arm the coachee will use for the good state and which for the bad state. Then, after asking for permission, take the specified wrist in your hand and ask the coachee to connect with the negative state. Briefly ask about the submodalities of the coachee's internal experience across all perception channels. Emphasis is on "brief" because the goal is simply for the coachee to become aware of the active submodalities. During this time, activate the anchored situation at the wrist at regular intervals and with consistent pressure. For instance, you can let the coachee's wrist rest in your hand and perform regular, even pressure movements with your thumb. Confirm if the pressure is comfortable for the coachee.

After identifying the submodalities, ask the coachee to mentally step out of the situation and release the wrist. Offer a separator to shake off the negative state, such as questions like "What is your favorite food?" or "Where do you like to travel?"

Continue with the format by asking: "In what situation do you feel particularly safe and/or powerful?" The coachee chooses a specific situation. You ask for the second wrist, initiate the VAKOG trance, and apply pressure with your fingers at the coachee's pulse point, as done previously. The other hand can rest during this time. Ensure that your hands are parallel to those of the coachee to avoid any confusion, which could hinder the merging of the hands later.

Here, it is essential to inquire about the mental representation through the perception channels' submodalities. The trance can be a bit longer, as the goal is to have the coachee fully experience the resourceful state with all senses. Therefore, no separator is needed afterward.

Next, take both wrists of the coachee, activate both anchors with regular pressure movements, and ask the coachee to transfer the submodalities from the positive state to the negative state step by step. You can support the process by verbally repeating the identified positive submodalities. Allow the coachee enough time to transfer the positive submodalities to the negative situation.

Merging the perceptions from the different contexts through the two anchors leads to a collapse of the previously experienced states, creating a new, third state. Neurobiologically, with Collapsing Anchors, the connections between the neural arrangements in the brain related to the two events collapse, resulting in a new, third state. This connection is novel to the brain, marking a significant pattern interruption. Our brain now attempts to apply this new, more attractive pattern to the problem state, which may take some time. The result is a new representation in the mental space.

To support this process, slowly bring the coachee's hands together until both hands are ideally one on top of the other, with the positive experience hand on top and the negative experience hand underneath (the positive experience enveloping the negative experience).

It may take some time for this new mental representation to appear in at least three primary perception channels (sight, hearing, feeling). Usually, all five perception channels are active in this third state. Once the new representation is active, ask about the values and new core belief that become tangible for the coachee when they use this state in the future. This formulation process may take some time, and it can be helpful if the coach repeats the new belief aloud. The coachee often reformulates it until it feels right for them.

Once the formulation is established, support the coachee in integrating it at their identity level with the question: "Who are you when you live this state with the help of your belief in the future? What shapes your identity then?" The identity level question is crucial for making the found solution part of the overall personality. This approach is not only suitable for resolving behavior-level blocks but also for belief and identity levels. Since our health is closely tied to our identity, this format is also applicable to health problems, particularly for treating allergies<sup>51</sup>.

### **Summary of the Format:**

1. The coachee identifies a recurring problem situation and chooses one of their wrists.
2. The coachee experiences the problem state in a brief VAKOG trance to become aware of the active submodalities. The coach regularly applies consistent pressure on the coachee's pulse point, anchoring the negative situation.
3. Release the wrist and perform a separator to detach from the negative experience.
4. The coachee identifies a situation where they feel powerful and secure. The coach takes the other wrist, initiates a VAKOG trance, and establishes the second anchor while the first wrist rests.
5. The coach takes both wrists, activates both anchors, and asks the coachee to transfer the positive submodalities to the negative state. Allow ample time for this process.

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<sup>51</sup> Dilts, 2015

6. Slowly bring the coachee's wrists together, ensuring the positive experience hand is on top. Allow time for the new state to emerge.
7. Identify the values and core belief associated with the new state.
8. Integrate the belief at the identity level by asking identity-related questions.
9. Conduct an ecology check.
10. Identify consequences and perform a future-pace.

#### **8.2.4.2. Collapsing Anchors with Ground Anchors and the Social Panorama**

The classic Collapsing Anchors format with ground anchors simply spatializes the process described above. However, experience with Mental Space Psychology (MSP) has shown that in many cases, the Social Panorama plays a crucial role in the problem. For this reason, whenever relevant, we incorporate the Social Panorama for both the problematic and the positive situation. This enhances the collapsing effect, as the coachee immediately perceives how their perception of the people involved shifts as well.

Ask the coachee to identify a location in the room that represents the problem. Mark this position with a ground anchor (e.g., a colored piece of paper). Then, have them stand on this anchor and fully experience the problem situation as if it were happening in the present moment. Also, ask them to place the people involved in the situation at specific locations in the space.

Figure 27: Collapsing Anchors with the Social Panorama

As with all formats, it is essential that the coachee selects a single representative situation, rather than a mix of multiple experiences. This ensures that the chosen situation accurately represents the problem pattern.

Once the coachee has fully represented the problem through all sensory channels, ask them to step out of the situation. Their sensory representation can be further explored through submodalities, while the Social Panorama positions the involved people within the space. It is important to introduce a separator at this stage, allowing the coachee to mentally disengage from the emotional intensity.

When working with ground anchors, invite the coachee to create spatial distance between themselves and the problem anchor. This step is often experienced as highly liberating, as it aligns with our natural mental spatial organization—we instinctively push unpleasant experiences further away to make them smaller and more manageable. This spatial distance is therefore a key step in dissociating from the problem.

A scaling query helps the coachee evaluate the intensity of the problem on a negative scale from 0 (neutral) to -10 (completely negative).

After the separator, ask the coachee to find a space in the room where they feel particularly strong and safe. Mark this position with a ground anchor as well. Invite them to step onto this anchor and fully connect with the experience of this positive state.

This positive state is deepened through a simple VAKOG trance, including submodality exploration, to help the coachee refine their sensory experience. As in the problem situation, ask them to assign locations to key people who are present in this positive context. Work with the coachee until they vividly experience the positive state. Then, conduct a scaling assessment on a positive scale from 0 (neutral) to +10 (completely positive).

Next, establish a third position in the room—the meta-perspective—marked by a colored anchor. From this meta-position, the coachee can observe their experiences from an external perspective. Useful questions here include:

- *“What is it that is effortlessly available to you in the positive situation, but that you need most in the negative one?”*

This reflection allows the coachee to recognize their own strategies and make them consciously accessible.

In the third step, invite the coachee to step from the positive anchor into the problem situation, carrying the experience of the positive state with them. The coachee focuses on the positive ground anchor and allows its sensory qualities to flow into the problem anchor.

You can support this process by reinforcing the positive submodalities previously identified. Additionally, ask the coachee to place key people from the positive situation within the problem space as well.

#### Figure 28: Collapsing Anchors in the Social Panorama

This process often requires some time before a new solution model emerges. However, integrating the Social Panorama into this format creates a more stable solution, as the positively experienced social environment becomes part of the transformation.

Next, ask the coachee to return to the meta-position and reassess the experience using the scaling question again. Typically, the coachee will already report a significant improvement at this stage.

As with body anchoring techniques, you can now ask the coachee what new beliefs have become actionable as a result of integrating this solution into their life.

Another key question is:

- *“Who are you now on an identity level, given that you are fully integrating this new solution?”*

Both approaches include an evaluation of consequences and a check for potential resistance (ecology check). Finally, the Future-Pace helps the coachee explore how this new belief and solution will shape their future.

## Summary of the Process

1. The coachee identifies a recurring problem behavior and experiences it through a brief VAKOG trance while standing on a ground anchor. The coach elicits submodalities.
2. Introduce a separator.
3. The coachee identifies a state of strength and safety, which does not have to be related to the previous situation. They step onto the positive ground anchor and fully experience it. The coach elicits submodalities, ensuring they fully engage with all senses. The emotional experience is intensified.
4. Ask the coachee to take a meta-position in the room, from where they can observe both situations. Questions to explore:
  - *"Which resources from the positive state are most needed in the negative situation?"*
  - *"How do the strategies in both states differ?"*
5. Invite the coachee to return to the original problem state while integrating resources from the positive state. Support this process by reinforcing the identified positive submodalities.
6. Give the coachee ample time to experience how a new, third state naturally emerges, integrating both perspectives.
7. Which beliefs become actionable as a result of this process?
8. Ecology check: What are the implications and consequences of this transformation?
9. Future-Pace: How will this new solution shape their future experiences?

### 8.2.5. Chaining Anchors in Space

It would be ideal if we could find a solution to a problem immediately. However, for most people, it's not that simple. Instead, the path to a solution is often perceived as difficult and lengthy. When this is the case, coachees need an entry path that allows them to move forward. Chaining Anchors, the process of linking anchors together, provides a highly effective way to facilitate this transition.

By working in physical space, the chaining of anchors becomes particularly intuitive when applying insights from Mental Space Psychology (MSP). This method makes use of the brain's natural associative process—the way our mind connects relevant steps along the path to a solution. This is exactly how our brain functions: once a solution is in sight, it immediately starts mapping the path to get there.

Chaining anchors is especially useful when the solution feels difficult or complex from the coachee's perspective. This method allows them to progress step by step through resourceful

states, which makes the transition from problem to solution more manageable. The process mirrors the way our brain establishes new neural pathways, reinforcing associations that guide us from frustration to curiosity to motivation. This technique can be applied to many real-life situations, helping individuals move from a problem-focused stuck-state into a resourceful state.

#### Figure 29: Chaining Anchors

Here's an example:

A 36-year-old coachee wants to gain more self-confidence in her professional life. First, I ask her to place a ground anchor in the room to represent her problem. I also place another ground anchor for a meta-position, from where she can observe herself externally in a typical problem situation. Helpful questions at this stage include:

- *How tall do you perceive yourself to be in this situation? The same height as in reality, or smaller or larger?*
- *What is your body posture, facial expression, and gestures when you experience this?*
- *How much space do you have for yourself? If you were to show this with your arms, what would it look like?*

These initial questions already provide valuable insights into how the coachee creates and experiences the problem. In challenging situations, we often perceive ourselves as smaller, with a stooped posture, downward gaze, withdrawn gestures, and limited space around us.

In the next step, I invite the coachee to step onto the problem anchor and briefly connect with the experience. Using a VAKOG trance, I help her explore the sensory representation of the problem.

Important: The goal is to help the coachee identify relevant submodalities, so they have a reference point for the solution. Each VAKOG channel, especially kinesthetic perception, should only be experienced briefly to avoid triggering a stuck-state. If the coachee becomes overwhelmed, they may disengage from the process and lose trust in the coaching.

To help the coachee disengage from the negative experience, I use a separator to shift them out of the problem state.

Next, I invite her to choose a location in the room to represent the desired state or outcome. This location is also marked with a ground anchor. From the meta-position, she explores the representation of her goal state. Helpful questions at this stage mirror those from earlier:

- *Who do you want to be in relation to your solution?*
- *How tall do you perceive yourself to be when your problem is completely resolved?*
- *What is your body posture, facial expression, and gestures?*
- *How much space do you have for yourself?*
- *How do you view the world from this perspective?*

I support the coachee in fully experiencing her goal state through a VAKOG trance, engaging all sensory channels. However, if she is not yet ready to experience the target state, that is also perfectly fine—the full sensory integration can happen later.

At this stage, the coachee begins to identify the resources she already possesses to move from the problem state to the goal state—and which resources are still missing. As a coach, you can support this process by using hypnotic language patterns, such as:

*"Imagine that the path from your problem state to your goal state is like crossing a bridge. Which of your existing resources can help you cross safely to the other side?"*

Helpful questions in this context include:

- *Which of your resources can you always rely on?*
- *What else?*
- *What is the first step onto your bridge?*
- *What is the core element of your bridge?*
- *What is the final decisive step to reach the other side?*

The coachee establishes key steps along the bridge using ground anchors. Throughout the process, she has the freedom to move, rearrange, and adjust these bridge elements. She may also briefly step into a VAKOG trance at each step to ensure the transitions are supported by positive submodalities.

If a coachee encounters a block at a crucial step, coaches can facilitate by asking:

*"Which resource is missing for you right now?"*

*"Who could symbolically act as a mentor here, offering you the needed resource?"*

## Summary of the Process

1. The coachee names their problem.
2. Ask them to place a ground anchor for the problem state and briefly experience its submodalities in a VAKOG trance.
3. Invite them to place a ground anchor for the desired goal state.
4. Establish a meta-position and support the coachee in understanding how they want to experience the goal state and what is achieved there.
5. Invite the coachee to experience the goal state in a VAKOG trance.
6. Return to the meta-position.
7. Help the coachee identify the necessary resources to move from the problem state to the goal state, as if crossing a bridge.
8. Ecology check.
9. Future-Pace.
10. Work out the consequences that result from this process.



## 9. The Self as a Spatial Construct

### 9.1 Introduction

To define and describe ourselves, we need a concrete mental representation of who we are. This self-image is shaped by our inner perception and the beliefs we hold about ourselves and the world.

When asking someone with a strong and confident self-image to describe themselves, their internal representation is typically life-sized and positioned about an arm's length in front of them. If we examine the submodalities of their perception, the kinesthetic impression, sound, color, and even smell/taste are often closely aligned with their actual physical characteristics.

#### Figure 30: Confident Self-Image

If a person's self-image is characterized by limitations, this immediately affects their internal spatial representation. When someone perceives themselves as limited, their internal representation also shrinks accordingly. This is often reflected in body language—under stress, people tend to make themselves smaller, pulling their head into their shoulders.

When asked how much space they allow themselves in a challenging situation, coachees demonstrate a noticeably reduced area around themselves. If we go further and ask how large they feel, their self-image—whether internal or external—appears significantly smaller than their actual size.

#### Figure 31: Limited Self-Image

Since we take on different roles in life, our self-image shifts depending on the role we are in at a given moment. However, in most cases, these different roles overlap significantly. If this were not the case, we would be dealing with a split personality.

In cases of severe dissociation, the different parts of a person's identity may be so distinct that they have no awareness of each other. This phenomenon is typically triggered by extreme trauma, where survival was only possible by creating an entirely new Social Panorama as a means of escape. Such cases go beyond the scope of this book, as they often require intensive therapeutic intervention<sup>52</sup>.

What this book does address, however, are the natural identity shifts and developments we all experience throughout life. These can be triggered by life transitions, illness, loss, or significant disruptions on our personal path.

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<sup>52</sup> See chapter 5

At its core, identity work is complex. We do not simply change our identity overnight. Instead, we tend to cling to our existing self-concept for as long as possible, even when we already know that holding onto it is no longer in our best interest.

In relationships and professional settings, we often remain in situations long after dissatisfaction has set in, holding onto the relationship or job instead of actively shaping a new future. In these cases, there is no clear vision of our future self.

Creating a strong, confident self-image and developing a path to reach it is therefore one of the key objectives of working with our sense of self.

The following formats demonstrate how Mental Space Psychology (MSP) can be used to achieve this transformation.

## 9.2 Coaching Formats

### 9.2.1 The Sphere of Intimate Space

A confident self-image can only be sustained if we mentally and spatially experience ourselves as protected. Many people describe protection as the ability to set boundaries.

From a mental space perspective, this means maintaining intimate space even under stress. If this is not possible, people often experience it as if someone else is mentally or physically invading their space, taking it over, and leaving them no room to move.

Figure 32: Sphere of Intimate Space

A simple method to become aware of the space we need to feel confident and secure is by establishing the sphere of intimate space. The specific representation of this sphere varies individually. Some people visualize a protective dome of glass, others imagine a gaseous sphere, a shielded enclosure, or even a completely open space. There are no limits to how this experience can be represented.

As coaches, it is essential to help our coachees clarify the following key aspects:

- *"I am the only person who decides who may enter this sphere and who may not."*
- *"I am aware of my real size and claim the space that belongs to me."*
- *"I ensure a firm stance so that I can feel my strength."*

To support this process, ask the coachee to demonstrate their actual size by indicating it with their hand in front of them. Then, gently adjust their indicated height to match their real size. By placing your hand on top of theirs (after asking for permission) and inviting them to press against it, the coachee can physically sense their true size.

Invite the coachee to spread their arms and feel the full extent of their intimate space. Then, ask them to imagine this space as a sphere with a protective surface surrounding them. Encourage them to visualize that they alone decide who, what, how, and when something may enter this boundary.

A helpful technique is to flood the sphere with warm sunlight, ensuring that every part of it is evenly illuminated from within.

Move around the coachee at approximately 45-degree intervals, speaking in an even, neutral tone:

- *"You are a good person."* (or *"You are worthy."*)

Encourage the coachee to describe how this statement feels from different positions.

Experience from hundreds of coaching sessions has shown that, initially, this statement is often perceived differently from various angles. When this happens, invite the coachee to adjust the lighting in their sphere, ensuring it is evenly illuminated with no remaining shadows.

Once this is achieved, the statement *"You are a good person."* will be emotionally perceived the same from all directions.

### Figure 33: Calibrating Intimate Space

From our experience, this exercise is an effective introduction to self-image and self-awareness work. It often leads to creative and highly personal representations of intimate space.

For coachees, the effect is a noticeable strengthening of their self-image, particularly in interactions with people they perceive as challenging or uncomfortable.

If this format does not work immediately, there is often an unrecognized person or unresolved issue present in the coachee's intimate space. In such cases, it is important to first remove that person or issue before continuing. The Social Panorama method (Chapter 10) describes the necessary approach for this process.

## Summary of the Process

1. Mark the intimate space on the floor using tape, aligning with the coachee's full arm length.
2. Walk around the coachee, repeating in a steady tone:
  - *"You are a good person."* (or *"You are worthy."*)

- Ask the coachee to evaluate how they perceive this statement emotionally from different angles.
  - Any positions where the statement is perceived negatively should be specifically addressed in the next step.
3. Ask the coachee to imagine a sphere around them and ensure it is evenly illuminated. The size of the sphere should be at least as large as their arm span.
  4. Support the coachee in evenly illuminating the sphere. If certain areas remain dim, guide them to transfer light from brighter areas to the darker ones.
  5. Once the sphere is fully illuminated, test again by repeating the statement:
    - *"You are a good person."*
    - Ensure the statement is now perceived consistently from all angles.
    - If inconsistencies remain, return to Step 3 and adjust the illumination further.

### 9.2.2. Recalibrating Self-Image

To value ourselves, we need a confident image of ourselves. As previously described, this self-image should ideally be about as large as we truly are. However, many people do not experience this. This discrepancy often stems from imprints and learning models we carry from childhood and adolescence. If we were pressured or limited during those stages, the tendency for many is to make themselves smaller.

When asked how they experience themselves under pressure, coachees often respond, "I feel small...", "I can't breathe...", "I have no space at all..." This spatial-mental description clearly indicates what the person is experiencing in their mental space at that moment.

If we wish to support a coachee effectively in such a situation, recalibrating their self-image is a fundamental element of the solution process.

The first step is to work with the coachee to determine how large they perceive themselves to be when they experience pressure in a specific situation. Additionally, it helps to ask how much personal space they allow themselves in that moment.

In the second step, use tape to mark a circle around them at arm's length, aligning as closely as possible with the coachee's reach.

Next, you can ask the coachee to grow into their full size in the space while they are in the negatively perceived situation and to show this physically. If they are unable to do so, ask what is preventing it, and invite them to name the issue or simply place it as an element in the space. Then, ask what this element represents.

If there is such an element, the goal is to find an appropriate place for it that allows the coachee to assume their normal size in that situation. This process can take some time, as additional people or beliefs that hinder inner growth often emerge.

Once their true size is restored, the coachee should claim their personal space as well. This means allowing themselves at least the space that corresponds to their arm's reach. Most coachees, however, need an additional 30 to 50 cm beyond their intimate space to feel comfortable.

Once this space is established, the coachee may want to move certain elements in the room to avoid encroaching on their personal space.

When the space feels stable, test the original situation to see how they now perceive it. Additionally, find another situation in which this pattern had previously occurred to test the stability of the solution.

If the results remain stable, an ecological check and Future Pace can then be used to assess what consequences and possibilities the new perception contains.

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### Summary of the Format

1. Identify a situation where the coachee perceives a distorted self-image.
2. Ask the coachee how large they perceive themselves to be in that moment, and have them demonstrate this in the space.
3. Ask the coachee how much space they allow themselves at that moment, and have them show this in the space as well.
4. Mark an area around the coachee approximately equal to their arm reach (intimate space).
5. Invite the coachee to adopt their full body size. If they struggle, ask them to name the elements that prevent this and place them in the space as floor markers. Continue this step until all limiting elements have been placed in the room.
6. Move the limiting elements around until the coachee can re-evaluate and find an appropriate placement for them.
7. Ask the coachee to perceive their full size now.
8. Invite the coachee to extend their intimate space to cover the entire intimate area or beyond, until they feel a sense of personal calm. Allow them time to experience this.
9. Revisit the stressful situation to test whether the calibration remains stable. If not, return to step 5.
10. Have the coachee test the new model in another situation where they typically feel pressured.
11. If the calibration is stable, conduct a thorough ecological check and Future Pace to evaluate the consequences and integrate the insights into the coachee's life.

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### 9.2.3. Core Identity – The Core of the Self

Life constantly exists within a field of tension. Obligations related to work, family, and health are often cited as the central influencing factors in a person's life. However, this focus can lead us to lose sight of what we personally need in different areas of life to lead a happy and fulfilling existence.

Positive psychology has developed an alternative perspective in this regard<sup>53</sup>. Instead of asking what is missing or problematic in life, it asks:

*"What do we need to build a stable identity?"*

This identity is composed of five pillars:

- Work and Activities
- Relationships and Social Environment
- Body and Health
- Personal Development, Spirituality, and Values
- Material Security

These pillars are shaped by our upbringing, socialization, and life experiences. The way we perceive them is neither right nor wrong but simply a reflection of how we have lived so far.

Figure 34: Core Identity

In our work, we use this model as an initial tool to analyze the coachee's current situation. The goal of this format is to identify where they currently stand in relation to each pillar of their identity.

One of the first questions is:

*"How satisfied are you with each of these pillars, expressed as a percentage?"* This helps coachees develop awareness of their position in different areas of life.

Important: This process requires time (at least two hours, based on our experience), so coaches should plan accordingly.

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<sup>53</sup> The **Core Identity Format** is based on the model of the **"Five Pillars of Identity"**. This concept traces back to the German psychologist **Hilarion Gottfried Petzold**, who, in the 1960s, developed the **integrative therapy** approach. In 1974, together with **Johanna Sieper** and **Hildegund Heini**, he founded the **Fritz Perls Institute for Integrative Therapy, Gestalt Therapy, and Creativity Development**. Later, in 1981, he established the **European Academy for Psychosocial Health** in Hückeswagen. For more information, see: [Hilarion Petzold – Wikipedia](#).

The next step is to assign each life area a specific position in mental space. This is important because it allows the coachee to spatially position these different aspects as sub-identities within their overall personality.

Example from Coaching Practice: P., a 53-year-old coachee, sought coaching due to health-related issues.

I first ask P. to find a place in the room for each of the five pillars. Then, I ask him which pillar he would like to start with.

Now, the goal is to imagine what this sub-personality would look like if it existed with 100% satisfaction in life. At this stage, it is important to first explore this state dissociated, describing the ideal version from an external perspective across all sensory channels. Key questions include:

- *How tall is this ideal self in relation to this pillar?*
- *What does this personality look like? How is its posture?*
- *What is its muscle tone and body tension?*
- *How does this personality sound? What does its voice sound like?*
- *How does this personality feel from the outside?*
- *Is there anything to smell or taste in this state?*

Remaining in this dissociated state, I then guide P. to identify the resources he already has that can help him achieve this state.

Once the image is fully developed, I ask him to step into this version of himself, fully associating with it. Using a VAKOG trance, I help him deeply experience this state.

At this stage, coachees often adjust or expand the necessary resources to reach 100% satisfaction in each pillar.

Important: 95% or 99% is not enough. The coachee must allow themselves to reach 100%. Even if they feel mostly satisfied, the remaining gap can be crucial.

For example, P. initially stated: *"I am very satisfied with my family life. I love my wife, I have two wonderful children, and everyone is healthy."*

When asked what the additional 5% satisfaction would mean for him, he responded:

*"It would mean that the responsibility for our financial security doesn't rest solely on me. I would feel more balanced if my wife returned to work and contributed to our household income. I would have less stress and feel much more at ease."*

In the second step, I ask him to step out of this pillar. We then repeat the process for all the remaining pillars and their corresponding sub-personalities.

Finally, P. returns to his starting position and observes how the overall picture of his identity has changed. I ask him to decide where each sub-personality should be placed. Most coachees position their sub-personalities at even distances around them.

### Figure 35: Core Identity in Mental Space

To conclude, I invite P. to connect with all his sub-personalities and absorb their energy and insights, integrating them fully into his core identity. This process is guided by a VAKOG trance, helping to actively transfer the sub-personalities into the whole self. At this stage, it is crucial to ask: *"Who are you now, having integrated these sub-personalities?" "What meaning does this bring to your life?"* These questions of identity and purpose significantly support the integration process.

The effect is that coachees experience how to bring together and balance the different pillars of their identity. This process takes time and should not be rushed.

After completing this step, ask the coachee to step out of the format and observe the entire process from an external perspective. Encourage them to reflect on the consequences of this realization and explore how they can integrate these insights into their daily life to make them a stable part of their identity.

This is followed by a Future-Pace, where coachees envision the long-term effects of their newly aligned identity.

## Summary of the Format

1. Analyze the current status of each identity pillar in a dissociated state.
  - Draw the five pillars on a flipchart and ask, *"How satisfied are you with each pillar as a percentage?"*
2. Establish ground anchors for each pillar, emphasizing that each represents a sub-personality to maintain the coachee's integrity as a whole person.
3. For each sub-personality, ask dissociated questions:
  - *What would make this pillar 100% fulfilled? (99% is not enough.)*
  - *If you fully embody this sub-personality...*
    - *How do you look? How do you see the world?*
    - *How do you sound? What do you hear?*
    - *How do you feel?*
    - *What do you smell and taste?*
4. Have the coachee step onto each ground anchor and fully associate with the experience of 100% satisfaction in each pillar.



5. Repeat this process for all pillars.
6. Return to the starting position and have the coachee assess where each sub-personality belongs in their mental space. Adjust positions as needed until the placement feels right.
7. Guide the coachee to fully integrate the sub-personalities into their whole identity.
8. Ask: *"Who are you now, as a result of integrating these sub-personalities?" "What meaning does this bring to your life?"*
9. Ecology check and Future-Pace (dissociated view):
  - *What are the consequences of this integration?*
  - *How will you apply these insights in your daily life?*
  - *What long-term effects will this have on your identity?*

#### 9.2.4 Developing the Target Identity

Especially during transitional life phases where we feel uncertain about our path forward, we often ask ourselves:

*"Who am I, and who do I want to become?"*

This question is closely tied to another:

*"What is holding me back from becoming the person I want to be?"*

A common strategy many people adopt is simply continuing as they always have, hoping that a new path will eventually reveal itself. As a result, countless individuals go through life without ever truly answering these fundamental questions.

The reason why many of us avoid actively addressing these questions lies in our belief systems—especially when contradictory beliefs are at play (see Chapter 0).

#### Figure 36: Developing the Target Personality

This coaching format allows coachees to explore these conflicting beliefs, often leading to a significant mental, emotional, and physical relief, particularly regarding deep-seated issues.

Important: The goal here is not to suppress limiting beliefs—this doesn't work. Any belief or issue we try to consciously or unconsciously suppress will eventually resurface with full force until we resolve it.

Interestingly, when working with our personal shadows or "inner monsters"—the aspects of ourselves that seem to cause harm—we often find that they actually hold a positive intention and valuable resources if we are willing to confront them. Running away only makes our shadows grow larger and darker, as seen in the impact of negative goals (see also Chapter 11).

For this reason, Mental Space Coaching (MSP) and NLP-based methods focus on integrating limiting behavioral patterns and belief systems. When we do this, we find inner peace and gain clarity about how we want to shape our future.

This approach is based on Robert Dilts' "Conflicting Beliefs" format<sup>54</sup>, which has been modified and simplified by integrating insights from Mental Space Psychology (MSP).

As a coach, you should allocate at least two hours for this process. Below, we present two variations of the format.

Using an example from a coachee helps illustrate the format process:

M., aged 43 at the time of coaching, described her life as a "never-ending stream of demands, with the feeling that I can't meet these, either in quantity or quality." Despite several professional successes in recent years, she saw them as worthless. When asked, she said, "I'm not good enough at my job. I constantly do more than others but can't enjoy the success."

In the first step, I ask her to set down a floor anchor representing her current self. As she observes it from the outside (dissociated), I invite her to describe the person standing on that floor anchor.

Helpful questions include: "How large do you see yourself here?" "What is your posture, gestures, facial expressions?" "How do you feel here?" "How much space do you allow yourself here?" "What belief guides you here?"

In the second step, M. sets down a floor anchor for the person she wishes to become—a more developed version of herself. Just as in the previous chapter, she describes her ideal self from a dissociated perspective, noting how this person would look, feel, and act. It's also essential to clarify the physiology of the ideal self. The sequence follows the VAKOG trance method, with M. describing her ideal self as if viewing herself from the outside: how large she is, her posture, her gaze on the world, her voice, and how she feels to the touch. What is her muscle tone? And what might she smell or taste? When asked what belief guides her ideal self, she replies, "I am good as I am and define my own level of effort."

Once the image is complete, I ask her to step into her ideal self and experience it fully through VAKOG. After just a moment, M. jumps off the anchor and exclaims, "This doesn't work! It's not allowed!"

After calming down, I ask her to describe what prevents her from being the person she wants to be.

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<sup>54</sup> Dilts, Robert B.: Die Veränderung von Glaubenssystemen, Junfermann Verlag, Paderborn, 2010, S. 157 ff.

After a moment's thought, she replies, "I don't deserve it. That's selfish! I can only be happy if I achieve more than others."

When I ask where this belief comes from, she responds, "It's pretty old. I only got love and recognition from my parents when I excelled in school."

When asked at what age the belief began, she says, "around 10." At this point, invite your coachee to lay out a timeline on the floor symbolizing their life, starting with a floor anchor for "Here and Now" and moving back to the point where the belief originated. Mark this with a floor anchor.

After some searching, M. places the floor anchor a few meters away from "Here and Now." I invite her to step into this place and moment. After a few seconds, tears begin to fall, and she explains, "When I see myself at ten years old, I see a child whose academic and athletic successes went unacknowledged, treated as a given. That's why I see myself here with hunched posture, always doubting if I'm good enough to meet others' expectations."

In the next step, she steps off the anchor and views the process from a chosen meta-position, allowing for some detachment. This distancing takes time, as distance in space correlates directly with mental distance.

Then, we identify the belief that was active on the floor anchor: "I am only loved when I do more than others."

From this dissociated position, M. identifies the resources her younger self needed to find a belief that would have empowered and supported her growth. M.: "It would have been the assurance that I am good just as I am, with all my abilities, talents, and resources, and that my parents and environment would have reinforced this." After several revisions, she formulates, "I am good as I am."

I then ask her to reconnect with her younger self on the anchor and ask what she needed back then to experience the situation with more resources. Here, M. takes time to ensure her younger and adult selves have a congruent perception of the specific resources she needed to grow strong and confident. M. states, "Self-confidence, self-love, and knowing that I can achieve what matters to me."

The coachee then develops the following affirmations: "I am good as I am," "I can trust myself and my abilities," and "My parents' problems are theirs, not mine."

The next step, which we learned from Dr. Lucas Derks, is critical. I ask M. to stand on a floor anchor marked "Age 9" on her personal timeline, about a year before the negative belief originated. At this point, she confirms, her world was still intact. The limiting belief isn't yet active, which is the key. In the mental space at this time, there are no limiting MSIs (Mental Space Indices). We use this in coaching, as the brain doesn't differentiate between fiction and

reality. As soon as it identifies a better alternative behavior, it will adopt it, provided no older belief or preceding MSI interferes. This is not the case for M.

During this process, it is important to allow the coachee sufficient time to experience the new belief in full sensory (VAKOG) awareness.

After a few minutes, M. stands on the anchor representing her 10-year-old self, upright and smiling. I ask her to proceed step-by-step through the original event, experiencing how she grows with the belief “I am good as I am” through subsequent life phases. As a coach, you can support this process by repeating the associated VAKOG representations. Growing up with this new belief allows coachees to test its impact on other potentially challenging life phases.

At the end, invite your coachee to return to the present moment, step off the timeline, and reflect on the entire process from a meta-position.

The results may already feel congruent and empowering for the coachee, as they did for M. I then invite her to step, associated, onto her timeline and

walk purposefully to her ideal self’s anchor. In VAKOG, she re-experiences all representations tied to her new belief. Finally, I invite her to take a step into the future to sense how her life will unfold if this belief continues to guide her for 2, 5, or even 10 years.

From the meta-position, we examine whether anything prevents her from embodying her developed self from now on and what consequences may result.

### **9.3 Expansion of the Format with Resource Exchange**

You can ask your coachee to consider what resources or beliefs their younger self might need from their adult self to grow up feeling empowered.

In our example, M. brings the belief “*Courage to stand by your decisions*” along the timeline to her younger self and establishes it there.

Next, the coachee moves back to the meta-position to reflect and sense whether the younger self has a message for the adult self. M. writes down “*Lightness and joy in life*” and brings this message—written on a ground anchor—to her adult self.

M. then walks through her entire timeline again up to the present moment. When asked how far she feels from her target personality, she instinctively moves the corresponding ground anchor closer to the present and steps onto it with determination.

Using a VAKOG trance, you can support the full embodiment of the target physiology. M. reaffirms her new belief:

*“I am good as I am and define for myself how much I achieve.”*

To conclude the format, a thorough **ecological check** and **future pace** are conducted to explore and evaluate the consequences and strategies for her further life path.

## Summary of the Format

1. Place a ground anchor for the problem state, and have the coachee describe how they perceive themselves from a dissociated meta-position.
2. Ask the coachee to place a ground anchor for their target personality.
3. Guide them in developing a dissociated image of their target personality.
4. Support them in associating into the experience through VAKOG.
5. Have the coachee dissociate again and identify what is preventing them from becoming their ideal self. Ask where the limiting belief originates.
6. Let them find the point on their timeline where the belief was formed and place a ground anchor there.
7. Dissociated, explore what resources would have been needed at that moment to prevent this imprint from forming. These resources can usually be found elsewhere in their life. If not, they can be accessed using the New Behavior Generator<sup>55</sup>, which allows the coachee to receive resources from mentors. Once the resources are complete, work on formulating the new belief that emerges. Write it on a note/ground anchor and give it to the coachee.
8. Ask the coachee to stand on the timeline about a year before the imprint and experience how all the resources of the new belief were already present there.
9. Now, have the coachee walk through their timeline up to the present, holding their new beliefs, and observe how they now perceive and evaluate past situations differently.
10. Upon arriving in the present, the coachee experiences how all past knowledge integrates into this moment.
11. Ecological Check & Future-Pace: The coachee steps onto another ground anchor to experience their future in 1, 2, or 5 years, exploring how their life develops with the new belief.

## Expanding the Format

12. After Step 10, add a resource exchange: Dissociated, have the coachee reflect on what message or belief their younger self might still need from their adult self in the present. Formulate the belief and bring it along the timeline to their younger self.
13. Let the younger self experience it and then ask what message it has for the present self. Formulate this belief as well.

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<sup>55</sup> The coachee imagines having to learn this ability as an actor would. To do this, they choose a personality who embodies this trait and, with the coach's support, associate themselves with this person until the trait becomes evident in their facial expressions, gestures, and speech.

14. Slowly bring this belief forward along the timeline to the present and integrate it. Let the coachee sense how the belief might evolve at this moment.
15. From the meta-position, gather insights, conduct the ecological check and future pace, and allow sufficient time to explore the consequences of integrating the new concept.

### **9.3.1 Untangling Overlaps**

For an individual to experience their self as sovereign, they need sufficient space in their mental realm (see Chapter 9.2.1). If this space is persistently restricted, many coachees express this as: "I feel under pressure" or "I don't have enough room to breathe." When asked to illustrate, they often gesture with their hands, showing a very limited space around themselves.

However, there is another form of restriction that may not be immediately visible—for example, when a coachee feels strongly dominated by another person. In such cases, their personality becomes overlaid by someone else's. Mentally and spatially, the coachee is either trapped within another person's shell or shares their space within their social panorama. This process often takes place unconsciously, yet it remains continuously active within the coachee's social panorama.

A common example is a dominant mother who continues to exert influence over her child well into adulthood. In such cases, the mother's presence mentally and spatially surrounds the adult child. When asked how they perceive their mother, coachees who feel constantly enveloped by her tend to make constrictive gestures with their hands or arms. Their language reflects this dynamic as well, with responses such as: "My mother says...", "We always do it like this...", or "I have to take care of my mother...". Even when asked about themselves—e.g., "How are you?"—their responses often reference their mother's opinion: "My mother thinks I look too thin/too pale/too stressed right now."

#### **Figure 37: Coachee is surrounded by another person**

A similar phenomenon occurs when a person is unable to let go of a deceased loved one or feels the need to replace them. In such cases, the individual unconsciously carries the deceased within them. The way they speak about themselves reflects this, as if two personalities are overlapping.

#### **Figure 38: Coachee carries a deceased person within**

These overlapping identities can significantly limit self-perception. The individual primarily defines themselves through the person causing the overlay. In many cases, a clear sense of self is missing entirely. Those who experience this describe it as exhausting and overwhelming. At the same time, this phenomenon can serve as a protective mechanism, as it relieves them of

personal responsibility—since it is the overlaid person who is perceived as being in charge. Letting go of this overlay also requires a willingness to relinquish the secondary gain that comes with it. The desire to take ownership of one's own life must become the dominant priority.

Once this readiness is established, the next step is to dissolve the overlay through a ritual. The coachee is asked to stand in the space while a boundary marking their intimate space is drawn around them using tape. They are then invited to indicate where they perceive the overlaid person—whether inside their body, in their intimate space, or elsewhere in the surrounding area. At this point, coachees often gesture toward a specific area of their body or define a constricted space around themselves with their hands.

The next step involves guiding the coachee to symbolically "remove" the overlaid person from their body or to step out of the presence of the overlaying person. If the latter is the case, they are encouraged to take a decisive step forward and assign the overlaid person a new location in the space. In cases of strong dominance, coachees tend to place the overlaid person far away from themselves. When working with the presence of deceased individuals, they often place them in a nearby position—either in front of or beside them. By making the overlaid presence explicit within the space, it becomes possible to engage in a dialogue. The coachee is supported in expressing everything they were previously unable to say due to the overlay.

It is also essential to explore the purpose the overlay served and the benefits it provided. This often leads to an additional repositioning of the overlaid person. The process reaches a resolution when the coachee is able to express gratitude for the beneficial aspects of the overlay and to say a clear farewell. This often involves a ritual that acknowledges the presence's role while setting a firm boundary, such as: *"Thank you for protecting me all these years, but now it is time for me to follow my own path."*

From our experience, such overlays are frequently accompanied by psychosomatic symptoms. In our coaching practice, we have repeatedly observed links between these overlays and conditions such as eczema and other skin irritations. It almost seems as if the skin is trying to signal that something is buried underneath that does not belong there.

Resolving such an overlay inevitably brings about a significant reorientation of the self. The formats described earlier for recalibrating the self (see Chapter 9.2.2) are particularly helpful at this stage. Other formats, such as alignment or goal-setting processes, serve as important additions, as they facilitate the process of designing a new life without the influence of the overlaid person.

This also means that a crucial part of working with coachees in such cases involves carefully exploring the consequences of the resolution process.

### Summary of the Format

1. Ask the coachee to stand in the room and mark their intimate space, for example, with tape. This space corresponds to the space taken up when they extend their arms.

2. Ask the coachee to indicate where the overlaying person is located.
3. Invite the coachee to “pull” the person out of their own body or “step out” of them. Note: For the second variant, a new intimate space must be created in the room to clearly separate the two intimate spaces.
4. Support the coachee in engaging with this person and expressing what has previously been unsaid.
5. Explore what benefit the overlay provided over the years. Discuss how the coachee “benefited” from it (e.g., by not having to fully grow up).
6. Help the coachee find a new, suitable place for this person.
7. Support the coachee in a ritual to express gratitude for the positive aspects and to clearly state that they are now beginning their own path.
8. Check with the coachee whether the self-image is now at rest. If not, return to step 2 as needed.
9. Have the coachee step out of their intimate space and observe the new arrangement from the outside.
10. Carefully work with the coachee to explore the consequences, as reorienting the entire personality is required here.

### 9.3.2 Healing Injured Parts of the Self

In her book *\*Coming to Wholeness: How to Awaken and Live with Ease\**, Connirae Andreas presents a very interesting approach for healing and integrating injured parts of our personality.<sup>56</sup>

The core idea is that, in moments when we try to solve a problem or experience emotional pressure, we often rely on ideas like, “I must solve this problem...,” “I can’t allow that...,” “I’m not entitled to that...,” “I must...” This “I” perception plays a central role because the way we define ourselves is mentally and spatially tied to our self-image. For instance, if we hold negative self-assessments about an issue, this affects our self-perception.

The result is that we reduce our sense of self. This can be likened to a part of our personality that contracts and makes itself small, accompanied by a specific way of perceiving the world in

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<sup>56</sup> See Connirae Andreas, *Auf dem Weg zur Ganzheit, Mit der Wholeness-Methode zur persönlichen Transformation*, Junfermann Verlag, Paderborn, 2020. See also <https://wholenesswork.eu/the-method/?lang=en>.



this state. However, when we are fully connected to our sovereign self, we experience ourselves in our real size, perceiving the space around us in a defocused and open way. Ideally, we feel like part of a larger whole, feeling safe within it. People reach this state of calmness and connectedness when they feel truly secure and free of fear. Meditation can also help achieve this. In such states, we experience ourselves as complete and whole.

But how can we reach this state of wholeness and integrate it into our lives and personalities? Or, conversely, what prevents us from experiencing this wholeness as an everyday state?

The root cause lies in past learning processes that we have mostly internalized unconsciously, leading parts of ourselves to contract. This learned self-model from the past continues to influence the present. The process we aim for is: How can we locate and “heal and make peace” with the contracted parts of ourselves along our timeline? If successful, our current self has no reason to maintain these contracted parts.

Our sense of self is a learned construct. The self we experience today has developed through multiple stages of our younger selves. We all began as infants without any concept of ourselves—an open state of wonder in which everything is new, and our “awareness” is receptive to everything entering our consciousness. At the same time, we are entirely vulnerable and dependent on caregivers for survival. These caregivers thus become our first models of how to navigate this world. We learn to interpret their behaviors in detail, and all their words and gestures carry enormous importance since we have no reference to other behaviors yet. If we are insulted, demeaned, or treated with violence by these primary figures, it shapes our self-image. Without coping strategies, such experiences can become trauma that may impact our entire lives.

If we learn in this early stage of life that love and trust are always present, the likelihood of developing a stable self and self-image is high. Our perceptual submodalities remain open, allowing us to experience the world with all our senses.

However, if we face limitations in these early experiences, it leads to a narrowing of submodalities. This then triggers a self-image in which we may experience ourselves as small and helpless. On the other hand, one might also feel they have to become very large to survive. Either way, it necessitates a change in perceptual submodalities at the level of self-perception. Connirae Andreas<sup>57</sup> uses a fitting metaphor for this state, describing it as a clenched fist of our younger self that we forgot about. This clenched fist remains, becoming an unconscious habit that manifests itself in behavior and experiences in both suitable and unsuitable situations. When we bring this clenched fist back into our consciousness, the spontaneous desire to relax and release it arises.

Since we unconsciously carry this clenched fist of our younger self, it also influences the subsequent development of our self. This does not always affect our entire personality but often specific parts of ourselves.

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<sup>57</sup> Andreas, 2020, S. 29 f.

The foundation of this format is based on the basic process of Connirae Andreas' Wholeness Method.<sup>58</sup>

To start this format, it is helpful to establish an understanding of the concept of one's self and consciousness. Ask the coachee, regarding the topic in question, to connect with their body and notice what feeling arises related to it. Invite them to sense what emerges in their awareness and what is part of it. Typically, the coachee will first mention internal sensations and then external perceptions. We support this process by noting that external stimuli, such as sounds or even mental connections to specific people, are also part of the current field of awareness. It is important to mention that our consciousness can be much larger than we often think and that its boundaries may not always be clearly defined. This guidance helps connect the coachee with their entire field of awareness, which is essential for supporting the later process of relaxation and release. It allows the coachee to feel connected instead of isolated, even if their perception is currently limited by a problem state.

In the next step, invite the coachee to notice what happens to their self when the topic becomes active. This can occur within their body or externally.

To heal this part of the self, it is important to trace the development of individual self-images related to the limiting topic.

**Figure 39:** Finding, relaxing, and releasing injured self-positions

It is helpful to identify a recent situation in which the coachee felt restricted. Ask them to describe the relevant submodalities. Then, work to identify the belief that describes this state of self and ask them to describe the behavior associated with this self-state.

Then, go into the room with your coachee and ask them to place a floor anchor for this situation and connect with it. Ask where and when this self learned this behavior, inviting them to go to a point in the room where they sense this self.

The three-dimensional space plays an important role here, as younger self-states can be located not only horizontally but also above or below the current self. At each self-state, pause and have them identify the belief that is active there. Often, a specific behavior is also mentioned that belongs to this self. Continue this process until reaching the earliest self-state, which may lie in the pre-verbal stage, as indicated by a slowed or absent ability to find words. Here, it no longer makes sense to ask about beliefs; instead, coachees can be guided by their feelings and stop at the point on their timeline where they identify the origin of the contracted self-pattern.

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<sup>58</sup> Andreas, 2020, S. 55 f.

Once the coachee has reached the place of the youngest self that pertains to the issue, ask them to invite this younger self to relax, let go, and experience how it becomes part of the larger field of awareness again.

Then, have the coachee gradually revisit each self-state along their timeline, inviting each self to relax, release, and become part of the larger awareness field.

When the coachee returns to the starting point, we often find that their original perception at the submodalities level has changed. The original belief then transforms into a resourceful concept.

Since working with the self and therefore with a person's identity is fundamental, it is essential to conduct an in-depth clarification of the consequences of this new experience of the self.

### **Summary of the Format**

1. Invite your coachee, for example, through a VAKOG trance, to reflect on their experience of awareness regarding a specific topic.
2. Have the coachee observe where their self is located when the topic is active.
3. Identify the submodalities of this state and the associated belief.
4. Set up a floor anchor for this self and invite your coachee to connect with the self that is active here.
5. Ask, "Where did this self learn that?" Encourage the coachee to find the place in the room where they sense this self.
6. Go to this place, lay down a floor anchor, and allow them to connect with the self that is active there. It is helpful to ask about submodalities and beliefs here.
7. Repeat steps 5 and 6 until reaching the youngest self-state.
8. At this place, ask the coachee to invite the younger self to relax, release, and become part of the larger awareness. Wait until the coachee's physiology visibly relaxes.
9. Repeat step 8 at each floor anchor until the coachee reaches the first floor anchor.
10. Have them observe how they now experience their self and how the submodalities respond.
11. Help them develop the new belief.

12. Conduct a thorough ecological check and Future-Pace so that the consequences at the self level can be assessed.

Figure 40: See Connirae Andreas, *Der Weg zur Ganzheit (Coming to Wholeness)*, p. 55

### 9.3.3 Re-Mission of Identity

Health requires a healthy self-image. This statement, in its broad generalization, is undoubtedly provocative. However, if we wish to recover from an illness, we need a concrete idea of what it is like to be healthy again. If we mentally stay fixed on illness, our bodies will align with this mindset. The world we create in our minds is real for our bodies. Even our immune system does not respond to reality but to what we perceive as real<sup>59</sup>. For example, an artificial flower can trigger a strong allergic reaction in an asthma patient.

Without hope for recovery, it is challenging for the ill to mobilize the inner resources necessary for healing.

*“As one perspective suggests, relationship issues, internal conflicts, and limiting behaviors frequently lead to or exacerbate physical symptoms. Different states and emotions produce a distinct chemical balance in the body and form the basis of illnesses. When you help someone resolve a conflict on the level of identity, you often address the internal states that cause illness.”*<sup>60</sup>

To recover from a life-threatening illness, people need a reason to continue living. For this reason to be strong enough, it must connect with a concrete life goal and a sense of purpose<sup>61</sup>. According to Robert Dilts, working solely with the concept of health is insufficient. This is, at first, only a dissociated projection—an image of ourselves. Yet it is essential, allowing the person to envision how they will experience themselves once healthy, so that their inner system can align accordingly. However, this image alone is not enough.

The key to actual recovery lies in asking: Who will you be when your illness is gone? What will be possible for you then? What goals would you have if illness or pain were no longer a part of your life?

Robert Dilts states, “In my experience, it is very helpful to support people in finding their mission in life. Because if you have no reason to live, why would you strive for healing?”<sup>62</sup>

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<sup>59</sup> Ian Mc Dermott, Joseph O'Connor, 1997, S. 107 ff.

<sup>60</sup> Dilts, 2015, S. 147.

<sup>61</sup> Dilts, 2015

<sup>62</sup> Dilts, 2015

The goal, then, is for coachees to reconnect with their life mission or perhaps discover it for the first time and consciously recognize its meaning on the levels of identity and purpose. If interventions occur only on the lower neurobiological levels, there is a risk that a change in behavior, for example, will quickly fade if the coachees see no sense in it or do not understand its significance for their identity.

Five essential steps are required for a successful re-mission:

1. An inner vision in the form of a projection of the "healthy" self in space.
2. Clarification of limiting aspects and beliefs that inhibit healing.
3. Building an appealing social panorama for the future.
4. Experiencing the healthy self across all neurobiological levels.
5. Timeline on which the coachee can progress toward their healthy self.

### **Step 1: The Projection**

Throughout our lives, we work with a vision of ourselves (see Chapter 9). However, this projection often occurs unconsciously. When we lose hope for a meaningful future, it is usually accompanied by an inability to envision a desirable future for ourselves.

For a re-mission, a compelling vision of our future self is essential. The focus is on creating a projection of what our developed self could and should look like in the future.

To create this, ask your coachee to consider where in the room they would place an image of their future self. Most people place it at a distance of about 1–1.5 meters. Once this is done, have them describe this person from a dissociated perspective. Suggested questions include:

- How tall is this person?
- How would you describe the person's posture?
- How does this person view the world?
- What does this person sound like?
- If you could touch this person, how would they feel?
- If you could smell this person, what would they smell like?
- What would this person taste like?

Once the coachee has fully described the projection, invite them to step into it and experience what it is like to have become this person.

Initially, there may be difficulties in accepting this projected self-image. This is often due to beliefs that may be blocking acceptance. At this point, the corresponding social panorama is also typically absent.

### **Step 2: Clarifying Limiting Beliefs**

Our internal beliefs function as self-fulfilling prophecies, leading us to cling to certain behaviors throughout life, even when we intellectually know they are unhealthy or harmful.

Without the inner conviction that health is a right, achieving health is difficult. Limiting beliefs thus pose a significant obstacle in the recovery process. Therefore, a detailed examination of limiting beliefs, using the Belief Cluster (Chapter 8.2.1) and the Target Identity Development Format (Chapter 9.2.4), is a valuable support. Once limiting beliefs are transformed, coachees are typically better able to integrate into their healthy self-projection.

However, it is equally important to respect if someone has accepted their desire to die and has internally concluded their life. Yet, from the coach's perspective, there remains the question of whether the desire to die might be connected to beliefs that preclude any hope for healing or a renewed sense of purpose.

"Someone may consider their life to be over due to limitations within their belief system or identity. This identity is indeed complete, but that does not necessarily mean that all aspects of their life are concluded."<sup>63</sup>

### **Step 3: Social Panorama**

In the recovery process, it is equally important that one's Social Panorama (see Chapter 10) is balanced to match the healthy self. This may also mean that the positions of certain individuals need to be significantly adjusted so the coachee can live self-assuredly within this healthy self. This is crucial because one of the primary factors of resilience is the network that supports us. Therefore, it's essential to examine whether the Social Panorama fulfills a person's eight relational needs<sup>64</sup>:

1. Need for security
2. Need to be appreciated, validated, and significant
3. Need for acceptance from a strong, dependable person
4. Need for affirmation of one's own experiences
5. Need for uniqueness and being seen and respected in its expression
6. Need for influence
7. Need for others to take initiative as well
8. Need to express love

If the Social Panorama is well-balanced, these relational needs are typically fulfilled by one's network of relationships.

### **Step 4: Alignment in Space**

Once the first three steps are complete, an alignment through the neurobiological levels can integrate and shape these experiences for the future healthy self.

We recommend doing this alignment physically in space, rather than cognitively. Moving through space makes each level a mental-spatial experience of the future self.

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<sup>63</sup> Ebenda

<sup>64</sup> Erkinson, 2015

To facilitate such an experience, it's essential to pass through each level in a waking trance in VAKOG (see Chapter 0). The stronger the connection to the future self-image across all sensory channels, the stronger the effect. All levels should be experienced once from the context and environment up to purpose and then back down from purpose to context and environment.

On the way back, it's important that at each step the connection to purpose is emphasized by the coach. This helps coachees sharpen their focus and understand the specific re-mission that arises from the purpose for their future life.

Figure 41: Alignment across neurobiological levels for remission

### **Step 5: Timeline**

Since recovery is often a process that requires a lot of patience and may progress only in small steps, a pathway toward the goal is needed. Small milestones are often essential anchors or markers for recognizing that we are on the road to recovery. To make this path visible and anchor it as a relevant experience in the body, it is helpful to establish a timeline in space that represents the gradual journey toward health.

As the coach, mark out a timeline in the room, establishing the point of today and the goal of the future healthy self. Along this timeline, use floor markers to represent the individual steps that are meaningful achievements on the path to recovery for the coachees.

For example, one of our coachees had jawbone cancer and, for a long time, could only eat using a feeding cup. His most important initial goal on the road to recovery was to be able to eat solid food again. The goal associated with his healthy self was to have dinner with his wife at a specific restaurant in Mallorca, watching the sunset. This process took two years. It was a joy to sit with him in that restaurant several years later, reflecting on this journey with a good glass of wine.

## **10. The Social Panorama – Relationships as Spatial Constructions**

### **10.1 Introduction**

Whether we let someone get close to us or prefer to keep our distance is tied to the emotional quality of our relationship with that person. When someone gets too close without our permission, we experience it as unpleasant or even threatening. If someone is "breathing down our neck," it can cause significant stress.

It's natural for us to convey the quality of relationships through proximity and distance, even expressing it in our language. Our brain does not distinguish between reality and our inner representation of that reality. Thus, the way we map out our social network within our internal experiential space is crucial to how we interact with ourselves and our "world."

The Social Panorama addresses precisely this question—how we map out our social network within our mental space. How we meet and interact with others is largely determined by our inner image and the positioning of these people in our Social Panorama.

Dr. Lucas Derks, the founder of the Social Panorama, summarizes this with the following thesis: “Our unconscious map of social reality determines who we believe we are.”<sup>65</sup>

#### Figure 42: Social Panorama

Every significant person is represented in our mental space concerning their direction and height. In this way, we create a landscape of representations of people—a Social Panorama.

The Social Panorama model allows us to analyze and adjust this map.

Starting points for this are common patterns that recur in similar forms. For example, coachees experience authority figures who dominate them as taller, too close, and often directly in front of them. The emotional reactions are usually feelings of inhibition around these people, with no sense of equality. When coachees can adjust their perception of the person concerning distance and equality, these inhibitions often dissolve.

#### Figure 43: A dominant person standing too close in the Social Panorama

We all tend to unconsciously project people into our mental space. These can include colleagues, bosses, friends, parents, siblings, partners, and children. The location where we place someone in this mental space determines the emotional significance of the relationship.

> Location = Emotional Relationship

The emotional impact of these images generates a range of subjective experiences, such as support/confrontation, power/powerlessness, dislike/love, loneliness/belonging.

#### Figure 44: Feeling of Threat

Proximity, intimacy, status, subservience, dominance, antipathy, and even hate are also determined by how we position people at a certain distance, in a particular direction, and at a particular size.

Dr. Lucas Derks refers to the way we represent people in our Social Panorama as personification.

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<sup>65</sup> Derks, 2022



Every person experiences themselves at the center of their Social Panorama, creating their own positions and roles from this point.

The core of our self-concept is surrounded by a three-dimensional spatial structure through which we define who we are. If we want to make changes here, we must first examine and balance our social map.

The Social Panorama makes our unconscious model of social reality transparent—this model each of us internally creates for ourselves. It significantly influences our thinking processes and beliefs. The ability to establish social relationships is essential for survival from early childhood, to ensure we are cared for. It is therefore a fundamental unconscious skill in our existence.

Derks refers to this as unconscious social knowledge since we perceive forming social relationships as entirely natural and "normal." The Social Panorama model deals with the fast and automatic unconscious thought processes that accompany these relationships. Because their social reality model is unconscious, most people do not realize:

- How they constantly carry images of others around with them
- That they recreate these images repeatedly
- That they are continually reorganizing and altering them.

Once people become aware of how the Social Panorama works, a whole new world often opens up to them. Dr. Lucas Derks notes:

"They realize that relationships are not uncontrollable phenomena but rather the result of unconscious processes. Once they become aware of this, the Social Panorama appears to most people as something natural because they have—albeit unconsciously—used it since early childhood."<sup>66</sup> Derks refers to it as the "logical result of their early social development."

The model enables us to see and experience how we position ourselves among others. This offers new options that were previously closed off, allowing us to positively transform our social relationships.

The Social Panorama is a highly universal tool that can be used for many issues arising from our social relationships. These include:

- Resolving family issues
- Clarifying professional conflicts
- Reducing and preventing stress
- Grief work
- Balancing different parts of the personality

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<sup>66</sup> Derks, 2022

## **10.2 Coaching Formats**

### **10.2.1 Resolving Issues in the Family System**

Many problems in our social environment stem from the family system. The systemic triad of mother-father-child holds particular significance in understanding our development, as it represents the first social role model we learn. These relationships are fundamentally unique and lifelong, and they cannot be easily “terminated.” This model later expands to include siblings, grandparents, relatives, friends, and sometimes even pets.

Clarifying and balancing this system is foundational for our personal growth and for our ability to freely develop throughout life.

#### **Step 1: Marking and Calibrating the Intimate Space**

To create a Social Panorama, mark the intimate space on the floor with tape as described above. Ask your coachee to extend their arms, ensuring the size of their intimate space roughly matches their arm’s reach.

Invite them to recognize that this space is their very own intimate area, and only people who belong there should be allowed inside. To demonstrate how sensitive this space can be, you might approach the coachee slowly, observing their body language to identify the moment when they show signals that indicate the boundary of their intimate space. Most coachees start to feel uncomfortable 30 to 50 centimeters before the intimate space boundary.

Coachees often report being surprised by the size of their intimate space. This may stem from the fact that many people allow themselves far less space than they actually need to feel comfortable, which impacts their body language. This in turn is sensed by others, who may unconsciously overstep this boundary.

#### **Step 2: Representing the Current Situation**

Once your coachee’s personal space has been calibrated, invite them to name the people who are part of their family system. Write each name on a floor marker and add a small arrow to indicate the person’s line of sight. Ask the coachee to intuitively place each person’s marker within or outside of their intimate space, locating each marker at the spot where the coachee typically perceives that person.

It’s essential to note that people may occupy multiple positions within the space. We refer to these as bi-locations or even tri-locations. These occur due to ambivalent relationships with some individuals or because the person holds different roles in the coachee’s life.

Alternatively, you can use small boards with photos as three-dimensional stand-ins for each person.

#### **Step 3: Analyzing the Current Situation**

Ask your coachee to step back to a meta-position (also marked in the room) and describe their overall perception of the Social Panorama. This step includes exploring the coachee's relationship with each person. Ask how large the coachee feels in comparison to each person and if there is a sense of equality. Be attentive here, as even small differences in height perception can significantly impact the evaluation of the relationship.

To describe each relationship, you may also use symbols to represent the quality of the relationship. Symbols can help make visible any "burdens" or unspoken expectations that have been placed on or adopted by the person. Symbols clarifying family dynamics are often helpful for this purpose.

#### Figure 45: Labeling of Relationships

### **Step 4: Balancing the Social Panorama**

Together with your coachee, examine individuals who do not appear on equal footing and those in the direct focus of the coachee's attention. For those on unequal footing, work toward positioning them in a way that allows for an equal view. The coachee decides whether this alignment is acceptable, but the realization of what conditions are necessary for equality is crucial. Some individuals may be removed from the Social Panorama altogether or positioned further away.

In general, people who are not perceived on equal terms are felt to be burdensome. Often, they are positioned directly in the coachee's line of focus. It's common for suppressed individuals to emerge only gradually in the process or be placed far away. Asking what it would take for these individuals to achieve equality can be very helpful. Often, additional processes are required to allow for forgiveness, acceptance, or reconciliation.

To help your coachee find a balanced position, offer adjustments by moving the floor markers in the room. Each change in position should be introduced with the question, "Is this position better, worse, the same, or just different?"

After some adjustments, most coachees begin to adapt the model on their own, either asking the coach to move the personification to a certain spot or making the adjustments themselves. Precision is critical here, as even minor deviations in perspective or distance can create irritations.

The process is complete once all personifications have been adjusted as needed, and the coachee can reclaim their intimate space. Invite the coachee to reflect on the transformed Social Panorama as a whole and notice how it feels to engage with it now.

If any irritations arise, the process should be continued.

### **Step 5: Ecological Check and Future Pace**

To conclude, ask your coachee to imagine interacting with the people in their Social Panorama in this new way, knowing that these individuals will always be present in their mental space as companions. Encourage them to take their time with this visualization. Once the representation is clear, ask what the implications might be for daily life and interactions with these people. Be attentive to any reservations the coachee might have and address them if necessary.

## Summary of the format

1. Mark and calibrate the intimate space.
2. Represent the current situation.
3. Analyze the current situation, including relationships, burdens, and expectations.
4. Balance the Social Panorama through changes, reassignments, or transformations of relationships.
5. Ecological Check and Future Pace.

### 10.2.2 Clarifying Professional Relationship Issues

The professional Social Panorama is similar in approach to the family system.

The key difference is that the professional environment is a far more hierarchical system where eye level holds a different significance, and personifications are typically placed outside the intimate space. When individuals appear within this intimate space, it is often due to longstanding personal connections, mentors who have supported the coachee's professional journey, or due to a conflict.

#### Figure 46: Typical positions of mentors

When a conflict exists in the workplace, we frequently observe personifications within the intimate space, positioned directly in front of the coachee and often appearing larger than life. Such individuals may feel overwhelming or intimidating. These representations can also symbolize issues, such as projects, which appear as insurmountable obstacles impacting one's life and development. If the coachee can move this personification aside and at a distance, it often reduces its perceived significance, recontextualizing its role in the coachee's life.

#### Figure 47: Oversized Personifications in Focus

### 10.2.3 Leadership Panorama

Browsing through business forums on social media, one might easily get the impression that everything would be fine if only leaders knew how to do their jobs perfectly. What is often overlooked, however, is that leadership is not a one-way street. Effective leadership centers around the art of building a team that collaboratively works toward common goals.

Leadership is, first and foremost, a temporary alliance focused on leveraging skills toward specific objectives. This description emphasizes the notion that respect—not love—is the core of workplace relationships. Unfortunately, this distinction is often blurred, leading to misunderstandings and imbalances in professional relationships. We frequently see that a leader's professional Social Panorama overlaps significantly with their personal one, leading to communication patterns shaped by this mental-spatial perception.

The Leadership Panorama aims to balance the role of a leader within the social system of the workplace, helping both the leader and team members clarify their roles.

Start by working with the leader to create a Social Panorama that clearly separates professional and personal relationships. Represent professional and personal figures in the panorama with different colors. This clear separation makes it easier to identify which relationships are relevant to which roles. Both panoramas should be calibrated to ensure that there is eye-level positioning and that beliefs associated with each relationship are examined.

Figure 48: Separation of Professional and Private Social Panorama

The Leadership Panorama emphasizes the importance of clearly defining and placing team members and supervisors in positions that support self-efficacy and encourage role clarity.

If a leader draws people too close in the Social Panorama, they may feel micromanaged or may overly rely on the leader for every decision.

Thus, in the Social Panorama, leadership entails:

- Establishing a firm and confident presence as a leader.
- Promoting self-efficacy, setting clear expectations, and reinforcing the leader's role.
- Periodically recalibrating one's position in the Social Panorama to reinforce role clarity.
- Sharpening the team's focus on shared goals.
- Address and recalibrate role violations in the Social Panorama immediately.

The Leadership Panorama, therefore, strengthens role definition and establishes clear boundaries between professional and personal relationships. In our experience, both are very helpful and support managers in marking their roles clearly and respectfully.

In a subsequent step, work with the entire team to establish a collective panorama that clarifies everyone's place in relation to surrounding systems (hierarchies, clients, etc.). This exercise allows the leader to explain each team member's role and importance in the shared panorama.

During this process, each team member should physically occupy their designated position and state their role, such as, "My role is to... in order to achieve our goals and support the team."

This stage tends to be personal for each team member, and thus we encourage participants to conclude by providing feedback to the group, sharing what they personally take away from the experience.

The collective Social Panorama can greatly help clarify role expectations and increase the team's overall effectiveness. In addition to efficiency-focused measures, having a well-functioning team where each member understands their role is often the most powerful and cost-effective strategy.

## **Summary of the Format**

### **Phase 1 – Individual Leadership Panorama**

1. Build a Social Panorama with professional and personal figures.
2. Separate the panoramas into professional and personal realms.
3. Balance each panorama and establish appropriate beliefs for each individual.
4. Define actions for individuals whose positions need adjustment.

### **Phase 2 – Team-Leadership Panorama**

1. Set up the team's panorama, including its relation to the broader system (hierarchy, clients, etc.).
2. Balance and clarify the position of each role in relation to the leader.
3. Define each role based on shared objectives.
4. Each team member takes their position in the room and states their role: "My role is to... in order to achieve... for the team."
5. Conclude with mutual feedback: "What I take away from this is..."

## **10.2.4 Reducing Stress Through the Social Panorama**

Stress is also a mental-spatial and neurobiological phenomenon. To experience stress, we need internal concepts that drive and hinder us while making us feel as if there isn't enough space and time in our mental representation of the world.

These concepts cause us to expend significant energy to fulfill existing beliefs. If these beliefs are not supportive for us, we demand a high level of neurotransmitter activity to remain functional. The longer this state persists, the more we feel burdened and exhausted. On a neurotransmitter level, this exhaustion grows until, in extreme cases, the body can no longer function, leading to burnout. The underlying cause is the depletion of neurotransmitters, which, at extreme levels, can result in an inability to act.

If we don't literally create enough recovery spaces, neurotransmitters cannot recharge. Sleep is the only opportunity to restore neurotransmitters. We can observe this at the threshold between wakefulness and sleep when we see or do things in our dreams that do not exist in reality. This

happens because inhibitory neurotransmitters, which in waking life help us finish one thought before starting another, are inactive during sleep. This phase is essential for the brain to carry out cleaning and processing. When stress levels are too high, both sleep duration and quality suffer, shortening the period in which neurotransmitters can recharge.

In waking life, neurotransmitter exhaustion becomes evident when we can no longer think clearly, lose our sense of control, or feel that things are slipping away.

If you want to apply these insights in coaching, we suggest supporting your coachees with the following aspects:

- Educate and transform limiting belief concepts to reduce daily neurotransmitter consumption by relieving mental strain.
- When working with trauma, move toward the point before inhibitory neurotransmitters could become active (see Chapter 6.10).
- Create relaxation spaces to allow neurotransmitters to recharge.

The more resourceful our inner concept is, the more balanced our neurotransmitter demands and regeneration will be.

The previous chapters illustrate that the Social Panorama is well-suited for organizing not only people but also issues, giving each an appropriate place. Stress is a mental-spatial phenomenon, often involving not just people but also topics that take up significant space and dominate focus. If a person's mental space is fully occupied by issues or people, blocking any free view, this often results in feelings of overwhelm and hopelessness.

The Social Panorama allows you to sort and place these people and topics step by step, so they either lose importance or are positioned in a way that enables sequential processing.

In the first step, place all relevant issues and people in your coachee's three-dimensional space. For each person or topic, ask how large the coachee perceives them to be and what significance they hold. Use floor markers in the coaching process to simplify, noting each person or topic's direction, size, and possible height in the space. This is particularly important for stress-related issues, where the sheer number of topics can be overwhelming, taking up focus or figuratively "breathing down the person's neck."

This approach follows the same procedure as clarifying family systems (see Chapter 10.2.1).

### **Summary of the Format**

1. Mark and calibrate the intimate space.
2. Present the current situation and note height, position, size, and direction on the floor markers.
3. Analyze the current situation.

4. Balance the Social Panorama.
5. Perform an eco-check and Future-Pace.

### **10.2.5 Grief Work with the Social Panorama**

Grief is a process of processing and coping. It is essential to remember that grief requires time for processing, regardless of the technique used. The format presented here can actively support this process. The problem with grief is that it often overwhelms people, filling their entire perceptual space, mentally enveloping them in grief. This can lead to a state of paralysis. While some people gradually release themselves from this encompassing grief on their own and find new paths for the future, others remain trapped in it. This form of grief work is designed for those individuals.

A significant phenomenon in grieving is that the deceased person or the person separated from fills the grieving person's entire space. Wherever they turn, they only see this lost person.

Without a strategy for processing this grief, the grieving person may unconsciously integrate the deceased into themselves. This can be envisioned as if they are carrying the deceased person within. This phenomenon is often supported by beliefs such as feeling one must replace or not let go of the deceased. A common belief is that letting go of the person means they would be gone forever. Mentally, this leads to a situation where new or existing relationships in the grieving person's life cannot find space—essentially, the space is occupied by the “deceased.”

#### Figure 49: When grief cannot be let go

The essential step here is to assign a new place to the person being mourned in the grieving person's Social Panorama. This might even mean several places, as this person may represent different aspects in the grieving person's life.

To facilitate this process, invite your client to let go of the deceased from within themselves through a ritual of release. This ritual could involve, for example, the client expressing gratitude to the deceased for their lifelong support or the love they shared. In our experience, this helps clients give the deceased a place in their Social Panorama, where they can observe them from an external perspective.

In the next step, support your client in finding a place in their Social Panorama where they can remain resourcefully connected with the deceased. Often, beloved individuals are positioned at the edge of the client's field of vision, so they can serve as a guiding star for future life. Another common placement is at the edge of the Social Panorama as a companion. A third common placement is behind the client as a mentor, about half a meter back, offering a felt sense of a hand on their shoulder as support.



The common factor in all these placements is that they end the client's envelopment by the deceased, allowing space and vision to reopen for their own life and future. At the same time, the deceased gains not only a new location but also a new supportive role for the future.

It is crucial that this newly freed space be filled with something that gives the client's life a new perspective. If this does not happen, the original model of grief remains more attractive, as it may have provided a sense of meaning for a long time.

### **Summary of the Format**

1. Mark and calibrate the intimate space.
2. Identify the location of the person being mourned.
3. Through a ritual of respect, bring the deceased person into the room in such a way that they can once again be observed externally.
4. Find one or more new locations for the person being mourned and assign them a new role.
5. Identify new tasks and goals that fill the now-vacant space within the client's self.
6. Conduct an eco-check and Future-Pace.

## **10.2.6 Advanced Reimprinting in Mental Space**

As mentioned earlier, timeline work is one of the most effective methods of change work for trauma. In NLP, reimprinting is among the most effective tools for clarifying trauma. Unlike Change History, reimprinting includes the social environment in the clarification process. In our coaching practice, we help clients set up a Social Panorama at the location of the trauma. The advantage is that the entire social environment of the client can be included in the process. At the same time, participants—especially those who contributed to the trauma—can be reassigned new positions within the Social Panorama. This greatly aids trauma processing and provides a deep clarification of the imprint.

### **Figure 50: Reimprinting with the Social Panorama**

Through the Social Panorama, all individuals and threats in the personal perception space of perception can find a new, appropriate place. This also enables changes to beliefs about the traumatic events. Note: As with all other NLP formats, the goal is not to minimize or “erase” an event but to provide clients with a processing strategy. Reimprinting allows them to walk through the traumatic event on their timeline without carrying the trauma through their lives daily.

The primary objective is to make the event, regardless of how traumatic it was, a memory. It finds its rightful place on the timeline instead of being ripped out and relived by the client daily.

To access the imprint, it is essential to ask clients about the submodalities associated with the recurring experience. This is critical because the causal chain leading to the trauma's original imprint is often inaccessible cognitively. However, the submodalities of perception tied to the

recurring sense of shock are always accessible. We recommend following the pattern of specific submodalities rather than focusing solely on the client's verbal accounts.

At the beginning of the process, clarify the specific submodalities that consistently trigger the shock. This can be done through a brief VAKOG (Visual, Auditory, Kinesthetic, Olfactory, Gustatory) query. Ask the client to stay connected to these submodalities and, associated with the present, to move backward on the timeline and pause at points where the same submodalities reoccur. If possible, ask them to briefly name the event and/or approximate age.

After a few steps back along the cognitive chain, clients usually go into a timeline trance, gradually encountering events that share the same submodalities but may not have been consciously remembered. These steps continue until the original imprint is reached. This imprint is identifiable by its intensity for the client and the certainty of whether there was an earlier event with the same submodalities.

At the imprint point, ask the client to step off the timeline and view the event from a dissociated, adult perspective. Invite them to connect with their younger self from this external position. Based on our experience, the first message the adult self often needs to convey to the younger self is that they will survive the event, as the younger self lacked a processing strategy. Next, determine what specific resources the younger self would have needed in that moment to handle the situation. This is not just about providing some missing resources but about establishing a genuine processing strategy.

To facilitate this, set up your client's Social Panorama at the imprint point, reflecting the social reality that shaped them at the time. By dissociating and rebalancing the Social Panorama at the imprint location, the entire relationship system as experienced by the client at that time can be reorganized. Often, beliefs tied to the individuals involved are connected to this. Repositioning these people and determining where they can be encountered at eye level often allows clients to find peace with the trauma and establish resourceful beliefs that are supportive.

As previously mentioned, it's essential to remember that the goal is never to downplay a traumatic situation but rather to use the knowledge of the adult self to establish a processing strategy, allowing the trauma to become a memory that can rest appropriately on the timeline.

Once this is achieved, have your client step onto the timeline about one to two years before the imprint event. Mark this position with a floor anchor as well. This is crucial, as the new insights can now be established at a point where the traumatic event has not yet occurred. Invite your client to proceed through the originally traumatic event equipped with the new understanding from the Social Panorama and their resources. In our experience, reimprinting often leads clients to no longer experience the original event as shocking or traumatic.

With the original event behind them on the timeline, clients move through their life events in the space step-by-step, testing how they experience these with their new insights. Typically, all events are newly evaluated and no longer experienced as shocking.

Arriving in the present, clients are then encouraged to take a determined step into the future, anticipating what may now be possible. We often add the visualization of observing themselves on their future life path to see what possibilities this newly organized experience can create. A VAKOG trance can assist with this.

Back in the meta-position (e.g., the discussion chair), clients usually need some time to complete processing their insights before an eco-check and Future-Pace can be conducted effectively.

In our experience, reimprinting is highly effective for trauma because it enables clients to access the trauma's origin through their submodalities, which are otherwise inaccessible through cognition alone.

The format again highlights the brain's flexibility, which is unaffected by whether the timeline is real or traversed in a waking trance using floor markers. The brain's spatial orientation works either way, aiding clients in finding solutions that remain stable.

### **Summary of the Format**

1. Establish rapport and clarify the topic that repeatedly causes shock from a meta-position (e.g., discussion chair).
2. Identify the submodalities associated with the shock.
3. Set up a timeline in the space with at least three floor markers (present, birth, and future).
4. Have the client stand at the present point on the timeline, facing the future, and ask when and where the discussed submodalities last occurred. This can be guided with a simple VAKOG trance. Invite the client to move backward along their feelings associated with these submodalities.
5. Briefly inquire about the event or approximate age (language is often reduced if it involves early childhood experiences).
6. Repeat steps 4 and 5 until the imprint is reached. Clients usually know if there was an earlier event with the same submodalities.
7. When the imprint is reached, ask the client to describe the event and context as much as possible. This may take time as the brain typically releases these inner representations gradually.
8. Ask the client to step off the timeline. Important: Set a clear separator here and create sufficient distance from the imprint so that the client is fully dissociated.
9. Establish contact between the dissociated adult self and the younger self: "When you see your younger self there, what would they have needed in that moment?"
10. Make an offer: "Maybe you can give your younger self a message. You might say, 'I am you, and I am living proof that you will survive this. Because we are one.'"
11. Establish the Social Panorama at the imprint and determine which people were relevant at that time. Lay floor markers for these people, including their direction. Inquire about beliefs tied to these individuals.

12. Step-by-step, rebalance the Social Panorama by repositioning and orienting the individuals involved. Continue this process until the client finds true peace. The beliefs associated with these individuals should also be resourceful for the client.
13. Have the client step onto the timeline one to two years before the event and proceed slowly up to the present, gathering experiences.
14. Encourage the client to take a step into the future to feel the impact of the new insights.
15. Step off the timeline and return to the meta-position, allowing time to settle.
16. Perform an eco-check and Future-Pace.

### **10.2.7. Balancing Personality Parts**

Every individual has various facets and roles in life. Depending on the situation, role requirements, or emotional state, different aspects of our personality become active. For instance, we are a father, mother, sibling, employee, leader, partner, friend, etc. In terms of personality parts, some parts are confident and maintain an overview, others are fearful, and some are focused on stopping us from taking any initiative, such as engaging in sports (the “inner slacker”).

Sometimes, a minor trigger is enough to unbalance us, even if we were relaxed just a moment before. Each personality part within us developed because, in certain contexts and with the knowledge available at the time, we aimed to exhibit the best possible behavior. Rarely do we consciously decide what the best behavior is; rather, we act based on what is available at that moment. Behavioral patterns emerge when our brain attempts to retrieve familiar patterns in seemingly similar situations. This happens even if, upon reflection, the behavior no longer seems appropriate. This often leads to self-criticism or attempts to justify our actions. In any case, what remains is a sense of irritation that is usually not easy to resolve.

The Social Panorama offers a way to position these different parts or roles within the room and engage in a dialogue with them.

Ask your client to first establish a place for themselves. Mark their intimate space to provide a frame of reference for proximity and distance, making it easier for them to calibrate their position within the system.

Invite them to place each role or personality part in the room and explore what attributes they assign to each role or part. Clarify with them what positive intentions each part brings. To check this, have the client “step into” the role or personality part by moving to the assigned spot and looking back at where they previously stood.

This frequently leads to new insights and perceptions of the client’s overall personality, often even changing the spatial positioning of the respective roles or parts.

Figure 51: Different Roles in the Social Panorama

It is essential to first acknowledge that every personality part has its right to exist and aims to achieve something beneficial for the whole personality. If we try to exclude or ignore a part, it will assert itself more forcefully. Trying to compensate for it with another part will only lead to conflict. For instance, using the parent role to fill a leadership role is likely to create friction.

A crucial step, therefore, is to clearly define and honor each personality part and role with its positive intentions and behavior patterns. This allows appropriate behavior patterns to be assigned to the specific personality parts, enabling the client to experience clarity and strength in each role.

Invite the client to return to their central self after calibrating each part and reintegrate this aspect. Often, this leads to further adjustments in the spatial positioning of each part.

Gradually, all parts become clearly defined, calibrated in perception, and then reintegrated into the whole self. In our experience, this integration process often leads to a profound sense of inner harmony, as each part has its proper place and can support the others. This process clarifies that, in specific roles—such as a parental role—certain behaviors are necessary that might differ from those expected in a leadership role. The stance required in a leadership role may activate a side of us that we might reject in other contexts.

### **Summary of the Format**

1. Identify distressing roles or personality parts.
2. Mark the center of the Social Panorama and the intimate space.
3. Assign each part a place in the room and describe them.
4. Clarify the positive intentions of each part.
5. Move to each part's spot, allowing the client to experience being that part. Clarify the distinct positive role of each part.
6. Return to the central self in the middle and integrate the insight.
7. Repeat steps 5 and 6 until all parts are defined and integrated.
8. At the center, experience what it feels like when all parts have clear roles and places.
9. Conduct an eco-check and Future-Pace.

## **11. Designing Goals Means Shaping Time and Space**

### **11.1 Introduction**

Goals are deeply spatial experiences. If we don't have a clear, sensory-specific image of what we want or need to achieve, it is hard to take action. When goals are vague or hazy, motivation is nearly impossible. This means that goals must always be positive in nature for the person aiming to reach them. Comparative goals like "I want to be better, thinner, stronger, etc." are insufficient since a positive state cannot be mentally visualized. The same applies to negative

goals, like “I don’t want to...,” which are not particularly motivating as there is no internal image of what the goal looks like once the problem is resolved. The same applies to hidden negative goals, such as “We need to increase revenue...” Measured by what? How can participants recognize that enough revenue has been generated? Here, the hidden negative goal is a current insufficiency in revenue—a negative goal the company seeks to avoid.

The problem with negative goals is that they may initially sound logical: “I don’t want to be overweight anymore,” “...not drink alcohol anymore,” “...never experience another pandemic,” or “...never be so sick again.” But our brain does not process words like “no,” “not,” “never,” or “nothing.” It reacts only to the image of what we no longer want, and the stronger our desire to avoid something, the stronger that inner image becomes. The more we try to move away from the negative image, the larger the shadow that follows us.

Following the NLP framework for well-formed goals<sup>67</sup>, our brain needs a clear image of the objective. What is there to see, hear, feel, smell, or taste when I have achieved my goal? If this image is attractive and motivating enough, our entire internal system will orient toward it.

Generally, larger goals should align with the individual’s reward system to create a connection with the goal over the long term.

For goals involving more than one person, it is crucial that each participant has a clear vision of the objective. Additionally, the goal must align with each participant’s reward system, fostering connection and commitment. This explains why some company-mandated goals fail in practice when corporate leadership neglects to engage managers and employees. Therefore, such imposed goals are often resisted internally.

## 11.2 Coaching Formats

### 11.2.1 Balancing Your Timeline

*“A hamster wheel looks like a career ladder from the inside.”*

—Unknown Author

As previously discussed, there are different forms of time perception (Chapter 6.9). Building on these basic models in mental space, we have our own individual sense of past, present, and future, often perceived unconsciously. This perception profoundly affects our understanding of time and what we feel is possible within a certain timeframe. Some people feel “time is breathing down my neck” or “slipping away,” while for others, it passes “at a snail’s pace” or even “stands still.” Similarly, how much time we feel we have left in life significantly influences how we handle time.

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<sup>67</sup> Padberg, 2010

If, for example, we believe we have sufficient time to reach a goal, we're more likely to pursue it. Conversely, we may abandon the goal if we feel it cannot be achieved within the available timeframe.

Since perceived time (internal time orientation) follows an individual concept, distinct from measured time (external time orientation), pressure can quickly distort this perception, adding considerable stress.

Many stress-related phenomena are linked to a distortion of the timeline, which can limit confidence in one's abilities. The purpose of this coaching format is to allow individuals to clear up distortions in their sense of time.

Let's take the example of C. C. wants to change his career but has repeatedly failed to act on his intentions. He describes himself as "ambitious," "goal-driven," and feels "internally driven" and "constricted," yet he consistently misses the chance to apply for attractive job openings. He works for a large company and values the financial security it provides. His goal is to make a career leap and create more room for personal growth. In the intake interview, he often speaks of feeling "driven" and is extremely dissatisfied with his situation.

To determine where the distortions in the internal time model lie, Dr. Lucas Derks' Timeline method can be helpful, as experiencing time in three dimensions allows clients to identify what hinders them from reaching certain goals and what time concept would be more supportive.

1. **Step 1:** This format begins by helping the client become aware of how they perceive their timeline regarding the present, future, and past. To illustrate this, you can offer them three virtual balloons representing these points in time.
2. **Step 2: Mapping the Current State:** The client starts by placing their virtual "present" balloon in three-dimensional space, naming its size, color, position, height, and distance from them. Ask the client how large they perceive this present balloon to be and what that size signifies regarding their current situation. Once positioned, the client expresses the belief connected to this balloon.
3. **Step 3:** The client then positions the past and future balloons using the same questions as in Step 2, creating a kind of virtual timeline.
4. **Step 4:** Beginning with the present balloon, you can suggest moving its position, height, and distance. At each step, ask whether the new position feels "better, worse, the same, or simply different." This simple questioning technique ensures the coach doesn't interpret the positioning, while allowing the client to assess how each new position feels. Even with highly rational clients, this process quickly reveals that only specific positions in three-dimensional space feel genuinely good. Once the ideal position is found, ask the client again about their associated belief, which usually changes significantly.
5. **Step 5:** Invite your client to adjust the positions of their past and future balloons. Give them time to recalibrate, especially for the future timeline. Clients who set goals that are too high or too close often experience a steep or overly immediate timeline, limiting their ability to reach their goal confidently. Help your client determine the best course of the timeline to make their goals feel achievable.

6. **Step 6:** With all balloons correctly positioned and associated beliefs identified, the next step is to interpret this new model. In some formats, the eco-check and Future-Pace steps are brief, but in this one, exploring the implications of the new time model often takes longer than the actual setup, as a whole life plan may hinge on this altered perception of time.

After the exercise, C. decides to pursue an entirely new career path. The coaching reveals that his focus on financial security had distanced him from the enjoyment he once found in music. He also realizes how much time he has left to shape his life. Now, he's inspired to go into business for himself and reclaim control over his time.

Figure 52: Reorganizing the Timeline

### **Summary of the Format**

1. The coach introduces three symbolic colored balloons, one for the present, one for the past, and one for the future.
2. The client describes each balloon's size and perceived distance. With each position, they express the belief linked to it.
3. The coach offers spatial repositioning of the balloons, asking, "Better, worse, the same, or just different?" The balloon is "fixed" once the client is satisfied with its placement. This may lead to a complete repositioning of the timeline.
4. After positioning, the client shares any new beliefs tied to the balloon's placement.
5. When all balloons are positioned, conclusions about the new time model are drawn. Additional coaching formats may be needed to develop the newly possible pathway.
6. Finally, explore the consequences of these insights and whether the client is prepared to commit to them in their future life.

### **11.2.2 Goals as a Time-Spatial Concept**

The quote by Helmut Schmidt, "If you have visions, you should go to the doctor ...," is fundamentally wrong in many ways. Only through our imagination are we able to project successes and events into the future. This ability allows us to envision goals and aspirations in a positive sense. Negatively, it can also create frightening scenarios and fears in mental space, which can severely paralyze our capacity to act. When we imagine large goals and simultaneously hold limiting beliefs that classify these visions as unreachable, these perceptions can be restrictive.

**The basic rule is: What we cannot imagine, we cannot focus on, and thus we cannot strive for it.**

To accept a goal for ourselves, we need a sensory-specific image of what it will be like to achieve it. The clearer a goal is and the stronger the positive emotions associated with this vision, the greater the desire to achieve it.



Although in our society the ability to be a “dreamer” is often negatively connoted, this very ability contains the competence to fuel goals with the energy needed for successful realization. For this energy not to dissipate, however, something else must be added: the competence to plan goals spatially, organizing individual steps and milestones into manageable, achievable sub-goals.

Through mentally connecting sub-goals via floor markers, a chain of anchors is created that helps clients experience the achievement of each sub-goal as a sensory-specific physical experience. This bodily experience of reaching a sub-goal is what helps the reward system in our brain to realize even large goals step by step.

Without such a vision, many people experience their lives as slow, aimless, and often frustrating. Having nothing to commit to or find meaning in can lead to a state of boreout over time. Such individuals describe their lives as “chronically boring,” “I feel perpetually under-challenged,” or “I’m bored to death in my job.” It’s no wonder that, as with burnout, illnesses can result from this.

For goals to be effective, they need to be experienced through all sensory channels in mental space and especially as a bodily sensation to activate the goal.

In individual coaching, many clients come to us when they lack goals. While this can be relaxing in the short term, many people become restless without a life focus within a short time. “Business as usual” is often carried on for a while, but this only lasts so long, and then internal restlessness increases.

To regain focus, it is essential to create a vision of our future self that is large and attractive enough for us to want to reach it (see Chapter 9.2.4). Only when the emotional relevance is strong enough is there a reason to embark on this path and develop a plan for how to achieve it. Otherwise, our reward system doesn’t get enough “fuel,” and as mentioned in Chapter 6.4.1, this memory system is the key player in any process of change.

The following example concerns the development of fundamental life goals. These questions are often posed to us as coaches, especially when people are at thresholds in their personal development.

The first step in goal setting is therefore to create an image of a future self in space together with your client. The accompanying questions might be, for example:

1. “What do you want to see more of in the world and what less of?” or: “What do you want to stand for when you look back in, say, 10 years?”
2. “What is the ‘unique’ benefit you want to contribute to make this possible?”
3. “What is your mission?”
4. “With which role can you best fulfill this mission?”

5. Let your client experience the goal in VAKOG (all sensory channels) and enhance the body sensation with the invitation: “And perhaps you’d like to spread this feeling throughout your entire body ... from head to toe.”

Certainly, other questions can be used here. The core question is: “Who do I want to be in the future?”

In a second step, it’s about making this goal achievable for your client.

#### Figure 53: Relevance of Concrete Goal Representation

The easiest way to achieve this is for clients to walk backwards through their timeline from the achieved goal state, using floor anchors. This way, every milestone can be experienced starting from the goal. This creates a sequence of bodily anchors, giving clients the secure feeling that the sub-goals are also attainable. It’s important to address each milestone through sensory channels and then individually reinforce it with body anchors.

However, goals can only be pursued and consistently followed if the client’s belief system supports them. Therefore, it’s essential to ask what might hold the client back. Here, the “target identity” format (Chapter 9.2.4) can be effectively integrated. In this way, not only is the goal itself clearly defined, but possible obstacles that could hinder goal achievement are also clarified.

#### **Summary of the Format**

1. Lay out floor anchors for the present and the future self.
2. Ask the client to associate themselves with the future self and answer questions about their future identity there.
3. View this target personality from the outside and ask, “How did you get there? What was the last crucial step before this?” If there are any reservations in the client’s personality about allowing themselves this goal, the target identity format (Chapter 9.2.4) should be carried out first.
4. Reassociate them with the future self and let them identify the last step important for reaching the goal as they walk backwards on the timeline. Let them experience each sub-goal from the perspective of achievement.
5. Repeat steps 3 and 4 until they reach the present.
6. Now, have the client review each milestone once more, making any necessary adjustments or additions until the image feels achievable and aligned for them.

#### Figure 54: Developing Goals in a Team

The format works equally well for an entire team. When a team is tasked with or wants to achieve a goal, each team member must be able to connect to the goal individually. If this does

not happen, there is no “team spirit.” This beautiful spatial term of a common spirit precisely reflects the essence: connecting with the mental space of others to achieve shared goals.

To ensure this succeeds, it’s essential to guide all participants through this goal format together. What will be seen, heard, and felt when the team reaches the goal? The first step is to distill what the common goal truly is that all participants can agree on. If different interpretations exist, it’s crucial to clarify until the goal is emotionally relevant to everyone. Without this agreement, the entire process would be mere performance, as those who don’t connect with the goal would emotionally disengage immediately.

This goal clarification phase is treated too superficially in many group contexts. As a result, goals often fall apart shortly after being set or can only be sustained with high effort. In numerous projects and change processes, this leads to failure or economic loss.

### **11.2.3 Strategies as Spatial Concepts**

As described above, the visual image and emotional significance of a goal are essential prerequisites for us to pursue it. When we consider an entire strategy, however, it involves more than just the goal image; it encompasses a complete goal scenario. This scenario includes the path to reach the goal and ensuring that all involved are appropriately engaged along the way at the right times. For strategies to be successful, it’s also important to recognize that a strategy exists because we aim to develop ourselves by achieving strategic goals. This means that a developed self-image (see Chapter 9.2.4) is necessary for a successful strategy, whether for individuals, teams, or entire organizations<sup>68</sup>.

The biggest mistakes we observe in strategy work as project coaches are when what is to be achieved together isn’t defined in a sensory-specific way. Moreover, the participants do not see themselves in the strategy—they can’t integrate their ideas, resources, talents, and values into it. If we cannot identify with the goal image, it is difficult to generate energy toward the strategic goals. Often, there is also a lack of systematic review of the consequences associated with achieving the goal, including the personal implications of the journey and the achievement of the strategic goals.

For a strategy to function effectively, the following elements, among others, need to be developed:

- Establishing a well-formed goal scenario in which all participants can identify themselves
- Developing the self-image of the team and its members. Who can best contribute their skills, talents, etc., in which way?
- Breaking down the overall scenario into sub-scenarios for each participant
- Systematically examining the consequences of the goal scenarios

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<sup>68</sup> See also Padberg, 2010

- Creating a shared timeline, as well as individual timelines for each participant
- Building an agile goal management system that includes a sponsor (a decision-maker who genuinely supports the goal) and a coordinator who drives the goal attainment process forward
- Setting up a system for regular status assessments and adjustments to remove obstacles from the goal achievement process; this is intended as a solution-oriented goal management system, not fault-oriented controlling

### **11.2.3.1. Building the Goal Scenario**

To lay out the goal scenario in physical space, it's important to establish one or more timelines. This means setting a fixed point on the floor for the "here and now." The coach then asks where in the room the point lies that represents the successful completion or implementation of the strategy. This applies to both individual and team strategies. Here, we describe a team process that can also be applied individually.

Through the spatial anchors established with the team, intuitive images are formed in participants' minds, creating a sense that something identifiable must exist at that point.

This is the second step in this process: together, the team moves to the endpoint and describes what has been achieved both individually and collectively and how they recognize that the strategy is a success. This description should be as sensory-specific as possible. We often write these insights on floor markers and lay them around the endpoint. Through team discussion, a shared goal image gradually emerges. As a coach, it's important to ensure that everyone has a voice here. If even one participant does not see themselves in the goal image, this creates disruptions that can ripple through the entire process.

Figure 55: Team Goal Scenario

### **11.2.3.2 Developed Self in the Goal Scenario**

Once the goal image is developed, the next step is to establish commitment to this goal. Commitment only arises when we can also see an opportunity for personal growth in achieving the "strategic" goal. This requires a vision of ourselves as we experience the goal's fulfillment.

To shape this vision, as a project coach, you have two options. For an individual, you can use an imagination exercise of their future self (see Chapter 9.2.4 for detailed instructions).

Figure 56: Team Kaleidoscope

For a group process, a team kaleidoscope can be useful, where each member works on how they will fulfill their role within the goal scenario. The neurobiological levels for each participant are laid out in a star shape around the goal point. This format is called a kaleidoscope because it allows participants to step into each other's roles and perceive what shapes each other's roles.

All participants take their places for their respective roles at the context and environment level. Level by level, the team, under the coach's guidance, moves toward each other, with each person answering the following questions:

- "What does the environment for your role look like at this goal point?" "How will you know the environment has reached a point where the strategy is fully lived?"
- "What are you doing here concretely to advance the strategy through your daily actions?"
- "Which of your talents and abilities do you bring in particular to make this strategy vibrant?"
- "What attitude, beliefs, and values do you uphold to live this strategy consistently?"
- "Who are you for the team?" "What role do you fulfill for the team to succeed?" Playfully, you might introduce a virtual "superhero" for each role to sharpen the role definition, such as Super-Marketing-Woman, Master of Finance, etc.
- At the top is the goal scenario describing the purpose of the project. For example, the team could develop a shared slogan that encapsulates their unity.
- An essential part of this exercise is that with each level, all participants clearly articulate what they feel each other's role should additionally entail. If the group isn't too large, it can be very helpful to associate with each other's role briefly to understand it better. As described in multiple parts of this book, this is one of our brain's unique abilities. Through association, we can create a sensory-specific image of another person in our mental space, deepening our understanding of their role.

### **11.2.3.3. Reviewing Consequences**

Making the consequences of goal achievement transparent early in the process is crucial for everyone involved, so that both the positive and negative outcomes are visible to all participants.

It's helpful to assign these consequences a specific place in the room and to position them along the timeline. This allows everyone to see when each consequence will impact the strategy achievement process. It's essential to ensure that all participants contribute their perspectives on the consequences. Unspoken consequences can act like "ghosts" in the space, disrupting the goal achievement process.

Figure 57: Goal and Consequence Timeline

### **11.2.3.4. Building the Timeline(s)**

Now that the goal scenario exists, the team is aligned, and the consequences are visible, each role involved needs its own timeline to outline how the strategy will be achieved step by step.

As a project coach, you can tape a line on the floor for each role, stretching from "Today" to the target scenario. All participants then work together to discuss and determine: What must be accomplished by whom and by when for the strategy to succeed? Working physically in the

space helps everyone calibrate intuitively to the timeline, aligning the timeframes of each timeline with one another. At the end of the process, adding specific numbers and dates to the milestones creates a strong internal commitment to the timeline. Any new consequences that emerge during this process should be discussed immediately and also placed in the space.

*Figure 58: Building Goal Scenarios*

### **11.2.3.5 Regular Positional Checkpoints**

It is an often costly misconception to believe that setting a goal once is enough and that everyone will work unwaveringly toward achieving the strategic objectives. Instead, it is essential to have regular checkpoints and adjustments throughout the journey.

This can be likened to a ship at sea, where, by tradition, the position is assessed every noon to make course corrections if necessary. Regular checkpoints provide reassurance to everyone involved that they are on the right path and reinforce the sense of working toward a shared goal.

Since this is also a highly spatial mental process, it makes sense to conduct checkpoints in a visible space. At regular intervals, ask the participants to illustrate where each one stands in relation to the project timeline. Team members can position themselves on the project timeline to show what has already been achieved and what they are currently working on. As a project coach, you can further support the team in considering any necessary adjustments.

These “lessons learned” are essential for identifying disruptions early on and creating agreements on how to proceed. The results of these checkpoints should be recorded like a ship’s log. This documentation ensures that the outcomes of each checkpoint and the agreements made are accessible to everyone, both during and after the project.

## **12. Body, Space, and Health**

### **12.1 Introduction to Body Space**

The concept of spatial perception doesn’t end with the external world; it continues within our own bodies. The central nervous system is a 3D spatial structure that connects the brain with the body’s organs and senses. In the 1980s, neuroscience discovered large populations of neurons around the gut and heart<sup>69</sup>. One resulting hypothesis is that these nerve cells have a close connection with the brain. Some researchers even refer to them as brain-like structures in which semi-autonomous intelligent processes occur. The hypothesis suggests that major organs, through these brain-like nerve cells, have an active influence on our thinking and experience.

When examining the language people use to describe bodily experiences, it’s notable how often specific organs are mentioned, such as:

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<sup>69</sup> Gershon, 1998.

- "It's getting on my nerves."
- "It's giving me a stomachache."
- "It's breaking my heart."
- "It's enough to make me sick."
- "My head says yes, my heart says no."

Until recently, most neuroscientists regarded these expressions as merely metaphorical, considering them just phrases. However, when these expressions are taken seriously, they may actually describe processes tied to these organs that are closely connected to them.

Current research increasingly supports the idea of intelligence housed around the major organs<sup>70</sup>. If psychotherapy were to successfully activate these different intelligent regions selectively within various centers of the nervous system, it would revolutionize psychology. According to this hypothesis, the head (Cephalic Brain), heart (Cardiac Brain), and gut (Enteric Brain) could potentially be accessed and engaged separately. This notion is supported by body responses we label accordingly:

"Decisions and dilemmas such as: my head says yes, my heart says no, and my gut doesn't agree" reveal the significant connection between the head, heart, and gut. When caught in such a dilemma or decision, we need high cognitive activity to reconnect and resolve the issue<sup>71</sup>.

Dr. Lucas Derks further explores this phenomenon:

"The distinction between head, heart, and gut implies that these organs may each provide unique input that aids in decision-making. Whether this truly means we can tap into the local intelligence of these central nervous system regions remains uncertain. This ambiguity arises from how easily similar effects can be achieved with other psychotherapy techniques. The ease with which most clients can communicate with any part, body part, or organ makes it difficult to definitively attribute these specific locations to organ-based intelligence. Furthermore, these parts can be 'spatially anchored' within the mental space, making it challenging to determine how distinct head, heart, and gut positions truly are."

According to Oka and Soosalu<sup>72</sup>, the gut-brain handles identity, self-preservation, and mobilization. This hypothesis suggests that "gut intelligence" might extend beyond digestion. Given how quickly disruptions in the gut impact our entire perception and state, this hypothesis is noteworthy. Should this be the case, "preference" and "disgust" could be closely linked to gut perception. While it's certain that complex neurochemical processes around the gut interact with

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<sup>70</sup> Kast, 2009

<sup>71</sup> Derks, 2022, S.67.

<sup>72</sup> Oka & Soosalu, 2012

the brain through the vagus nerve<sup>73</sup>, it remains unproven whether we can fully regard this as gut intelligence.

Expanding on this idea, the heart-brain is thought to govern compassion, relationships, and values. This could explain why people mentally disconnected from their hearts may struggle to understand others emotionally. Moreover, some people report experiencing personality shifts following heart transplants, suggesting a connection between heart and identity.

*“According to cardiologist and psychologist Paul Pearsall from the University of Hawaii, some transplant recipients believe in a “cell memory” that transfers certain psychological traits from the donor. For example, Pearsall describes an 18-year-old heart transplant recipient who felt the donor was a “lover from another time.” While these reactions are rare, in Pearsall’s study, 17 out of 73 participants reported them. Psychologist Brigitta Bunzel found that 6% of heart transplant recipients noted personality changes”.*<sup>74</sup>

Common sayings continue to underscore the special connection between the heart and emotions, love, and passion, with expressions like “Just follow your heart.” Language reflects that the gut and heart often communicate through kinesthetic channels as a “gut feeling” or “heart feeling.”

Some psychotherapists have begun interpreting these metaphorical expressions as signals that the central nervous system sends through major organs.

*“In recent decades, it has not been unusual for psychotherapy clients to engage with their younger or older selves, immune system, hormones, cancer cells, or white blood cells. Psychotherapy involving ‘parts’ likely functions just as well when the symbols used are the brain, heart, and gut.”*<sup>75</sup>

The big question yet to be answered is whether we can isolate the intelligence of a single organ and use its knowledge separately from other parts of the central nervous system.

When we seek to make these insights usable in the coaching process, the primary concern is not which researcher can provide concrete proof but rather how a client represents their connection to their organs. If we assume that metaphorical naming of certain organs is not random but has a real spatial reference in the client’s 3D space, we can work with this representation. Given our current understanding, we can be confident that the organs are closely connected to the brain via the vagus nerve. If this is the case, our 3D perception of the world doesn’t end with the external world but extends into our body space. All work with Mental

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<sup>73</sup> The vagus nerve is a regulatory switchboard between the brain and the organs. It has a calming and balancing effect on various bodily functions and also influences our well-being. As the tenth cranial nerve, it runs from the brainstem in the head through the neck and chest to the abdominal cavity. Along its path, it branches out, running to the heart, kidneys, liver, spleen, and digestive organs—and then back again.

<sup>74</sup><https://www.sueddeutsche.de/wissen/leben-mit-einem-fremden-organ-mit-dem-herzen-eines-fischers-1.908173-2>

<sup>75</sup> Durlinger, 2009 from Derks, 2022.



Space Psychology (MSP) demonstrates that spatial localization of unconscious phenomena helps clients build a connection with these phenomena and engage with them actively. If the phenomenon is perceived in isolation without body reference, engagement isn't possible.

Since coaching in this realm sits at the intersection of medicine, psychology, and coaching, thorough diagnostics are crucial. Coaches must remain aware that coaching in this area is not a substitute for medical treatment but rather supports the healing process mentally.

Without an inner alignment toward health within the mental space, there is no basis for paying attention to our health and well-being in the outer world. Orienting toward a healthy self is essential for becoming and staying physically healthy.

Body, mind, and spirit form an inseparable unit that requires mindful support and care on all these levels. The challenge lies in continually balancing these areas to remain or become healthy. With knowledge of the power of the MSP, we can actively support our clients in this endeavor.

## **12.2. Diagnosis, Organ-Language, and Space**

As demonstrated in various parts of this book, we often unconsciously use language to express our three-dimensional experiences, and this applies to health phenomena as well. In working with these, paying close attention to both language and body language is essential for diagnosis. Many causes of psychosomatic health phenomena are expressed through these channels.

A case in point is one of our clients who suffered from neurodermatitis. P. had been under medical treatment for this condition for years and had managed to keep the symptoms somewhat under control. He came to coaching looking for a fresh approach to his business context. In the first session, it became clear that he often said, "This job doesn't suit me anymore; I just feel trapped in it..." When I asked why he didn't simply change jobs, given that his qualifications were highly sought after, he replied, "My mother would say it's irresponsible to take such a risk." As he spoke, he was unconsciously scratching his forearm. The phrase "My mother would say..." appeared several times afterward, always accompanied by the scratching. I spontaneously asked if he might be "in the wrong skin" and if it might be time to shed this old skin.

He looked at me in shock, then cautiously nodded. When I asked if he might be "in his mother's skin," he grew pale and nodded again.

In the process that followed, I asked him to stand in the room and define a place for his mother. P. made a gesture with his arms around himself and said, "She's always around me and controls my life." I invited him to perform a ritual to step out of his mother's skin. I suggested: "Perhaps you'd like to thank your mother for all her care and attention, and then tell her: From here on, I'm

going my own way, and I'm giving you a new place that suits your role today, because I am me, and you are you."

After P. spoke these words aloud, he took a step as if stepping out of something and found a new place in the room. I marked the original position with a floor marker labeled "Mother." Standing in his new position, P.'s skin went through several color changes. His breathing became deeper and more even until he said, "I suddenly feel so free, as if I had been carrying a huge burden." Over time, he moved his mother's position a few more times until he could meet her at eye level.

When P. returned a few weeks later, his neurodermatitis had almost disappeared, and his manner of speaking had also changed significantly. He now spoke more confidently, and his voice was noticeably deeper.

A deeper look into this topic reveals countless expressions addressing bodily health issues. In Mental Space Coaching, the so-called organ language is thus a valuable tool for diagnosing and supporting clients with health issues.

People often describe these issues metaphorically<sup>76</sup>. It seems that the unconscious mind takes a person's expressions literally, potentially exacerbating symptoms suggested by their language.

Robert Dilts, in his book *\*Identity, Beliefs, and Health\**, emphasizes the importance of organ language in health coaching and offers several examples:

- **Skin Problems:** Have you made a rash decision and gotten a rash? Are you itching to move forward? Is something getting under your skin? Do you feel like you're scratching to make a living?
- **Ulcers/Stomach Problems:** Is something gnawing at you? Is someone or something making you sick? Are you unable to digest something? Is something upsetting your stomach? Does what you're currently doing require a lot of guts?
- **Migraines/Headaches/Neck Problems:** Is someone or something getting on your nerves? Are you repeatedly diving headfirst into the same problems? Do you feel burdened by the weight of the world? Do you sometimes feel like you're out of your mind? Are you bearing constant blows?
- **Weight Issues:** Are you waiting on something? Are certain parts of you wasting away, or is everything turning into "love handles"?
- **Vision Problems:** Is there an issue you don't want to face? Is there something going on that you refuse to see? Are you in complete disagreement with someone's behavior?
- **Constipation:** Are you always holding back? Keeping a tight rein? Clinging to your problems? Do things never seem to go smoothly?
- **Heart:** Is something or someone breaking your heart? Are you experiencing heartache after heartache? Are you heartbroken? Are you doing things half-heartedly?
- **Hemorrhoids:** Is something or someone simply a pain in the butt?

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<sup>76</sup> Dilts, 2015, Pg. 136 ff.

While in medical practice, it is often necessary to treat the symptom directly, Mental Space Coaching focuses on providing a new mental-space approach to engaging with the issue.

## 12.3 The Healthy Self

A vital first step toward a healthy self is a healthy self-image. As long as we lack a vision of ourselves as healthy, it is difficult, if not impossible, to mentally pursue this state (see also Chapter 9).

But what does a healthy self-image entail? In our culture of optimization and self-improvement, it sometimes seems that physical perfection must be part of a healthy self. But that's not the goal. A healthy self arises where we bring all areas of our lives and personality aspects into balance so that body, mind, and spirit are in good contact with each other and remain so.

This phrasing illustrates that health is not a state but a process of balancing our lives and ourselves to stay grounded while powerfully addressing life's challenges. It means that, in the face of both self-demands and external expectations, we can continually align our life areas well with each other (see also Chapter 9.2.3 on Core Identity).

Research on resilience reveals a proven link between mental and physical health. A metastudy from the German Medical Journal (Deutschen Ärzteblatt) highlights this relationship:

*"...In terms of medical psychology practice, this study provides insights into targeted psychosocial support for managing physical illnesses. The appearance of psychological stress symptoms increases the need for care within medical treatment and can affect the course of illness and treatment success. Screening for resilience levels could help identify people with low resilience early on in clinical practice, allowing for greater support through external resources during treatment."<sup>77</sup>*

Targeted resilience support thus clearly leads to better treatment outcomes in physical illnesses.

### Figure 59: Resilience Factors

Taking this as a basis, supporting the development of a healthy self-understanding forms a strong foundation for physical health and the ability to become and remain healthy. The resilience factors outlined above show how much resilience depends on factors that need to be established in our mental space to form a healthy, resilient self. The internal image of one's health is thus one of the essential supportive factors in achieving and maintaining health.

This means that in coaching, it is crucial to help clients develop a "healthy" self-image across all resilience factors. Establishing this self-image in the spatial context is a particularly constructive approach to addressing health issues directly and facilitating real mental changes.

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<sup>77</sup>

<https://www.aerzteblatt.de/archiv/200707/Zusammenhang-von-Resilienz-und-psychischer-Gesundheit-bei-koerperlichen-Erkrankungen>

To establish such a "healthy" self-image, the following elements are necessary:

- The client's commitment to take responsibility for their health.
- An imagination of a fully healthy self across all neurobiological levels of perception in the spatial context.
- Development of a resilient self-image for all resilience factors.
- Creation of concrete strategies and actions conducive to health.
- Willingness to make the necessary lifestyle changes.

## **12. 4 Coaching Formats**

### **12.4.1 Designing a Healthy Self-Image**

Our self-image is a complex internal representation of our personality, composed of various sensory impressions established in our mental space as internal images, colors, sounds, movements, submodalities, beliefs, and behavior patterns (see Chapter 9). To bring about a change in one's health perception, it is necessary to reshape these facets of the self-image in the mental space to create an individualized "healthy" self-image. Since chronic health issues are often deeply intertwined with one's identity, it's crucial to reshape these facets with respect and to verify continually whether these changes truly align with the client's personality.

This format addresses each facet individually.

#### **Step 1: Preparation**

1. Identify symbolic submodalities for the concept of "health."

If "health"...

- ...were a color, what would it be?
- ...were a sound, what would it sound like?
- ...were a movement, what would it be?

2. Elicit submodalities associated with credibility or "reality."

- a) Perceive yourself in a neutral past situation or activity, and inquire about the submodalities.
- b) Imagine the situation unfolding differently, then ask about the submodalities again.
- c) Conduct a contrast analysis of the submodalities. Which are decisive for "reality"?

## Step 2: Imagining the "Health Image"

Developing a health goal image as a sensory-based representation is essential for becoming or staying healthy since our brains respond to an internal representation similarly to a real experience. If the health goal image is appealing to the client, it will generate the motivation necessary for taking the first critical steps toward it.

### 1. Sensory-Based Representation of the Current Health Experience

- Ask the client to place a marker on the floor for their current health situation.
- Invite them to describe themselves from a metaposition, in a situation where the health issue is present.
- Then have the client step onto the marker and guide them through a brief VAKOG trance to experience the issue, focusing on a short experience to become aware of the submodalities without intensifying them or creating a stuck state.
  - Ask: "How large do you perceive yourself? Are you your actual size, smaller, or larger? What is your posture, gesture, expression?"
  - "How much space do you have for yourself here?"
  - "What specifically do you see, hear, feel, smell, and perhaps taste here?"
- Use a separator to disengage.

### 2. Sensory-Based Representation of the Health Image

- From the meta-position, invite the client to imagine themselves dissociated as "healthy" and to grant themselves this state 100%.
  - If there are reservations, use a hypnotic language pattern like: "Just suppose, even if only for a moment, you allowed yourself this 100%... What would be different?"
  - "What does this healthy, dissociated self look like? How do you recognize it as healthy?" (eye expression, gestures, facial expressions, skin tone, type of movement, etc.)
  - "How does the dissociated self perceive its surroundings? Is it in color or black-and-white?"
  - "What does your dissociated self smell like, or what scents does it perceive?"
  - "What sounds does this healthy dissociated self make?" (voice tone, etc.) "And what sounds, tones, or noises does it perceive?"
  - "How does this dissociated self feel if you were to touch it?" (skin texture, muscle firmness, body temperature, etc.)
  - "How much space does your healthy, dissociated self have?"

### 3. Adding Submodalities

- Enrich the health image with the symbolic submodalities from Step 1, for instance, imagining a color as an aura around the dissociated self.
- Ensure the imagination uses the submodalities for "reality." Invite the client to enhance the health image with the submodalities identified as "real" in Step 1.

### **Step 3: Conclusion**

- Invite the client to experience the goal state in a VAKOG trance, reinforcing each sensory modality's submodalities. Focus particularly on the feeling and encourage the client to strengthen it throughout their body and perhaps anchor it with a small gesture. The kinesthetic channel, or body sensation, is especially crucial. If there is a feeling associated with the goal image, implementation becomes easier. You can also offer to anchor the sensation between their shoulder blades with your hand.
- A possible addition with a trance at the end of the VAKOG trance: "And perhaps you would like to let your unconscious decide how, when, and where it will use this imagination as a powerful attractor for your health."

### **12.4.2 Aligning Head, Heart, and Gut**

As described in Chapter 12.1, science currently suggests that the body's main organs are surrounded by brain-like cells. This implies that the perception of these organs can be influenced through mental space. For instance, if we ask someone, "What does your gut feeling say about this?" we usually receive an immediate response tied to that organ. The same applies when asking, "What does your heart say about this?" We can leverage these phenomena in coaching to check decisions. It's essential to pose clear questions with yes-or-no answers; vague questions trigger vague feelings.

The answers from the head, heart, and gut can vary greatly. Clear decisions are usually straightforward to confirm. Decisions where the head, heart, and gut disagree tend to feel ambivalent. When one organ gives a "no," we can ask what it would take to support the decision on that sensory level. Clients typically have a spontaneous sense of what needs clarification and what might significantly impact their decision.

This format can also be self-applied. Here's how:

1. Consider a dilemma involving a decision, such as buying a new item, choosing a travel destination, or leaving a job or partner.
2. Relax and deepen your focus through breathing.
3. Formulate one side of the dilemma as a clear positive statement: "I will do X."
4. Place your dominant hand on your heart and repeat the statement, noting how your heart feels about it. Is your heart in complete agreement, or are there doubts? Rate the level of doubt on a 5-point scale (0 = completely disagree; 5 = fully agree).
5. Place your dominant hand on your forehead and repeat the statement, noting how your head feels about it. Use the same 5-point scale.
6. Place your dominant hand on your abdomen and repeat the statement, noting how your gut feels about it. Use the 5-point scale again.

7. Explore the source of doubt from each center.

### **12.4.3 Health Panorama**

The Social Panorama model is discussed in detail in Chapter 10. Here, we'll emphasize the specific aspects critical to the health panorama.

Many chronic health issues, such as skin problems, migraines, herniated discs, gastrointestinal disorders, or allergies, are triggered or exacerbated by external stress factors in the Social Panorama. Even conditions like tumors may be additionally triggered by these stressors.

Let us reframe this perspective: in a positive social environment, where relationships with people and issues are balanced and at eye level, it becomes easier to maintain or regain health. This seems almost self-evident.

Thus, the following questions are fundamental in creating a health panorama:

- Where can I place health-related issues in the space to facilitate a dialogue with them?
- How should my social environment be structured to support my health and well-being?
- Where and how should potential stressors be positioned in my Social Panorama to make them manageable?
- Which beliefs will help me maintain balance in my Social Panorama, and how can I anchor them there?
- By repositioning people, health issues, and beliefs that hinder my well-being, how can I give them a place that provides the space to be fully healthy? Where should they be positioned to establish complete eye level?

### **Summary of the format**

1. Mark the intimate space.
2. Consider how much space you allow yourself.
3. Engage with health symptoms inwardly, symbolically remove them from the body, and place them in the room. How does the health issue appear? How large or small does it seem? How much space does it occupy in your life? What belief accompanies it? Add related symptoms if applicable.
4. Gradually reposition the symptoms in the room until eye-level balance is achievable. Then, check for belief shifts.
5. When the issues settle, reintegrate them into the body and embed the new belief across every cell using VAKOG.
6. Conduct an ecology check and future-pacing.

### **12.4.4. Depression as a Spatial Phenomenon**

In the past, depressive disorders were often dismissed as mere mental downturns, but today, depression is widely recognized as a diagnosable illness. This shift is partly because more people are willing to speak openly about it, and it's also now diagnosed as a medical condition within psychology.

From a medical-therapeutic perspective, depression is a serious condition that affects the thinking, emotions, and actions of those affected, is often accompanied by disruptions in bodily functions, and causes significant distress. People suffering from depression rarely manage to free themselves from their low mood, lack of motivation, and negative thoughts on their own. When this experience is intense, medication may be necessary to help the individual out of a stuck state.

Looking at the phenomenon of depression from the perspective of Mental Space Psychology, we observe the following:

*“An important principle in the psychology of mental space is the movement of a mental object (image, feeling, memory, etc.) from the center of attention (right in front of the nose) to a less conscious area outside the central focus of perception<sup>78</sup>. This means that people experiencing difficult-to-handle phenomena can move them to the side or even to areas where they are less disturbing. This form of incomplete processing provides temporary relief, but the client still cannot effectively manage the problematic stimuli. Walker (2014) observed that traumatic images can retain a 'perceptible connection' to the person's body, keeping the emotional intensity high regardless of how far away or to the side a problematic image is moved. This suggests that merely shifting the image does not suffice for coping.”*

*Furthermore, people can train selective attention to block thoughts before they become conscious, a suppression pattern Freud called repression. However, repression has its costs, as most clinical psychologists and psychiatrists have known since Pierre Janet's work at the end of the 19th century. Over time, such suppression can lead to a variety of symptoms, many of which can be labeled “depression.”<sup>79</sup>*

The type of depression addressed in Dr. Lucas Derks' clinical experiment <sup>780</sup> can be seen as a depressive episode resulting from suppressed frustration. Symptoms can persist for many years, and individuals often have no clear idea of the cause of their sadness and lack of energy. This low mood typically appears upon waking and may recede slightly when the person is engaged in daytime activities.

When a depression arises because the client has repressed an area of life they cannot manage, certain brief technical steps have proven helpful. The simplicity and speed of the following process can paradoxically be its greatest drawback, as the relief it brings may feel so

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<sup>78</sup> ????

<sup>79</sup> Derks, 2022, Pg. 131 ff.

<sup>80</sup> Derks, Lucas, 2022, Pg. 133 ff.



disproportionate to the intensity and duration of the suffering that clients sometimes hesitate to accept it as a genuine treatment<sup>81</sup>.

## Summary of the format

### Phase One:

1. Ask the client to feel the sadness characteristic of their depression.
2. When the client confirms feeling the sadness, ask the following presupposing question: "Where in the space around you do you sense an area of darkness?"
3. Have the client indicate the exact position of this dark area.
4. Next, suggest that the client imagine the sun rising somewhere behind them, casting light on the dark area.
5. Encourage the client to move the dark area to the center of their field of vision.
6. Give the client time (around 10 minutes) and a quiet space to do this and observe what happens. Allow them to discuss their experience.
  - The dark area is expected to gradually shrink or dissolve and become noticeably lighter.
7. Then ask the client to explore what problem was hidden behind the dark area—likely something they could not handle or had given up on. Allow the client enough time to investigate this.

### Phase Two:

Dr. Lucas Derks and some colleagues have had success by continuing this process using a classic NLP technique, known as the "Social Coping Model"<sup>82</sup>.

Ask the client: "Do you know anyone, or can you imagine someone, who would be able to handle the problem hidden behind the dark area? How would they solve it? What exactly would they do?"

This example, real or imagined (the Internal Coping Model), serves as a guide for finding an appropriate response to the problem.

If the model's imagined response seems suitable, the client first tests it in a dissociated way, then steps into it. As a coach, you can support this step through a VAKOG trance.

The client needs to feel congruent with the new coping method and behavior. If they don't, you can either repeat the model approach or try other models for the same purpose.

After developing an effective coping strategy, have the client imagine their younger self from a time before the problem emerged. Encourage this younger self to adopt the coping strategy provided by the model. Once this younger self knows how to handle the issue, the client

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<sup>81</sup> ????

<sup>82</sup> Bandler & Grinder, 1979

envisions returning to this self with these new skills. Laying out the client's timeline in the space can aid this process.

A thorough check of the new behavior's ecology is essential, particularly regarding consequences if the client can no longer use the old behavior as an avoidance strategy.

Moving through both phases is vital for establishing a comprehensive solution<sup>83</sup>.

#### **12.4.5 Health Six-Step Reframing in Space**

Health issues that we attempt to suppress will always find a way to manifest physically. This is because we instinctively make mental or physical adjustments to protect an area that hurts. For our skeletal and muscular systems, this concept is familiar territory for physiotherapists or osteopaths. Many muscle blockages and tension arise from attempts to avoid pain through protective postures.

By considering the body as a spatially integrated whole, we also react physically to influences within our mental space. For instance, when we feel pressured, we may tense our back and neck muscles to counter the mental burden. Simultaneously, our entire metabolic system, including our hormonal and immune systems, responds to this stress.

When stress levels are high, we release more adrenaline and cortisol. This causes the digestive system and metabolism to adapt, mobilizing energy to endure the strain. Our hormonal system also responds to these stresses, attempting to support us. These mechanisms are essential for survival, ensuring we remain highly active and capable in emergencies. However, if this strain continues for too long, it can damage our metabolism. Examples include:

- Overweight: The body under stress tries to retain every energy reserve.
- Digestive issues: An imbalanced metabolism due to stress.
- Autoimmune diseases: Chronic stress leads to organ fatigue, e.g., the thyroid.
- Migraines: Tension in the neck that exceeds manageable levels, sometimes with neurological impairment.
- Herniated discs: High baseline tension, especially from prolonged sitting, can press discs out from between the vertebrae.
- Fertility issues: Difficulties in getting pregnant, for example, because we are internally "on the run" from ourselves or not fully present within ourselves, facing inner resistance to pregnancy, or because traditional family beliefs become active and trigger fear ("Having a child will ruin your career," "Childbirth is very painful," "There are many miscarriages in our family.")

The range of effects is vast, and our bodies often react first through specific weaknesses.

All chronic stress-related issues share a common factor: they have a spatial representation in our mental space that we respond to with submodalities, ultimately affecting our bodies.

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<sup>83</sup> Krakauer, 2000

For instance, clients experiencing a person or topic as stressful or threatening consistently report a spatial representation of it, whether as a dark, threatening cloud or as personified figures, “monsters,” etc. While varied, these representations often share descriptors such as:

- Dark
- Oversized
- Very close
- Frontal
- Foggy
- Obstructive
- Murky
- Restrictive
- Fixed
- Immobile
- Frozen

Every client we’ve worked with was able to assign these representations a specific location in space, even those with no prior coaching experience. This consistent spatial representation of health issues led us to the natural conclusion that direct engagement with these spatial phenomena is essential.

In Mental Space Psychology (MSP) and NLP, this is known as “**parts work**,” as each perception within us has, in some way, become a part of us that has learned to cope with stress in a specific manner. From a neurobiological perspective, we might refer to this as a Mental Space Identifier (MSI) (see Chapter 6.6), representing a specific stimulus-response pattern.

It’s important to remember that we cannot simply “eliminate” any unpleasant part of our personality. Since body, mind, and soul form an inseparable unit, unresolved issues will find other ways to manifest within us, indicating an internal imbalance. The willingness to acknowledge and address health issues constructively is the most crucial step toward recovery and maintaining health. Working with these topics spatially provides excellent access points, as demonstrated in the Six-Step Reframing in Space format. This approach is based on the Six-Step format, a core component of every NLP Practitioner training (DVNLP), which we have enhanced with MSP principles.

### **Step 1**

Work with your coachee to establish a positive reference experience, allowing them to experience creating and calibrating a protected space for themselves.

Mark the intimate space (see also Chapter 9.2.1) in the form of a circle around the person, approximately at arm’s length. Slowly and cautiously approach the person from the front, asking where the boundary of this neutral space is and where the body begins to react. As a coach, you can observe this reaction in the subtle micro-expressions and gestures on the face, which you can mirror back to the coachee.

Once the space is calibrated, invite the person to imagine a sphere that protects this space. Ask them to visualize illuminating their space, which, in our experience, greatly helps establish positive submodalities often associated with a good state.

## **Step 2**

Ask the coachee to connect with their problem and place their hand on the part of their body where they feel the sensation most strongly. Briefly ask them to describe how it feels—whether it's warm or cool, what color it has, and perhaps even its shape.

Invite the coachee to take the problem out of their body, place it in the room, and assign it a location. If there's any resistance, you could ask, "Imagine that your problem is about to come through the door—what would it look like, and where would it position itself?" In our experience, every coachee has been able to locate it.

Next, ask how much space the coachee has left once this phenomenon (the part) has entered the room. The metaphors the coachee uses to describe the phenomenon or part across all sensory channels provide the foundation for working with it.

If multiple parts appear, ask which part takes the lead. This is the part you should work with first. Now, bring the part to life, as if the protagonist is stepping onto the stage in a play. Engaging actively with the problem or personality part in this way can make it easier for the coachee to connect with their topic. Ask the coachee for a detailed description, e.g., "What exactly does this part or problem look like?" "What shape does it take?" Mark the location in the room with a floor anchor to create a visible reference point.

## **Step 3**

Regardless of how the part presents itself, always emphasize to the coachee that all parts of us serve a fundamentally positive purpose; sometimes, they simply resort to a health issue to draw attention to themselves. Without the pain, we might not notice certain issues and might attribute neck pain solely to stress. This approach is fine. However, if the coachee is genuinely ready to work on their issue, the Six-Step method in space has proven to be a very helpful tool.

Once the part is "alive," it's time for the coachee to communicate with it. As in the Six-Step format, the coachee and their part establish communication signals—yes/no signals—to indicate agreement or disagreement.

## **Step 4**

The next crucial step is uncovering the positive intention. As in the Six-Step NLP format, clarify the purpose behind, for example, neck pain, and what it wants to signal to the coachee. Initially, coachees may feel confused, thinking, "How can pain be helpful? It feels so negative and limits my life." Here, as a coach, hold the thread gently and ask further respectful questions, e.g., "Suppose the pain and the restriction it brings were trying to give you a message...what could that be?"

Continue asking until the core intention of the part becomes clear to the coachee. Often, it's about letting go of burdensome issues or people, giving them a new place, or allowing oneself more rest. Just recognizing this positive intention can change the part's representation in terms of shape and position within the space. If there are multiple parts, work to determine which one is the guardian of the positive intention.

### **Step 5**

Invite the coachee to connect with their creative part, bringing it into the space and establishing yes/no signals with it. It's worthwhile to let this part come alive by engaging the sensory channels, thereby making its creative energy accessible for the process.

The creative part's sole task here is to suggest ideas. Mention that creative parts often suggest unique, "out-of-the-box" ideas. Each idea from the creative part is then presented individually to the main responsible part (the guardian of the positive core intention) for approval. Only when the part gives a clear OK is the suggestion accepted. If there's any hesitation, ask if there's a condition or reservation. This ensures the solution is attractive enough for the reward system to motivate a change. Also, make sure the suggestions are practical so they can be realistically implemented.

### **Step 6**

Once the three suggestions are developed, observe where the main part (protagonist) positions itself in the room and how it presents itself. Do the same with any additional parts if necessary. Now ask each part if they're satisfied with their position and solution. The coachee can thank them for their cooperation.

### **Step 7**

Since almost all chronic health issues are closely linked to identity and associated beliefs, the final step is to explore what changes on the level of identity and beliefs once the original issue has been transformed and the body no longer needs its previous coping strategy.

Ask the coachee, "What beliefs now support and sustain you?" In our experience, it takes some time to find the right beliefs, and careful wording is important. Then, clarify what has changed at the identity level: "Who are you now that you've resolved this issue, and what does this enable you to do as a person?"

These two questions often have a profound emotional impact, as they encapsulate and make clear the significance of resolving a chronic issue on the entire personality. Finally, invite the coachee to use VAKOG to sense how they feel after the resolution and how much space they now perceive for themselves.

## **Summary of the Format**

1. Identify a reference situation where the coachee feels comfortable. Conduct a VAKOG inquiry, noting the space they experience and their sensory perceptions, then calibrate the intimate space sphere.
2. Perceive a negative situation—ask how much space the coachee has here; use VAKOG to explore their experience of the phenomenon, and inquire about its metaphor.
3. Connect with the part(s) responsible for the negative feeling. For multiple parts, identify the leader. Invite the part into the space—where and how does it enter the space? Let it come alive, as if the protagonist is stepping onto a stage.  
Explore the representation concretely and in detail. Actively support the coachee in interacting with the representations to make the experience feel real.
4. Encourage a connection between the part and the person. Regardless of how the part shows itself, emphasize that it serves a positive purpose. Establish communication signals between the part and coachee: How does the part show agreement or disagreement? Yes/No signals.
5. Uncover the positive intention of the main part (protagonist) until the core intention is clear. For multiple parts, ask which one guards the positive intention. Observe any shifts in representation within the space.
6. Call upon the creative part, bringing it into the space and inquiring about its appearance. The creative part suggests how the positive intention can be fulfilled without triggering the negative feeling. Develop at least three realistic, practical suggestions and confirm agreement with the part(s).
7. Future-pace by exploring new beliefs and who the coachee has become at the identity level through the resolution, and what this enables them to achieve.

#### **12.4.6 Allergies as a Phenomenon of Mental Space**

Robert Dilts describes allergies as an overreaction of our immune system, similar to a phobia or an outburst of anger<sup>84</sup>. A phobia is an overreaction to something that is not necessarily dangerous. The fact that allergies often have a mental and physiological aspect is evident in cases where people react to artificial flowers or even imagine a flower and have an allergic reaction.

For Robert Dilts, the immune system is equivalent to the neurobiological level of our identity and is closely linked to our self-image in mental space. He refers to allergies as a “misidentification at the identity level.” Such misidentifications often arise during transitional phases of life, such as moving from kindergarten to elementary school, from elementary to secondary school, from childhood to adolescence, or later stages like moving out, relocating, or divorce. We shift from one self-image to another, which can destabilize our immune system or heighten its alertness when there are unresolved inner conflicts or unconscious misunderstandings at the identity level.

In these transitional phases, the individual often experiences a complete reorientation in mental space that impacts all neurobiological levels.

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<sup>84</sup> Dilts, 2015, pg. 158 .

*“During such times, your immune system may mistakenly perceive something harmless to your body as a serious threat...Once your immune system recognizes something as a danger and reacts to it, it never forgets.”<sup>85</sup>*

When people successfully navigate a transition, they often report “growing out of” an allergy.

Dilts provides the example of a friend from Peru who loved eating shrimp. After moving to the United States, however, he developed a severe allergy to them. Yet, when he visited home, he had no allergic symptoms.

*“If you are not centered in your mental space and are exposed to something harmless, your immune system might signal danger, as though trying to protect you from a foreign intruder in your intimate space.”<sup>86</sup>*

Upon reflecting on his allergy, Dilts’ friend realized that, although he had moved to the U.S., his identity remained rooted in Peru.

#### **12.4.7. Support for Allergies**

The necessary format for this has already been described in detail in Chapter 8.2.4.1. Here, only the summary of how we apply it in the context of allergies is provided. It is essential to focus on the internal representation of the allergen in the negative situation. For the positive situation, choose one that is associated with a maximum sense of security (for the identity).

#### **Summary of the Format**

- The coach asks the coachee how long they have had the allergy and whether they are currently taking medication. It is crucial that they continue to take any prescribed medication and only adjust it in consultation with their doctor.
- Coach and coachee sit on chairs positioned slightly offset, allowing both to have an unobstructed view forward. The coach asks which hand the coachee associates with the allergy.
- The coach invites the coachee to close their eyes, takes hold of the wrist associated with the allergy, and lets it rest on the coachee's thigh. A brief VAKOG inquiry is conducted, focusing on a specific incident when the allergy occurred, whether recently or long ago.
- During the VAKOG inquiry, the coach applies gentle, comfortable pressure to the coachee's wrist pulse with their thumb. This should be brief to identify core submodalities or triggers.
- Separator
- The coach releases the wrist and holds the other wrist, asking the coachee to recall a time when they felt safe and secure. The coachee is then asked to close their eyes

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<sup>85</sup> Dilts, 2015

<sup>86</sup> Dilts, 2015

again and immerse themselves in this resourceful experience through all sensory channels and submodalities. Throughout this inquiry, the coach repeatedly presses the coachee's other wrist with their thumb. The coachee is asked to amplify these positive feelings and spread them throughout their body.

Once the feeling is strong, the coach takes both wrists, pressing both pulse points with their thumbs. The coachee is then invited to gradually transfer the resources from the positive experience to the allergy context. This process takes time, during which the coach gently moves the coachee's hands closer together while maintaining light pressure on the wrists.

The coachee keeps their eyes closed and is offered the option to bring both hands together, allowing the positive resources to flow directly into the "allergy hand." If the coachee agrees, they place their positive hand on the allergy hand, or they bring both hands together themselves.

A third new state emerges, in which the positive submodalities override the allergy state.

- Once the new state is established, the coach asks, "What values are now active here?" This question requires time for reflection.
- Next, ask, "Who are you now that a third, new state has emerged?" Give the coachee ample time to experience this, as it often results in a profound reevaluation of their identity.
- For the eco-check and future pace, the coachee keeps their eyes closed. The coach may rest their hand on the coachee's closed hands or allow them to experience how it feels to connect their hands on their own.
- Finally, the coachee opens their eyes and returns to the present moment.
- For a final eco-check, the coach asks the coachee to attempt to recall the sensation of the allergic reaction. If any reaction remains, it is typically different than at the beginning. If necessary, a second session can address this new layer of the allergy phenomenon.

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## **Biographies**

### **Brigitte Padberg**

**\*Motto\*:** "Solutions are lurking everywhere" (Steve de Shazer)

Born in 1963 and raised in Mönchengladbach, she attended high school and a business school before starting her studies in Political Science in Heidelberg, later transferring to RWTH Aachen, where she graduated with a Master of Arts. Her goal was journalism, motivated by her love of research and direct engagement with people and their stories. From 1991 to 1998, she worked as a research assistant for Bundestag member and later Parliamentary State Secretary Achim Großmann in Bonn, focusing on press and public relations. From 1999 to 2000, she served as press officer at the Rhineland Regional Association in Cologne.



Starting her career as a political scientist with the aim of “making the world a better place,” her NLP training (Practitioner, Master, Coach, Trainer, DVNLP) revealed that change always begins within ourselves and that we can only do good for others if we are grounded in ourselves. Her personal encounter with Dr. Lucas Derks, observing his highly professional and graceful handling of even “heavy topics” through Mental Space Psychology, inspired her to become a coach. “Lukas Derks’ methods have taken NLP to a new level for me. Working with mental spatial experience opens doors for coachees that were always there but often unseen, simply because the necessary tools were missing. For me, Lucas Derks is one of NLP’s greatest innovators, and I personally owe much of my joy in coaching and training work to him.”

Her firm belief that everyone possesses the resources to lead a happy, fulfilling life is a central focus in her coaching and training work, as is her focus on solutions. She specializes in health and potential development. Building the Padberg Consultancy and NLP Academy with her husband Ekkehart has given her space for personal growth and fulfilled her childhood dream of living a self-determined and fulfilling life.

### **Ekkehart Padberg**

**Motto:** “Let’s create a peaceful and friendly world we all want to belong to.”

Born in Essen in 1963 and raised in Viersen on the Lower Rhine, he completed his high school education before undertaking civilian service as a night-shift worker in an emergency room, where he encountered many life stories. Initially studying medicine, he switched to business administration with a focus on organization and industrial management at RWTH Aachen. Fascinated from the start by systemic relationships within organizations and why people make certain decisions in specific situations, he graduated in 1992 as a business economist.

After early roles in management and leadership positions, he joined KGSt consult GmbH in Cologne in 1995, which later became part of Price Waterhouse Coopers. There, he worked as a controller, consultant, and senior consultant, focusing on sustainable development concepts for companies, with dialogue-oriented change processes that engage leaders and employees in the change process.

In 2000, he co-founded Padberg Consultancy with Brigitte Padberg, specializing from the start in strategic change management. He completed his first NLP training in 2001 as a Practitioner, became a certified DVNLP coach in 2004, and an NLP Master Trainer and Coach in 2006. During this time, he trained under Tom Andreas, Robert Dilts, Stephen Gilligan, Insa Sparrer, Mathias Varga von Kibéd, Michael Grinder, Herbert Warmbier, Bernd Isert, Eleonore (Noni) Höffner, and other inspiring figures.

In 2007, he launched the first NLP training at the Padberg Consultancy NLP Academy. In 2010, he published his first book, *\*Management by Excellence – Mobilizing and Utilizing Corporate Resources\**, combining insights from NLP, business consulting, and management studies. Today, he continues to work as a corporate consultant, master coach, and NLP trainer, and as an executive coach and trainer for numerous companies.